NIDS

DATA ELEMENTS

health
Department:
Health
REPUBLIC OF SOUTH AFRICA

National Indicator Data Set

DATA ELEMENT DEFINITIONS APRIL 2017 TO MARCH 2019

Reproduction of this publication for commercial distribution is prohibited.
Requests to reprint this publication should be addressed to the Department of Health,
Director: Monitoring and Evaluation,
Nhlanhla Ntuli at NtuliNH@health.gov.za.

Note:

Implementation of the Data Elements marked with * in Chronic, Mental Health and Quality groups will be confirmed after 30 October 2016.

The production of this document was made possible by the support of Health Systems Trust.

ABOUT THIS GUIDE

The National Indicator Data Set (NIDS) is a minimum group of indicators introduced by the National Department of Health that every public health facility is expected to collect, use and report on. Facilities are required to report only on indicators in which they render the service according to their service package. The NIDS is use for monitoring and tracking progress with the implementation of national strategic goals, objectives and priority health programmes. The NIDS will help the Department of Health to identify the needs in the healthcare system and provide managers with information to guide programme development and budgeting, provide support to improve the healthcare system, and demonstrate national commitment to globally accepted measures of care.

The District Health Management Information System (DHMIS) Policy requires that the NIDS be revised every two years; the last review was done in 2013 and was implemented in April 2013. Due to the rationalisation of the data collection tools in 2015, the NIDS was not revised in 2015. The 2017 NIDS will be implemented in April 2017 and is effective until March 2019. There are special cases, for instance: policy pronouncements, which will require the Department to make additions or subtractions to the NIDS. In these cases, an official document signed by the Director-General of Health will be sent to all the provinces for implementation.

The quality of the information collected is vital to ensure that planning and the distribution of funds, equipment and personnel match the needs of the communities served by the health facility.

This resource is a comprehensive guide to the NIDS, and is designed to assist in maximising the accuracy and understanding of the data required to be collected.

Each section (or programme) in the guide is colour-coded to make its usage easier for health workers at facilities engaged in this work.

One half of the guide is dedicated to the data elements, their definitions, inclusion and exclusion criteria and collection points, and the reverse half is dedicated to the indicators calculated from these data elements.

COLOUR KEY

- Adolescent Health
- ART Baseline
- ART Monthly
- ART Outcome
- Central Chronic Medicines Dispensing and Distribution
- Child and Nutrition
- Chronic
- Communicable Diseases
- Emergency Medical Services
- Environmental Health
- Expanded Programme on Immunisation
- Eye Care
- HIV
- Malaria
- Management Inpatients
- Management PHC
- Maternal and Neonatal
- Mental Health
- Oral Health
- PHC Ward Based Outreach Teams
- Quality
- Rehabilitation
- School Health
- Sexually Transmitted Infections
- TB Monthly
- TB Quarterly
- Women's Health

ACRONYMS

ANC	Antenatal care
ART	Antiretroviral therapy
BCG	Bacille Calmette-Guérin
CCMDD	Central Chronic Medicines Dispensing and Distribution
CDC	Community day centre
CD4	Cluster of differentiation 4
CHC	Community health centre
CPT	Co-trimoxazole prevention therapy
DTaP-IPV-Hib-HBV	Diphtheria, tetanus, inactivated polio vaccine, haemophilus influenza B, hepatitis vaccine
EH	Environmental health
EMS	Emergency medical services
EXP	Experienced
HIV	Human Immunodeficiency Virus
IC	Ideal Clinic
ICD	International Classification of Diseases
ICU	Intensive care unit
IPT	Isoniazid Preventive Therapy
ISHP	Integrated School Health Programme
IUCD	Intra-uterine contraceptive device
LTF	Loss/lost to follow-up
MAM	Moderate acute malnutrition
NAM	No acute malnutrition
MDR	Multidrug-resistant
MUS	Male urethritis syndrome
NCS	National Core Standards
OHH	Outreach households
OPD	Outpatient department
OPV	Oral polio vaccine
PCR	Polymerase chain reaction
PCV	Pneumococcal conjugated vaccine
PDE	Patient-day equivalent
PHC	Primary health care
PrEP	Pre-exposure prophylaxis
PSI	Patient safety incident
PPTICRM	Perfect Permanent Team for Ideal Clinic Realisation and Maintenance

RIC	Remaining in care
RIP	Died [rest in peace]
RV	Rotavirus
STH	Soil-transmitted helminths
STI	Sexually transmitted infections
TLR	Third-line regimen
TROA	Total remaining on ART
SAC	Severity assessment code
SAM	Severe acute malnutrition
SD	Standard deviation
SLR	Second-line regimen
StatsSA	Statistics South Africa
TB	Tuberculosis
Td	Tetanus and diphtheria
TFI	Transferred in
TFO	Transferred out
VDS	Vaginal discharge syndrome
Vit A	Vitamin A
VLD	Viral load
VLS	Suppressed viral load
XDR	Extensively drug-resistant

DATA ELEMENTS

PHC Headcount 10–19 years						
	DEFINITION	Adolescents and young clients 10–19 seen for Primary Health Care	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each client once for each day they appear at the facility, regardless of the number of services provided on that day. If the client is recorded as a headcount at the registration point, the observation of the client is done and the client then leaves the facility without receiving the service that he/she is supposed to receive, a headcount can be recorded.	EXCLUSIONS	» EXCLUDE national campaign headcounts » EXCLUDE cases where a client visits a facility just to make an appointment (e.g. transport/OPD or specialist appointments) » EXCLUDE cases where a client visits a facility only to submit sputum samples produced outside of the facility (sputum samples produced at facility with assistance of health professional should be recorded as a headcount) » EXCLUDE cases where a client visits a facility to get feedback regarding examination results without seeing a professional person » EXCLUDE telephonic consultations with clients » EXCLUDE medication collected by a home/community-based care (HCBC) worker or family member/friend as a headcount registered at the registration point » EXCLUDE cases where a professional nurse appointed by a HCBC NPO renders a service to individuals or to support groups, including the distribution of medication collected at PHC facilities on behalf of clients; the headcount should not be counted by the PHC facility from where the medication is collected, BUT as part of HCBC data.	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	In cases where a PHC service is rendered on an individual basis to a client at a place other than a PHC facility by a team consisting of professional practitioners and HCBC workers, all data for the relevant data elements should be recorded by the professional practitioners and not by the HCBC worker/s. If medication for a client is collected by a home/community-based care (HCBC) worker or family member/friend, a headcount of the client whose medication is collected CANNOT be registered as a headcount at the registration point.	FREQUENCY	Monthly	TOOLS	Daily Reception Headcount Register or HPRS

Delivery 10–14 years in facility	**DEFINITION**	Delivery where the mother is 10–14 years old. These deliveries are done in facilities under the supervision of trained medical/ nursing staff.	**INCLUSIONS**	None	**COLLECTED BY**	Clinician
	EXTENDED DEFINITION	None	**EXCLUSIONS**	None	**COLLECTION POINTS**	Delivery
	USE AND CONTEXT	None	**FREQUENCY**	Monthly	**TOOLS**	Delivery Register
Delivery 15–19 years in facility	**DEFINITION**	Delivery where the mother is 15–19 years old. These deliveries are done in facilities under the supervision of trained medical/ nursing staff.	**INCLUSIONS**	None	**COLLECTED BY**	Clinician
	EXTENDED DEFINITION	None	**EXCLUSIONS**	None	**COLLECTION POINTS**	Delivery
	USE AND CONTEXT	None	**FREQUENCY**	Monthly	**TOOLS**	Delivery Register

ART-NAÏVE START ART (TOT)	DEFINITION	ART naïve clients who started life-long treatment during this quarter. Naïve clients are the sum of the following: » Clients who have never been exposed to ART for more than 30 days in total » Pregnant women initiated onto life-long ART (Option B+) This also includes clients initiated on life-long triple therapy from the: » PEP programme » dual PMTCT programme	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data elements are disaggregated by age (adult and child).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	The following clients are excluded: » New treatment experienced » Transferred in	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART ADULT MALE NAÏVE START (MEN)	DEFINITION	ART naïve male adults who started life-long treatment during this quarter. Naïve clients are the sum of the following: » Clients who have never been exposed to ART for more than 30 days in total This also includes clients initiated on life-long triple therapy from the: » PEP programme	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	The following clients are excluded: » New treatment experienced » Transferred in	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART ADULT FEMALE NAÏVE START (FEM)	DEFINITION	ART naïve female adults who started life-long treatment during this quarter. Naïve clients are the sum of the following: » Clients who have never been exposed to ART for more than 30 days in total » Pregnant women initiated onto life-long ART (Option B+) This also includes clients initiated on life-long triple therapy from the: » PEP programme » dual PMTCT programme	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The following clients are excluded: » New treatment experienced » Transferred in	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	The following clients are excluded: » New treatment experienced » Transferred in	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART CHILD UNDER 1 YEAR NAÏVE STARTED ART	DEFINITION	Children under 1 year ART naïve who started life-long treatment during this quarter. Naïve clients are the sum of the following: » Clients never been exposed to ART for more than 30 days in total This also includes clients initiated on life-long triple therapy ART from the: » PEP programme	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART CHILD 1 TO UNDER 5 YEARS NAÏVE STARTED ART	DEFINITION	Children 1 to under 5 years ART naïve who started treatment during this quarter. Naïve clients are the sum of the following: » Clients never been exposed to ART for more than 30 days in total This also includes clients initiated on life-long triple therapy ART from the: » PEP programme	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	The following clients are excluded: » New treatment experienced » Transferred in	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART CHILD 5 TO UNDER 15 YEARS NAÏVE STARTED ART	DEFINITION	Children 5 to under 15 years ART naïve who started treatment during this quarter. Naïve clients are the sum of the following: » Clients never been exposed to ART for more than 30 days in total This also includes clients initiated on life-long triple therapy ART from the: » PEP programme » The dual PMTCT programme	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	The following clients are excluded: » New treatment experienced » Transferred in	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART CD4 BELOW 100 CLS/µL AT BASELINE (C<100)	DEFINITION	Client initiated on ART with CD4 below 100 cells/µl at the time of starting ART and this was recorded in the ART monitoring tool	INCLUSIONS	INCLUDE CD4 below 15 per cent in children	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART CD4 100 TO 199 CLS/µL AT BASELINE (C100–199)	DEFINITION	Client initiated on ART with CD4 between 100 and 199 cells/µl at the time of starting ART and this was recorded in the ART monitoring tool	INCLUSIONS	INCLUDE CD4 15–19 per cent in children	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART CD4 200 TO 349 CLS/µL AT BASELINE (C200–349)	DEFINITION	Client initiated on ART with CD4 between 200 and 349 cells/µl at the time of starting ART and this was recorded in the ART monitoring tool	INCLUSIONS	INCLUDE CD4 20–25 per cent in children	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART CD4 350 TO 499 CLS/µL AT BASELINE (C350–499)	DEFINITION	Client initiated on ART with CD4 between 350 and 499 cells/µl at the time of starting ART and this was recorded in the ART monitoring tool	INCLUSIONS	INCLUDE CD4 20–25 per cent in children	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART CD4 500 CLS/µL AND ABOVE AT BASELINE (C> = 500)	DEFINITION	Client initiated on ART with CD4 500 cells/µl and above at the time of starting ART and this was recorded in the ART monitoring tool	INCLUSIONS	INCLUDE CD4 above 25 per cent in children	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART CD4 DONE AT BASELINE (CDD)	DEFINITION	Client with a CD4 count done at the time of starting ART and this was recorded in the ART monitoring tool	INCLUSIONS	INCLUDE CD4 below 25 per cent in children	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data elements are disaggregated by age (adult and child) NOTE: This CD4 count must be taken within 6 months prior to starting ART.	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART ON CO-TRIMOXAZOLE PREVENTION THERAPY AT BASELINE (CPT)	DEFINITION	Client initiated on ART who was on Co-trimoxazole prevention therapy (CPT) when initiated on ART and this was recorded in the ART monitoring tool	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART ON INH PREVENTION THERAPY AT BASELINE (IPT)	DEFINITION	Client initiated on ART who was on INH prevention therapy (IPT) when initiated on ART and this was recorded in the ART monitoring tool	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART SCREEN FOR TB AT BASELINE (TBS)	DEFINITION	Client initiated on ART and NOT already on TB treatment who was screened for tuberculosis (TB) and the screening was recorded in the ART monitoring tool	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	The data element is disaggregated by age (adult and child)	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART ON TB TREATMENT AT BASELINE (TBT)	DEFINITION	Client initiated on ART who was on tuberculosis (TB) treatment when initiated on ART and this was recorded in the ART monitoring tool	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child)	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART EXPERIENCED STARTED ART (EXP)	DEFINITION	Client with treatment experience who started ART. A treatment experience client is a client who has received prior ART for more than 30 days.	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART ADULT NAÏVE START ART IN MONTH	DEFINITION	Naïve adults who started life-long ART Include the following: » Clients who have never been exposed to ART for more than 30 days in total » Pregnant women initiated onto life-long ART (Option B+) This also includes clients initiated on life-long triple therapy from the: » PEP programme » dual PMTCT programme	INCLUSIONS	None	COLLECTED BY	Clinician; Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	The following adults are excluded: » New treatment experienced » Transferred in	FREQUENCY	Monthly	TOOLS	Tiered Systems (paper register, TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART CHILD UNDER 15 YEARS NAÏVE START ART IN MONTH	DEFINITION	Children 5 to under 15 years ART naïve who started treatment during this quarter. Naïve clients are the sum of the following: » Clients never been exposed to ART for more than 30 days in total This also includes clients initiated on life-long triple therapy ART from the: » PEP programme » The dual PMTCT programme	INCLUSIONS	None	COLLECTED BY	Clinician; Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	The following children are excluded: - New treatment experienced - Transferred in	FREQUENCY	Monthly	TOOLS	Tiered Systems (paper register, TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART ADULT REMAIN ON ART END OF PERIOD	DEFINITION	Total adults remaining on ART (Adult TROA) at the end of the reporting month are the sum of the following: » Any adult who has a current regimen in the column designating the month you are reporting on » Any adult client who has a star without a circle (someone who is not yet considered lost to follow-up (LTF) in the column designating the month you are reporting on. Clients remaining on ART equals [naïve (including PEP and PMTCT) + Experienced (Exp) + Transfer in (TFI) + Restart] minus [Died (RIP) + lost to follow-up (LTF) + Transfer out (TFO)]	INCLUSIONS	None	COLLECTED BY	Clinician; Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	NOTE that clients who have an outcome in the specified month or months previous should not be counted in this total.	FREQUENCY	Monthly	TOOLS	Tiered Systems (paper register, TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART CHILD UNDER 15 YEARS REMAIN ON ART END OF PERIOD	DEFINITION	Total children under 15 years remaining on ART (Child TROA) at the end of the reporting month are the sum of the following: » Any child under 15 years who has a current regimen in the column designating the month you are reporting on » Any child under 15 years who has a star without a circle (someone who is not yet considered lost to follow-up (LTF) in the column designating the month you are reporting on. Clients remaining on ART equals [naïve (including PEP and PMTCT) + Experienced (Exp) + Transfer in (TFI) + Restart] minus [Died (RIP) + loss to follow-up (LTF) + Transfer out (TFO)]	INCLUSIONS	None	COLLECTED BY	Clinician; Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	NOTE that clients who have an outcome in the specified month or previous months should not be counted in this total.	FREQUENCY	Monthly	TOOLS	Tiered Systems (paper register, TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART ON FIRST-LINE REGIMEN (FLR)	DEFINITION	Clients who are still continuing first-line regimen (FLR) at interval after treatment started	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART ON SECOND-LINE REGIMEN (SLR)	DEFINITION	Adults who are still continuing second-line regimen (SLR) at interval after treatment started	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART ON THIRD-LINE REGIMEN (TLR)	DEFINITION	Clients who are still continuing third-line regimen (TLR) at interval after treatment started	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART STOPPED (STO)	DEFINITION	Client with treatment discontinued/stopped (STO) at interval after treatment started. Treatment is usually stopped for medical reasons.	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART LOSS TO FOLLOW UP (LTF)	DEFINITION	Clients who are lost to follow-up (LTF) within interval after treatment started. A client is lost to follow-up if he/she did not attend the facility for 90 days. If a client returns to the facility after being lost to follow-up, the client should be subtracted again from this value.	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART DIED (RIP)	DEFINITION	Client who died (RIP) within interval after treatment started	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART TRANSFERRED OUT (TFO)	DEFINITION	Client who has been transferred out (TFO) within interval after treatment started. If a client returns to the facility after being transferred out, the client should be subtracted again from this value.	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART TRANSFERRED IN (TFI)	DEFINITION	Client who has been transferred in (TFI) within interval after treatment started	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

ART VIRAL LOAD DONE (VLD)	DEFINITION	ART client with viral load done (VLD)	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
ART VIRAL LOAD UNDER 400 CPS/ML (VLS)	DEFINITION	ART client with suppressed viral load (VLS) of under 400 copies per millilitre (cps/mL)	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	The data element is disaggregated by age (adult and child) and duration (months).	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)
PREGNANT AT ART START REMAINING IN CARE AT 12 MONTHS RATE (RIC)	DEFINITION	Woman who was pregnant at start of ART and is remaining in care 12 months later	INCLUSIONS	None	COLLECTED BY	Clinician & Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

Stable client Central Chronic Medicines Dispensing and Distribution (CCMDD) programme - new	DEFINITION	All new clients only who are enrolled on the CCMDD programme who picked up their medicines for the first time at the designated pick-up point during the current month	INCLUSIONS	None	COLLECTED BY	CCMDD providers
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	CCMDD provider
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	CCMDD information system
Stable client Central Chronic Medicines Dispensing and Distribution (CCMDD) programme - repeat	DEFINITION	All stable clients who are enrolled on the CCMDD programme who collected their medicines for second or more times at the designated pick-up point during the current month	INCLUSIONS	None	COLLECTED BY	CCMDD providers
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	CCMDD provider
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	CCMDD information system
Central Chronic Medicines Dispensing and Distribution (CCMDD) prescriptions prepared	DEFINITION	Clients for whom CCMDD prescriptions were prepared during the period (expected collection	INCLUSIONS	None	COLLECTED BY	CCMDD providers
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	CCMDD provider
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	CCMDD information system

DEWORMING DOSE 12–59 MONTHS	DEFINITION	Deworming medication given to a child, preferably every six months from 12 to 59 months	INCLUSIONS	» INCLUDE deworming medication given at schools/crèches as part of school health services; » INCLUDE deworming medication given as part of community services	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE deworming medication given during a national campaign	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
VITAMIN A DOSE 12–59 MONTHS	DEFINITION	Vitamin A dose given to a child, preferably every six months from 12 to 59 months	INCLUSIONS	» INCLUDE local mop-up activities » INCLUDE Vitamin A given at schools/ crèches as part of school health services	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Deworming medication must be given together with the Vitamin A. Consisting of a single dose of 200 000 IU. Routine Vitamin A supplementation strengthens the immune system of children. Routine Vitamin A supplementation consisting of a single dose every 6 months until 59 months (4 years 11 months)	EXCLUSIONS	» EXCLUDE Vitamin A supplement given as part of a mass national campaigns » EXCLUDE any doses given for therapeutic reasons	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Children receiving routine multivitamin syrup can still receive routine Vitamin A supplementation. If a child is scheduled to receive a routine prophylactic dose of Vitamin A and has received a treatment dose within the past month, the routine dose should be postponed for one month. Record only the doses given as part of the Vitamin A immunisation schedule. The dose given should also be recorded on the Road-to-Health Booklet.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

Infant exclusively breastfed at DTaP-IPV-Hib-HBV (Hexavalent) 3rd dose	DEFINITION	Infant reported to be exclusively breastfed at DTaP-IPV-HepB-Hib (Hexavalent) 3rd dose immunisation (preferably 14 weeks after birth)	INCLUSIONS	» INCLUDE intake of breast milk only, and only medications approved by health professional for first six months.	COLLECTED BY	Clinician
	EXTENDED DEFINITION	If immunisation is delayed, the data item must still be recorded at the time when DTaP-IPV-HepB-Hib is given. Information should be obtained from the mother/caregiver by means of a 24-hour recall on what the infant drank or ate the previous day including water, and not by a YES or NO closed-ended question. Only record as exclusively breastfed if the child received breast milk ONLY (the child may have received medicines/vaccines prescribed by a health professional). A child who has received ANY solids, milk (other than breast milk) or other liquids (including water) should not be counted.	EXCLUSIONS	» EXCLUDE intake of water and formula for first six months	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
Child under 2 years underweight - new (weight between -2 SD and -3 SD new)	DEFINITION	A child under 2 years identified as being BELOW the -2 SD line but equal to or ABOVE the -3 SD line in the Road-to-Health Booklet (RTHB)	INCLUSIONS	» INCLUDE referrals from a private general practitioner	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Children under 2 years will be weighed and the weight plotted onto the RtH card/RTHB (weight for age chart), the client folder, and other relevant recording systems whenever the child visits the facility.	EXCLUSIONS	» EXCLUDE referrals from other state health facilities since these facilities would have already recorded a new case of 'underweight for age'	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Only record children presenting for the first time as underweight for age during this episode (i.e. new cases), not those coming for follow-up. A child previously identified as underweight for age that recovered, but who is later found to be underweight again, should be counted again (new episode). Count the child regardless of the cause of the underweight. Replaces the data element underweight for age under 5 years – new case from 1 April 2013.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

CHILD UNDER 5 YEARS ON FOOD SUPPLEMENTATION NEW	DEFINITION	Child under 5 years newly started on a food supplementation programme	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count once on the day the child was started on the food supplementation programme.	EXCLUSIONS	» EXCLUDE repeat visits for the same episode » EXCLUDE clients referred from another facility for continuation of food supplementation	COLLECTION POINTS	
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
DIARRHOEA WITH DEHYDRATION NEW IN CHILD UNDER 5 YEARS	DEFINITION	Child under 5 years diagnosed with diarrhoea with some or severe dehydration (IMCI definitions)	INCLUSIONS	» INCLUDE referrals from a private general practitioner	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Diarrhoea occurs when stools contain more water than normal. Mothers usually know whether their children have diarrhoea. Dehydration is present when diarrhoea is accompanied by any 2 of the following: Lethargy or unconscious state, sunken eyes, unable to drink/drinking poorly/not drinking eagerly, pinched skin goes back very slowly, restless/irritable and/or thirsty	EXCLUSIONS	» EXCLUDE follow-up visits for diarrhoea diagnosed with dehydration during a previous visit » EXCLUDE diarrhoea without dehydration under 5 years » EXCLUDE referrals from other public health facilities since these facilities would have already recorded a new case of diarrhoea with dehydration under 5 years - new	COLLECTION POINTS	PHC, OPD and Casualties
	USE AND CONTEXT	Only record children presenting for the first time for the current episode of diarrhoea with dehydration (i.e. new case)	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; OPD, Casualties
DIARRHOEA SEPARATION UNDER 5 YEARS	DEFINITION	Child under 5 years with diarrhoea as main final diagnosis at separation	INCLUSIONS	» INCLUDE all children with severe acute malnutrition, moderate acute malnutrition or no acute malnutrition	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The ICD-10 code is A00-A09 and K52. Diarrhoea occurs when stools contain more water than normal.	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Counting separations/deaths due to diarrhoea can be complicated because the condition often co-exists with other serious conditions for which data are collected, in particular in children with HIV/AIDS	FREQUENCY	Monthly	TOOLS	Tick Register, Ward Register

DIARRHOEA DEATH UNDER 5 YEARS	DEFINITION	A child under 5 years who died in a health facility where diarrhoea was documented as the main cause of death	INCLUSIONS	Where applicable, these deaths must also be counted under SAM deaths and MAM deaths	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The ICD-10 code is A00-A09 and K52. Diarrhoea occurs when stools contain more water than normal.	EXCLUSIONS	Count only the main diagnosis. A child can only be recorded as having died from diarrhoea or from pneumonia.	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Counting separations/deaths due to diarrhoea can be complicated because the condition often co-exists with other serious conditions for which data are collected. A child who dies from diarrhoea in the health facility before being admitted, must be admitted and separated as a death.	FREQUENCY	Monthly	TOOLS	Tick Register, Ward Register
PNEUMONIA NEW IN CHILD UNDER 5 YEARS	DEFINITION	Child under 5 years classified as pneumonia according to the IMCI definition	INCLUSIONS	» INCLUDE referrals from a private general practitioner	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Pneumonia is defined as cough or chest indrawing or stridor in calm child or difficult and fast breathing. The definition of fast breathing depends on the age of the child: » Age 1 week up to 2 months: 60 breaths per minute or more = fast breathing » Age 2 months up to 12 months: 50 breaths per minute or more » Age 12 months up to 5 years: 40 breaths per minute or more	EXCLUSIONS	» EXCLUDE children under 5 years diagnosed with other respiratory infections » EXCLUDE referrals from other state health facilities, since these facilities would have already recorded a new case of pneumonia under 5 years - new	COLLECTION POINTS	PHC, OPD and Casualties
	USE AND CONTEXT	Only record children presenting for the first time with the current episode of pneumonia (i.e. new cases), not those coming for follow-up	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; OPD, Casualties

PNEUMONIA SEPARATION UNDER 5 YEARS	DEFINITION	Child under 5 years with pneumonia as main final diagnosis at separation	INCLUSIONS	» INCLUDE all children with SAM, MAM or NAM	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The ICD-10 codes are J100, J110, J120, J121, J122, J128, J129, J13, J14, J151 to J159, J160, J168, J170, J171, J172 to J173, J178, J180 to J182, J188, J189, J851, P230 to P239. Pneumonia is defined as cough and fast breathing. The definition of fast breathing depends on the age of the child: » Age 1 week up to 2 months: 60 breaths per minute or more = fast breathing » Age 2 months up to 12 months: 50 breaths per minute or more » Age 12 months up to 5 years: 40 breaths per minute or more	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Counting separations/deaths due to pneumonia can be complicated because the condition often co-exists with other serious conditions for which data are collected. A child who dies from pneumonia in the health facility before being admitted, must be admitted and separated as a death.	FREQUENCY	Monthly	TOOLS	Tick Register, Ward Register
PNEUMONIA DEATH UNDER 5 YEARS	DEFINITION	A child under 5 years who died in a health facility where pneumonia was documented as the main cause of death	INCLUSIONS	Where applicable, these deaths must also be counted under SAM deaths and MAM deaths.	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The ICD-10 codes are J100, J110, J120, J121, J122, J128, J129, J13, J14, J151 to J159, J160, J168, J170, J171, J172 to J173, J178, J180 to J182, J188, J189, J851, P230 to P239. Pneumonia is defined as cough and fast breathing. The definition of fast breathing depends on the age of the child: » Age 1 week up to 2 months: 60 breaths per minute or more = fast breathing » Age 2 months up to 12 months: 50 breaths per minute or more » Age 12 months up to 5 years: 40 breaths per minute or more	EXCLUSIONS	Count only the main diagnosis. A child can only be recorded as having died from diarrhoea or from pneumonia.	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Counting separations/deaths due to pneumonia can be complicated because the condition often co-exists with other serious conditions for which data are collected. A child who dies from pneumonia in the health facility before being admitted, must be admitted and separated as a death.	FREQUENCY	Monthly	TOOLS	Tick Register, Ward Register

SEVERE ACUTE MALNUTRITION (SAM) INPATIENT UNDER 5 YEARS	DEFINITION	An inpatient under 5 years with severe acute malnutrition (SAM). This data item is only counted at separation.	INCLUSIONS	» INCLUDE all children under five years assessed as having SAM	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The ICD-10 codes are E40, E41, E42, E43. A child under 5 years is classified to be severely malnourished if the child has a weight for height/length (WHL) below -3 Standard Deviation scores (<-3SD) and/or presence of bilateral pitting oedema and/or child (6–59 months) has a mid-upper arm circumference (MUAC) of less than 115mm. The weight-for-age chart in the Road-to-Health Booklet shows body-weight relative to age in comparison to the median (0-line).	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	The nutritional status of all children must be assessed on admission and recorded as: SAM, MAM or no acute malnutrition. Although the assessment should be done on admission, the data item is only recorded at separation.	FREQUENCY	Monthly	TOOLS	Ward Register
SEVERE ACUTE MALNUTRITION (SAM) DEATH UNDER 5 YEARS	DEFINITION	Death in child under 5 years with severe acute malnutrition (SAM)	INCLUSIONS	» INCLUDE all deaths in children under five years assessed as having SAM	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The ICD-10 codes are E40, E41, E42, and E43. A child under 5 years is classified to be severely malnourished if the child has a weight for height/length (WHL) below -3 Standard Deviation scores (<-3SD) and/or presence of bilateral pitting oedema and/or child (6–59 months) has a mid-upper arm circumference (MUAC) of less than 115mm. The weight-for-age chart in the Road-to-Health Booklet shows body-weight relative to age in comparison to the median (0-line).	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	All children under 5 years who were assessed as having SAM on admission must be recorded here, irrespective of the cause of death (which does not imply that the child died from SAM).	FREQUENCY	Monthly	TOOLS	Ward Register

MODERATE ACUTE MALNUTRITION (MAM) INPATIENT UNDER 5 YEARS	DEFINITION	An inpatient under 5 years with moderate acute malnutrition (MAM) This data item is only counted at separation.	INCLUSIONS	» INCLUDE all children under five years assessed as having MAM	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The ICD-10 code is E44. The criteria for MAM are: » MUAC (mid-upper arm circumference) 11.5 – 12.4cm, if over 6 months, OR » WHZ (weight for height) -2 and -3 and no oedema.	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	The nutritional status of all children must be assessed on admission and recorded as: SAM, MAM or no acute malnutrition. Although the assessment should be done on admission, the data item is only recorded at separation.	FREQUENCY	Monthly	TOOLS	Ward Register
MODERATE ACUTE MALNUTRITION (MAM) DEATH UNDER 5 YEARS	DEFINITION	Death in child under 5 years with moderate acute malnutrition (MAM)	INCLUSIONS	» INCLUDE all deaths in children under five years assessed as having MAM	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The ICD-10 code is E44. The criteria for MAM is: » MUAC (mid-upper arm circumference) 11.5–12.4cm, if over 6 months, » WHZ (weight for height) -2 and -3 and no oedema	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	All children under 5 years who were assessed as having MAM on admission must be recorded here, irrespective of the cause of death (it does not imply that the child died from MAM).	FREQUENCY	Monthly	TOOLS	Ward Register
NO ACUTE MALNUTRITION (NAM) INPATIENT UNDER 5 YEARS	DEFINITION	An inpatient under 5 years with no acute malnutrition (NAM). This data item is only counted at separation.	INCLUSIONS	» INCLUDE all children under five years assessed as having NAM	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The criteria for NAM are: » MUAC (mid-upper arm circumference) ≥ 12.5cm, if over 6 months, OR » WHZ (weight for height) ≥ -2 and no oedema).	EXCLUSIONS	None	COLLECTION POINTS	Inpatient Facility
	USE AND CONTEXT	The nutritional status of all children must be assessed on admission and recorded as: SAM, MAM or NAM. Although the assessment should be done on admission, the data item is only recorded at separation.	FREQUENCY	Monthly	TOOLS	Ward Register

Death in facility 29 days – 11 months	DEFINITION	Death in facility in infant 29 days after birth to under 1 year	INCLUSIONS	» INCLUDE deaths in all health facilities	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count all facility deaths for infants 29 days to under 1 year in all wards, even if the direct cause of death is not related to the diagnosis and/or reason for admission. For instance, a death due to an accident or an act of violence must be counted.	EXCLUSIONS	» EXCLUDE infants who died outside the facility (e.g. dead on arrival)	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Midnight Census
Inpatient transfer out under 1 year	DEFINITION	Inpatients under 1 year transferred to another hospital	INCLUSIONS	» INCLUDE all clients transferred out to another public or private hospital or step down facility » INCLUDE clients transferred out on the first day of their stay	COLLECTED BY	Ward staff
	EXTENDED DEFINITION	A Transfer out is a patient admitted to a ward and who is directed or physically transferred to another hospital or step-down facility for immediate admission.	EXCLUSIONS	» EXCLUDE clients referred to a primary health care clinic, community health centre, community day centre, maternity obstetric unit without inpatient facilities, etc. » EXCLUDE clients referred to other hospitals for a specialist OPD visit and similar	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	This data element is a sub-set of Inpatient transfers out under 5 years and should be included in the Inpatient transfers out under 5 years	FREQUENCY	Monthly	TOOLS	Midnight Census
Inpatient transfer out under 5 years	DEFINITION	Inpatients under 5 years transferred to another hospital during the reporting period	INCLUSIONS	» INCLUDE all clients transferred out to another public or private hospital or step down facility » INCLUDE clients transferred out on the first day of their stay	COLLECTED BY	Ward staff
	EXTENDED DEFINITION	A Transfer out is a patient admitted to a ward and who is directed or physically transferred to another hospital or step-down facility for immediate admission there.	EXCLUSIONS	» EXCLUDE clients referred to a primary health care clinic, community health centre or any other primary health care facility » EXCLUDE clients referred to other hospitals for a specialist OPD visit and similar	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	This includes those under 1 year, even if they are captured separately.	FREQUENCY	Monthly	TOOLS	Midnight Census

Death in facility 12–59 months	DEFINITION	Death in facility of a child 12–59 months	INCLUSIONS	» INCLUDE any child who died inside a health facility	COLLECTED BY	Clinicians
	EXTENDED DEFINITION	Count all inpatient deaths for children under 5 years in all wards, even if the direct cause of death is not related to the diagnosis and/or reason for admission. For instance, a death due to an accident or an act of violence must be counted.	EXCLUSIONS	» EXCLUDE children who died outside the facility (e.g. dead on arrival)	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Midnight Census
Inpatient discharge under 1 year	DEFINITION	Inpatient discharges under 1 year are admitted clients under 1 year who completed a hospital stay of at least one night and are discharged to their usual residence including home, family, etc.	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE the discharge of day clients » EXCLUDE deaths » EXCLUDE transfers to other hospitals and step-down facilities	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	This data element is a sub-set of Inpatient discharge under 5 years and should be included in the Inpatient discharge under 5 years.	FREQUENCY	Monthly	TOOLS	Midnight Census
Inpatient discharge under 5 years	DEFINITION	Inpatient discharges under 5 years are admitted clients under 5 years who completed a hospital stay that includes at least one night and are discharged to their usual residence including home, family, hostel, etc.	INCLUSIONS	» INCLUDE inpatient discharges under 1 year, even if they are captured separately	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE the discharge of day clients » EXCLUDE deaths » EXCLUDE transfers to other hospitals and step-down facilities	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Midnight Census

Client 40 years and older screened for hypertension *	DEFINITION	Client 40 years and older, not diagnosed with hypertension, screened for hypertension in the facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Screening done according to the Screening Protocol. Count only if the client has not previously been diagnosed for hypertension and is not on treatment for hypertension.	EXCLUSIONS	None	COLLECTION POINTS	PHC and OPD
	USE AND CONTEXT	This should assist with increasing the number of people detected and referred for treatment.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
Client 40 years and older screened for diabetes *	DEFINITION	Client 40 years and older, not on treatment for diabetes, screened for diabetes in the facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Screening done according to the Diabetes Treatment Guidelines. Count only ONCE when the client is screened for diabetes.	EXCLUSIONS	None	COLLECTION POINTS	PHC and OPD
	USE AND CONTEXT	This should assist with increasing the number of people detected and referred for treatment.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
Diabetes client 40 years and older new *	DEFINITION	Client above the age of 40 years who is newly diagnosed with diabetes in the facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Diagnosis made according to the Diabetes Treatment Guidelines. Count only ONCE when the client is diagnosed with diabetes. Newly diagnosed clients with a fasting blood glucose of >7mmol/L or random blood glucose >11.1mmol/L	EXCLUSIONS	» EXCLUDE existing diabetes clients	COLLECTION POINTS	PHC and OPD
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
Diabetes treatment visit *	DEFINITION	Every visit for routine care by clients on treatment for diabetes	INCLUSIONS	» INCLUDE every visit by clients on treatment	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Treatment according to the Diabetes Treatment Guidelines	EXCLUSIONS	None	COLLECTION POINTS	PHC and OPD
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
Hypertension visit by clients on treatment *	DEFINITION	Every visit for routine care by clients on treatment for hypertension	INCLUSIONS	» INCLUDE every visit by clients on treatment	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Treatment according to the Hypertension Treatment Guidelines	EXCLUSIONS	None	COLLECTION POINTS	PHC and OPD
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD

BILHARZIA CASE REPORTED						
	DEFINITION	Any new case of bilharzia reported irrespective of the causative organism	INCLUSIONS	» INCLUDE all the bilharzia cases, whether clinically or laboratory confirmed (Schistosomiasis Mansoni, Schistosomiasis Japonicum, Schistosomiasis Haematobium)	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	To add all the cases of bilharzia irrespective of the causative organism	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; OPD Tick register

EMS P1 URBAN RESPONSE UNDER 15 MINUTES	DEFINITION	P1 calls in an urban area where the response time was under 15 minutes	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	P1 (Priority 1) calls refer to calls where the clients have life-threatening injuries or illnesses and urgently need to go to a medical facility. These calls require urgent response by an ambulance.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	Response time is the time it takes an ambulance to reach an emergency medical scene, calculated from the time of the first call to the control room up to the time of arrival on the scene.	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS P1 URBAN CALLS	DEFINITION	P1 calls received from urban areas by the communication centre	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	P1 (Priority 1) calls refer to calls where the clients have life-threatening injuries or illnesses and urgently need to go to a medical facility. These calls require urgent response by an ambulance.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS P1 RURAL RESPONSE UNDER 40 MINUTES	DEFINITION	P1 calls in a rural area where the response time was under 40 minutes	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	P1 (Priority 1) calls refer to calls where the clients have life-threatening injuries or illnesses and urgently need to go to a medical facility. These calls require urgent response by an ambulance.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	Response time is the time it takes an ambulance to reach an emergency medical scene, calculated from the time of the first call to the control room up to the time of arrival on the scene	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS P1 RURAL CALLS	DEFINITION	P1 calls received from rural areas by the communication centre	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	P1 (Priority 1) calls refer to calls where the clients have life-threatening injuries or illnesses and urgently need to go to a medical facility. These calls require urgent response by an ambulance.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS P1 URBAN OBSTETRIC RESPONSE UNDER 15 MINUTES	DEFINITION	P1 Obstetric calls in an urban area where the response time was under 15 minutes	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	P1 (Priority 1) Obstetric calls refer to calls where an Obstetric client has a life-threatening condition and urgently needs to go to a medical facility. These calls require urgent response by an ambulance.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	Response time is the time it takes an ambulance to reach an emergency medical scene, calculated from the time of the first call to the control room up to the time of arrival on the scene.	FREQUENCY	Monthly	TOOLS	Tick Register EMS

EMS P1 URBAN OBSTETRIC CALLS	DEFINITION	P1 Obstetric calls received from urban areas by the communication centre	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	P1 (Priority 1) Obstetric calls refer to calls where an Obstetric client has a life-threatening condition and urgently needs to go to a medical facility. These calls require urgent response by an ambulance.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS P1 RURAL OBSTETRIC RESPONSE UNDER 40 MINUTES	DEFINITION	P1 Obstetric calls in a rural area where the response time was under 40 minutes	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	P1 (Priority 1) Obstetric calls refer to calls where an Obstetric client has a life-threatening condition and urgently needs to go to a medical facility. These calls require urgent response by an ambulance.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	Response time is the time it takes an ambulance to reach an emergency medical scene, calculated from the time of the first call to the control room up to the time of arrival on the scene.	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS P1 RURAL OBSTETRIC CALLS	DEFINITION	P1 Obstetric calls received from rural areas by the communication centre	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	P1 (Priority 1) Obstetric calls refer to calls where an Obstetric client has a life-threatening condition and urgently needs to go to a medical facility. These calls require urgent response by an ambulance	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS EMERGENCY URBAN INTER-FACILITY TRANSFER UNDER 30 MINUTES	DEFINITION	EMS emergency urban inter-facility transfer occurs when the client is transferred within 30 minutes from the call being received	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	Inter-facility transport calls refer to calls where the clients had to be transported between health facilities.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS

EMS INTER-FACILITY TRANSFER	DEFINITION	EMS inter-facility transfer occurs when the client is transferred within urban and rural areas.	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	Inter-facility transport calls refer to calls where the clients had to be transported between health facilities.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS OBSTETRIC EMERGENCY RURAL INTER-FACILITY TRANSFER UNDER 60 MINUTES	DEFINITION	EMS emergency rural inter-facility transfer occurs when the client is transferred within 30 minutes from the call being received.	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	Inter-facility transport calls refer to calls where the clients had to be transported between health facilities.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS OBSTETRIC EMERGENCY RURAL INTER-FACILITY TRANSFER	DEFINITION	EMS emergency rural inter-facility transfer occurs when the client is transferred within a rural area.	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	Inter-facility transport calls refer to calls where the clients had to be transported between health facilities.	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS
EMS CLIENTS – TOTAL	DEFINITION	Client transported by an ambulance	INCLUSIONS	» INCLUDE P1, P2 and P3 clients	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	Emergency clients (all priorities) transported by ambulance, irrespective of the number of calls or trips	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS

EMS P1 OBSTETRIC CLIENT TRANSPORTED						
	DEFINITION	Obstetric P1 client transported to a health facility by an ambulance	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS

EMS P1 CLIENT TRANSPORTED						
	DEFINITION	P1 client transported to a health facility by an ambulance	INCLUSIONS	None	COLLECTED BY	EMS personnel
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EMS Call Centre/Station
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register EMS

EH Domestic water sample collected	DEFINITION	Domestic drinking water samples collected and analysed from a Water Services Authority and non-Water Services Authority (vessels, aircraft, farms, Transnet, etc.) in terms of the standards set out in SANS 241	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	This should exclude samples collected in outbreaks and for other specific purposes.	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Domestic water sample compliant	DEFINITION	Domestic drinking water samples collected and analysed from a Water Services Authority and non-Water Services Authority (vessels, aircraft, farms, Transnet, etc.) that complied with the standards set out in SANS 241	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	This should exclude samples collected in outbreaks and for other specific purposes. Routine analysis (normal water sampling schedule and points). Refers to E-coli and Coliform organisms.	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Food sample bacteriological analysis	DEFINITION	Food samples taken for bacteriological analysis according to Regulations promulgated in terms of the Foodstuffs, Cosmetics and Disinfectants Act (54 0f 1972)	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	This includes milk samples taken for bacteriological analysis. Exclude samples for exportation certification purposes.	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Food sample bacteriologically compliant	DEFINITION	Food samples bacteriologically analysed that complied with the Foodstuffs, Cosmetics and Disinfectants Act (54 0f 1972)	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	This includes milk samples taken for bacteriological analysis. Exclude samples for exportation certification purposes.	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Food sample chemical analysis	DEFINITION	Food samples taken for chemical analysis according to the Regulations promulgated in terms of the Foodstuffs, Cosmetics and Disinfectants Act (54 0f 1972)	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	This includes imported food stuffs, salt samples, milk samples taken for chemical analysis, samples for labelling and fortified food samples (maize meal and bread flour.)	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register

EH Food sample chemically compliant	DEFINITION	Food samples chemically tested that complied with the Foodstuffs, Cosmetics and Disinfectants Act (54 Of 1972)	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	This includes imported food stuffs, salt samples, milk samples taken for chemical analysis, samples for labelling and fortified food samples (maize meal and bread flour).	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Food handling premises compliant	DEFINITION	Food-handling premises inspected and in possession of a health Certificate of Acceptability (CoA), i.e. that comply with the minimum health standards as stipulated under Regulations R918, promulgated under Act 63 of 1977, read in conjunction with the National Health Act 2003	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	Regulations R918, promulgated under Act 63 of 1977, read in conjunction with the National Health Act 2003	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Food handling premises inspected	DEFINITION	Food-handling premises inspected and assessed in terms of compliance with the minimum health standards as stipulated under Regulations R918, promulgated under Act 63 of 1977, read in conjunction with the National Health Act (61 of 2003)	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	Regulations R918, promulgated under Act 63 of 1977, read in conjunction with the National Health Act 2003	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Notifiable medical conditions reported to District Health Office	DEFINITION	Notifiable medical conditions reported to the District Health Office	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	According to National Notifiable Medical Condition Regulation	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register

EH Notifiable Medical Conditions Investigated New	DEFINITION	Notifiable medical conditions reported to the District Health Office that were investigated by EHP officers	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	According to National Notifiable Medical Condition Regulation	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Chemical Poisoning Investigated New	DEFINITION	New case of chemical poisoning (all products registered under Act 36 of 1947 and according to the Hazardous Substance Act) including pesticides, mercury, lead and other poisonous substances investigated by EHP	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Chemical Poisoning Conditions Reported to District Health Office	DEFINITION	New case of chemical poisoning (all products registered under Act 36 of 1947 and according to the Hazardous Substance Act) including pesticides, mercury, lead and other poisonous substances reported to the District Health Office	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Milling Establishment Inspected	DEFINITION	Maize meal / bread flour mill establishment that was inspected	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	Inspected according to Regulation promulgated in terms of the Foodstuffs, Cosmetics and Disinfections Act (54 of 1972) and regulations promulgated in terms of the Act	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register

EH Milling establishment compliant	DEFINITION	Maize meal/bread flour mill establishment that was inspected and found to comply with the fortification Regulation	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Municipal
	USE AND CONTEXT	Regulation promulgated in terms of the Foodstuffs, Cosmetics and Disinfections Act (54 of 1972) and Regulations promulgated in terms of the Act	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Health care waste generator inspected	DEFINITION	Registered healthcare wastegenerators inspected according to SANS 10248	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Provincial
	USE AND CONTEXT	All waste generators should be inspected at least once a year. Inspection should be done according to SANS 10248 and any other applicable legislation. Only count the first inspection that complies during the financial year. Do not include follow-up inspections.	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Health care waste generator compliant	DEFINITION	Registered healthcare waste generators inspected that complied with minimum standards according to SANS 10248	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Provincial
	USE AND CONTEXT	All waste generators should be inspected at least once a year. Inspection should be done according to SANS 10248 and any other applicable legislation. Only count the first inspection during the financial year. Do not include follow-up inspections.	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Hazardous substance dealer inspected	DEFINITION	Hazardous substance dealers inspected	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Provincial
	USE AND CONTEXT	All hazardous substance dealers must be inspected and compliant with the Hazardous Substances Act (15 of 1973) and Regulations	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH Hazardous substance dealer compliant	DEFINITION	Hazardous substance dealers inspected that comply with the regulations for hazardous substances	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Provincial
	USE AND CONTEXT	All hazardous substance dealers must be inspected and compliant with the Hazardous Substances Act (15 of 1973) and Regulations	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register

EH International conveyance (commercial) inspected at first point of entry	DEFINITION	International conveyance arrivals at each international point of entry inspected in terms of the National Port Health Standard Operating Procedures (revised SOPs)	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Port Health
	USE AND CONTEXT	In terms of the National Port Health Standard Operating Procedures (revised SOPs) and International Health Regulations (IHR, 2005), 'conveyance' means an aircraft, ship, train, road vehicle or other means of transport on an international voyage. Arrival of a conveyance means: (a) in the case of a seagoing vessel, arrival or anchoring in the defined area of a port; (b) in the case of an aircraft, arrival at an airport; (c) in the case of an inland navigation vessel on an international voyage, arrival at a point of entry; (d) in the case of a train or road vehicle, arrival at a point of entry.	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH International conveyance arrival at first point of entry	DEFINITION	International conveyance arrivals at each international point of entry	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Port Health
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH International conveyance (commercial) compliant at first point of entry	DEFINITION	International conveyances compliant at each international point of entry in terms of the National Port Health Standard Operating Procedures (revised SOPs)	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Port Health
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register

EH International imported consignment inspected	DEFINITION	International imported consignments inspected at each international point of entry in terms of the National Port Health Standard Operating Procedures (revised SOPs) and International Health Regulations (IHR, 2005)	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Port Health
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH International imported consignment arrival at point of entry	DEFINITION	International imported consignment arrivals at each international point of entry	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Port Health
	USE AND CONTEXT	In terms of the National Port Health Standard Operating Procedures (revised SOPs) and International Health Regulations (IHR, 2005), 'port' means a seaport or a port on an inland body of water where ships on an international voyage arrive or depart. 'Point of entry' means a passage for international entry or exit of travellers, baggage, cargo, containers, conveyances, goods and postal parcels as well as agencies and areas providing services to them on entry or exit.	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register
EH International imported consignment compliant at point of entry	DEFINITION	International imported consignments compliant at each international point of entry	INCLUSIONS	None	COLLECTED BY	Environmental Health Practitioner
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	EHP Port Health
	USE AND CONTEXT	In terms of the National Port Health Standard Operating Procedures (revised SOPs) and International Health Regulations (IHR, 2005), 'port' means a seaport or a port on an inland body of water where ships on an international voyage arrive or depart. 'Point of entry' means a passage for international entry or exit of travellers, baggage, cargo, containers, conveyances, goods and postal parcels as well as agencies and areas providing services to them on entry or exit	FREQUENCY	Monthly	TOOLS	Environmental Health Tick Register

BCG DOSE	DEFINITION	BCG (tuberculosis) vaccine given to a child under 1 year of age – preferably immediately after birth	INCLUSIONS	» INCLUDE BBAs if they are given BCG	COLLECTED BY	Clinician
	EXTENDED DEFINITION	All babies/infants receiving BCG should be counted, including babies coming to clinics after home deliveries and babies/infants who received their BCG later than usual due to, e.g. temporary shortages of vaccine. BCG should still be given to HIV-exposed children.	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Tuberculosis (TB) is a mycobacterial disease caused by Mycobacterium tuberculosis and is a major cause of disability and death in many parts of the world. Do not give BCG vaccine to children who are sick with AIDS and other immune-suppressing conditions. Do not give BCG to a newborn if the mother is on anti-TB drugs. Do not give BCG to a child who is older than 12 months. For hospitals: BCG dose under 1 year should be in line with number of live births in the facility.	FREQUENCY	Monthly	TOOLS	Delivery Register; PHC Comprehensive Tick Register
DTaP-IPV-Hib-HBV (Hexavalent) 1st dose	DEFINITION	DTaP-IPV-Hib-HBV (also known as Hexavalent) 1st dose vaccination given to a child under one year – preferably at around 6 weeks after birth	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The numerator includes children under one year receiving their 1st dose of Hexavalent	EXCLUSIONS	» EXCLUDE vaccines given as part of a national mass vaccination campaign	COLLECTION POINTS	All facilities
	USE AND CONTEXT	DTaP-IPV-Hib-HBV 1st dose is given together with OPV, PCV and RV. DTaP-IPV-Hib-HBV is given to children at 6, 10 and 14 weeks and at 18 months.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
RV 2nd dose under 1 year	DEFINITION	Rotavirus (RV) vaccine 2nd dose given to a child under one year, preferably around 14 weeks after birth and NOT later than 24 weeks after birth	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	If the child missed the 1st dose of RV at 6 weeks of age and is younger than 20 weeks, give the 1st dose and the 2nd dose 4 weeks later. Keep a minimum interval of 4 weeks between the 2 doses of RV.	EXCLUSIONS	» EXCLUDE vaccines given as part of a national mass vaccination campaign	COLLECTION POINTS	All facilities
	USE AND CONTEXT	RV 2nd dose is given together with DTaP-IPV-Hib-HBV and PCV.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

DTaP-IPV-Hib-HBV (Hexavalent) 3rd dose	DEFINITION	DTaP-IPV-Hib-HBV (also known as Hexavalent) 3rd dose vaccination given to a child under one year – preferably at around 14 weeks after birth	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The numerator includes children under one year receiving their 3rd dose of Hexavalent.	EXCLUSIONS	» EXCLUDE vaccines given as part of a national mass vaccination campaign	COLLECTION POINTS	All facilities
	USE AND CONTEXT	DTaP-IPV-Hib-HBV 3rd dose is given together with PCV2 and RV2. DTaP-IPV-Hib-HBV is given to children at 6, 10 and 14 weeks and at 18 months.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
Measles 1st dose	DEFINITION	Measles vaccine 1st dose given to a child under one year of age preferably at 6 months after birth	INCLUSIONS	» INCLUDE 1st doses given to children between 6 and 12 months	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Measles is an acute viral infection transmitted by close respiratory contact and may also spread via inhaled droplets. All children older than 12 months who have missed the 1st measles dose at 6 months should receive this dose immediately and receive the second dose with a 4-week interval. Do not give measles vaccine to children who are sick with AIDS and other immune-suppressing conditions.	EXCLUSIONS	» EXCLUDE vaccines given as part of a national mass vaccination campaign	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Do not give measles vaccine to children who are sick with AIDS and other immune-suppressing conditions.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
PCV 3rd dose under 1 year	DEFINITION	Pneumococcal (PCV) vaccine 3rd dose given to a child under one year, preferably around 9 months after birth	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	PCV is given to children at 6 and 14 weeks and at 9 months	EXCLUSIONS	» EXCLUDE vaccines given as part of a national mass vaccination campaign	COLLECTION POINTS	All facilities
	USE AND CONTEXT	PCV 3rd dose is usually the last vaccine to be given for a child to be fully immunised.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

Immunised fully under 1 year NEW	DEFINITION	A child who has completed his/her primary course of immunisation before the age of one year	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	A primary course comprises: » BCG » OPV 0 &1 » DTaP-IPV-Hib-HBV 1, 2 and 3 » HepB 1, 2 and 3 » RV 1 and 2 » PCV 1, 2 and 3 » Measles 1 » All doses before 1 year	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	The child should only be counted ONCE as fully immunised when receiving the last vaccine in the course AND there is documentary proof of all required vaccines (e.g. on the Road-to-Health Chart/Booklet)	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
Measles 2nd dose	DEFINITION	Measles vaccine 2nd dose given to a child from 12 months after birth	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	All children older than 12 months who have missed the 1st measles dose at 6 months should receive this dose immediately and receive the second dose with a 4-week interval	EXCLUSIONS	» EXCLUDE vaccines given as part of a national mass vaccination campaign » EXCLUDE 2nd dose given after the age of 2 years	COLLECTION POINTS	All facilities
	USE AND CONTEXT	If any child older than 2 years has not received a 1st and 2nd dose of measles vaccine, it should be given but not recorded here.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
DTaP-IPV-Hib-HBV (Hexavalent) 4th dose	DEFINITION	DTaP-IPV-Hib-HBV (also known as Hexavalent) 4th dose vaccination given to a child around 18 months after birth	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The numerator includes children up to 24 months of age receiving their 4th dose of Hexavalent.	EXCLUSIONS	» EXCLUDE vaccines given as part of a national mass vaccination campaign	COLLECTION POINTS	All facilities
	USE AND CONTEXT	DTaP-IPV-Hib-HBV is given to children at 6, 10 and 14 weeks and at 18 months	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

TD DOSE AT 6 YEARS	DEFINITION	Td booster dose given to a child around 6 years of age	INCLUSIONS	» INCLUDE doses given in 'mopping-up' awareness programmes » INCLUDE Td doses given routinely » INCLUDE Td dose at 6 years given at schools as part of school health services	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Td is tetanus plus diluted diphtheria. Do not administer Td for children who are younger than 6 years of age	EXCLUSIONS	» EXCLUDE vaccines given as part of a national mass vaccination campaign	COLLECTION POINTS	All facilities and School Health
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
TD DOSE AT 12 YEARS	DEFINITION	Td booster dose given to a child around 12 years of age	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities and School Health
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

Cataract surgery – total	DEFINITION	Number of eyes on which cataract surgery was performed	INCLUSIONS	INCLUDE surgery with and without intra-ocular lens implants performed on clients of all ages	COLLECTED BY	Theatre staff
	EXTENDED DEFINITION	Count each eye as cataract surgery performed.	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Cataract is opacity of the normally transparent lens.	FREQUENCY	Monthly	TOOLS	Theatre Register
Spectacles issued – total	DEFINITION	Number of spectacles issued to patients	INCLUSIONS	Includes all patients who receive spectacles (both ready-made and custom-made spectacles) at public health facilities	COLLECTED BY	Eye clinic staff – optometrists or ophthalmic nurses
	EXTENDED DEFINITION	This should include all new spectacles issued to patients including a new pair every 2 years. However, this should exclude spectacles re-issued due to the original pair not working for the patient.	EXCLUSIONS	Exclude spectacles re-ordered due to wrong script or not working as expected	COLLECTION POINTS	PHC (reading glasses), CHCs and hospitals
	USE AND CONTEXT	These data must be collected by a facility that issues spectacles to patients.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
Spectacles required – total	DEFINITION	Number of new spectacles (ordered) required for clients	INCLUSIONS	Includes all spectacles ordered i.e. ready-made and custom-made spectacles	COLLECTED BY	Eye clinic staff – optometrists or ophthalmic nurses
	EXTENDED DEFINITION	This should exclude re-orders based on the fault of the lab making spectacles	EXCLUSIONS	Re-ordered spectacles due to 1st pair not working for the client	COLLECTION POINTS	PHC (reading glasses), CHCs and hospitals
	USE AND CONTEXT	These data must be collected by a facility that orders spectacles for clients.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD

HIV TEST 19–59 MONTHS	DEFINITION	All children 19 to 59 months who were tested for HIV	INCLUSIONS	HIV should be diagnosed using age-appropriate HIV test (antibody or PCR test)	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net
HIV TEST POSITIVE 19–59 MONTHS	DEFINITION	All children 19 to 59 months who tested positive for HIV	INCLUSIONS	HIV should be diagnosed using age-appropriate HIV test (antibody or PCR test)	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net
HIV TEST 5–14 YEARS	DEFINITION	All children 5 to 14 years who tested for HIV	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net
HIV TEST POSITIVE 5–14 YEARS	DEFINITION	All children 5 to 14 years who tested positive for HIV antibodies	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net

HIV TEST 15 YEARS AND OLDER (EXCL ANC)	DEFINITION	HIV test client 15 years and older (excluding antenatal clients)	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE antenatal clients » EXCLUDE repeat tests on the same client to confirm diagnosis	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net
HIV TEST POSITIVE 15 YEARS AND OLDER (EXCL ANC)	DEFINITION	HIV test positive client 15 years and older (excluding antenatal clients)	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE antenatal clients » EXCLUDE repeat tests on the same client to confirm diagnosis	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net
HIV-POSITIVE CLIENT SCREENED FOR TB	DEFINITION	HIV-positive clients who have been screened for TB after positive HIV test	INCLUSIONS	» INCLUDE antenatal clients who tested HIV-positive	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick register Hospital
HIV POSITIVE CLIENT ELIGIBLE FOR IPT	DEFINITION	All HIV-positive clients eligible for Isoniazid preventive therapy (IPT). A client is eligible for IPT when they have screened negative for TB using the TB screening tool.	INCLUSIONS	» INCLUDE HIV-positive antenatal clients » INCLUDE asymptomatic HIV-positive TB contact children older than 5 years and younger than 15 years.	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count clients who are eligible for IPT prevention therapy only once.	EXCLUSIONS	» EXCLUDE asymptomatic TB contact children younger than 5 years	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Criteria for excluding active TB refer to the 4 questions on the TB screening tool: » Cough of any duration in HIV-positive; » Unintentional weight loss (e.g. > 1.5kg in last month) » Fever > 2 weeks; » Drenching night sweats	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

HIV-positive eligible client initiated on IPT	DEFINITION	All HIV-positive eligible clients started on Isoniazid preventive therapy (IPT) for the first time	INCLUSIONS	» INCLUDE HIV-positive antenatal clients » INCLUDE asymptomatic HIV-positive TB contact children older than 5 years and less than 15 years	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count only clients starting on Isoniazid preventive therapy for the first time	EXCLUSIONS	» EXCLUDE the continuous issuing of INH to HIV-positive clients on a monthly basis » EXCLUDE asymptomatic TB contact children younger than 5 years	COLLECTION POINTS	
	USE AND CONTEXT	Criteria for excluding active TB refer to the 4 questions on the TB screening tool: » Cough of any duration in HIV-positive; » Unintentional weight loss (e.g. > 1.5kg in last month) » Fever > 2 weeks; » Drenching night sweats	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
Male condoms distributed	DEFINITION	Male condoms distributed from a primary distribution site to health facilities or points in the community (e.g. campaigns, non-traditional outlets, etc.)	INCLUSIONS	» INCLUDE ONLY condoms distributed by the designated primary distribution site at sub-district and/or district level	COLLECTED BY	Administrator
	EXTENDED DEFINITION	Monitors distribution of male condoms for prevention of HIV and other STIs, and for contraceptive purposes. Note that research indicates only around 60% of distributed condoms are used for the intended purpose.	EXCLUSIONS	» EXCLUDE condoms distributed by health facilities	COLLECTION POINTS	Primary distribution site
	USE AND CONTEXT	In some cases the primary distribution site might be at a hospital. The designated primary distribution points will be registered as separate "facilities" in the DHIS.	FREQUENCY	Monthly	TOOLS	Stock control; Stock card
Female condoms distributed	DEFINITION	Female condoms distributed from a primary distribution site to health facilities or points in the community (e.g. campaigns, non-traditional outlets, etc.)	INCLUSIONS	None	COLLECTED BY	Administrator
	EXTENDED DEFINITION	Monitors distribution of female condoms for prevention of HIV and other STIs, and for contraceptive purposes	EXCLUSIONS	None	COLLECTION POINTS	Primary distribution site
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Stock control; Stock card

MEDICAL MALE CIRCUMCISION 10-14 YEARS	DEFINITION	Males 10 to 14 years who are circumcised under medical supervision	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Facilities where male circumcisions are performed
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Theatre Register; MMC Register; PHC Tick Register
MEDICAL MALE CIRCUMCISION 15 YEARS AND OLDER	DEFINITION	Males 15 years and older who are circumcised under medical supervision	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Record all males who are circumcised under medical supervision.	EXCLUSIONS	None	COLLECTION POINTS	Facilities where male circumcisions are performed
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Theatre Register; MMC Register; PHC Tick Register
CD4 DONE ON NEWLY DIAGNOSED HIV CLIENT	DEFINITION	CD4 test done on newly diagnosed HIV client	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count only the first CD4 test done on newly diagnosed HIV client.	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register or TIER.Net
HIV POSITIVE CLIENT KNOWN BUT NOT ON ART	DEFINITION	Clients with known HIV-positive status but not on ART. In the absence of documented proof, verbal confirmation of HIV status is acceptable and a CD4 count test must be done.	INCLUSIONS	None	COLLECTED BY	Clinician and Data Capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Electronic Tiered Systems (TIER.Net, SMARTER, non-standard system aligned with NDoH definitions)

MALARIA CASE REPORTED	DEFINITION	Parasitological-confirmed malaria cases reported in South Africa (includes both local transmission and imported cases)	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Surveillance system
	USE AND CONTEXT	Monitors the progress of South Africa towards elimination of malaria cases	FREQUENCY	Monthly	TOOLS	Malaria Information System
MALARIA DEATH REPORTED	DEFINITION	Malaria death reported in South Africa, resulting from primary malaria diagnosis at the time of death	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Surveillance system
	USE AND CONTEXT	Monitors the progress of South Africa towards elimination of malaria cases	FREQUENCY	Monthly	TOOLS	Malaria Information System

Hospital (public)	DEFINITION	A hospital in the public sector that is classified according to the Government Gazette as a District Hospital, Regional Hospital, Tertiary Hospital, Central Hospital or Specialised Hospital	INCLUSIONS	None	COLLECTED BY	N/A
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	N/A
	USE AND CONTEXT	This semi-permanent data element will have the value 1 for each hospital that fulfils the criteria.	FREQUENCY	On change	TOOLS	N/A
Emergency headcount – total	DEFINITION	All clients attending the emergency unit in a facility with conditions requiring emergency treatment	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	A medical emergency is an injury or illness that is acute and poses an immediate risk to a person's life or long-term health. Do not count non-emergency cases that attend the Casualty Department after hours when OPD is closed. Non-emergency cases should be counted as one of the OPD General Clinic data elements (new cases with or without referral letters or follow-up cases.	EXCLUSIONS	» EXCLUDE non-emergency cases	COLLECTION POINTS	Hospitals
	USE AND CONTEXT	Typical examples are strokes, cardiac arrests, drowning, poisoning, clients in shock, children with high fever, asthma attacks, assaults, gunshots, motor vehicle accidents, and rape cases. Emergency treatment is given to stabilise the client, whereafter the patient will usually be admitted for further treatment. If a patient who is treated in the emergency centre requires care while waiting for an ambulance, or requires stabilisation before being transported to the referral hospital, the patient should be counted as an emergency visit and should also be registered as an admission.	FREQUENCY	Monthly	TOOLS	Casualty Register
OPD headcount not referred new	DEFINITION	New clients attending a general or specialist Outpatient clinic WITHOUT a referral letter from a PHC facility or a doctor	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Clients can be seen in the emergency unit or in the OPD section of the hospital. Count each client only once for the day irrespective of the number of services provided.	EXCLUSIONS	» EXCLUDE inpatients of the same hospital seen at an outpatient clinic » EXCLUDE new OPD clients with referral letters and those for follow-up visits	COLLECTION POINTS	Hospitals
	USE AND CONTEXT	All assessments of pregnant women to determine progression of labour must be registered as an OPD visit. The assessment may take place in the emergency centre or in a specific area and the facility should keep a register of the visits. Clients/patients seen in the OPD/emergency and then admitted should be recorded as an OPD or emergency visit as well as an admission. Outpatients transported by ambulance to another facility and requiring emergency attention at a facility en route must be recorded as an emergency visit en route.	FREQUENCY	Monthly	TOOLS	Tick Register OPD

OPD HEADCOUNT REFERRED NEW						
	DEFINITION	New client attending a general or specialist Outpatient clinic with a referral letter from a PHC facility or a doctor	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Clients can be seen in the emergency unit or in the OPD section of the hospital. Count each client only once for the day irrespective of the number of services provided.	EXCLUSIONS	» EXCLUDE inpatients of the same hospital seen at an outpatient clinic » EXCLUDE new OPD clients with referral letters and those for follow-up visits » EXCLUDE cases where a private doctor examines a pregnant woman at his/her private rooms and then refers the patient for admission as his/her private client. This should only be registered as an admission and not as an OPD visit as well.	COLLECTION POINTS	Hospitals
	USE AND CONTEXT	All assessments of pregnant women to determine progression of labour must be registered as an OPD visit. The assessment may take place in the emergency centre or in a specific area and the facility should keep a register of the visits. Outpatients transported by ambulance to another facility and requiring emergency attention at a facility en route must be recorded as an emergency visit en route. If an inpatient at one facility attends a second facility as an outpatient, the second facility must record the attendance as an OPD visit.	FREQUENCY	Monthly	TOOLS	Tick Register OPD

OPD HEADCOUNT FOLLOW-UP	DEFINITION	Client attending a general or specialist Outpatient clinic for follow-up care	INCLUSIONS	» INCLUDE clients for follow-up visits at general and specialist Outpatient clinic » INCLUDE visits to service groups like physiotherapist, dietician, X-rays, etc., as well as previous inpatients returning to the hospital for a check-up	COLLECTED BY	Clinician
	EXTENDED DEFINITION	This would include visits to service groups like physiotherapist, dietician, X-rays, etc., as well as previous ward attendees returning to the hospital for a check-up. Clients can be seen in the emergency unit or in the OPD section of the hospital. Count each client only once for the day irrespective of the number of services provided.	EXCLUSIONS	» EXCLUDE inpatients seen at an outpatient clinic » EXCLUDE cases where a private doctor examines a pregnant woman at his/her private rooms and then refers the client for admission as his/her private patient. This should only be registered as an admission and not as an OPD visit as well.	COLLECTION POINTS	Hospitals
	USE AND CONTEXT	All assessments of pregnant women to determine progression of labour must be registered as an OPD visit. The assessment may take place in the emergency centre or in a specific area and the facility should keep a register of the visits. Outpatients transported by ambulance to another facility and requiring emergency attention at a facility en route must be recorded as an emergency visit en route. If an inpatient at one facility attends a second facility as an outpatient, the second facility must record the attendance as an OPD visit.	FREQUENCY	Monthly	TOOLS	Tick Register OPD
EXPENDITURE – TOTAL	DEFINITION	Total expenditure for the facility or management unit – often referred to as 'cost centre'	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	Obtain from financial manager, who should extract it from BAS	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	To quantify the total expenditure for the facility	FREQUENCY	Monthly	TOOLS	Management

INPATIENT BEDS	DEFINITION	All inpatient beds that are approved for use within the health facility, regardless of the capacity of the hospital to admit patients for inpatient care, for example if wards are closed due to staff shortages, the number of inpatient beds must NOT be decreased accordingly. The number of approved beds is a semi-permanent data element. Any increase or decrease must be motivated and can only be done through the office of the HOD, and National Department of Health should be informed when such changes occur.	INCLUSIONS	» INCLUDE Gynaecology, ICU, Maternity, Medicine, Neonatal High Care, Neonatal ICU, Paediatric, Psychiatry, Surgery and Orthopaedic beds	COLLECTED BY	Manager
	EXTENDED DEFINITION	A bed is a unit of accommodation in a health facility. A bed is used for admitting patients. An admission is a patient who undergoes the hospital formal admission process for either a patient or a day patient because they require inpatient care that cannot be administered on an outpatient basis. A bed is used for an admitted client and not for a boarder (e.g. kangaroo mothers), a well-baby, a client undergoing a procedure (e.g. in theatre) or an outpatient undergoing procedures, examinations or resuscitation. Depending on the level of care and the package of service the hospital renders, hospitals have sub-categories of inpatient beds like Medicine, Surgery, Gynaecology, Orthopaedic, Paediatrics, Maternity, Psychiatry and ICU. The total is specialities plus all others that do not appear on the identified specialities. **Intensive Care:** is a special department in a hospital where seriously ill patients, that require close observation and monitoring, are cared for by specially trained staff that provides intensive treatment for patients requiring advanced respiratory support, support of two or more failing organ systems or treatment of life threatening infections. **Surgery:** Patients are prepared for surgical procedures, where post-surgical care is provided as well as care of patients with surgical conditions that are treated by surgeons. **Medicine:** The diagnosis, treatment, and prevention of disease excluding surgery. **Paediatric:** The medical care of infants and children below 13 years of age. **Gynaecology:** Investigates and treats problems relating to the female urinary tract and reproductive organs. **Maternity:** Provides antenatal care, delivery of babies and care during childbirth, and postnatal support. *(continued on next page)*	EXCLUSIONS	» EXCLUDE operational beds » EXCLUDE delivery beds, surgical tables, recovery trolleys, cots for normal neonates (well baby cots), stretchers, chairs and recliners (e.g. as used for renal dialysis patients, etc.)	COLLECTION POINTS	Inpatient facility

INPATIENT BEDS (CONT.)	EXTENDED DEFINITION (CONT.)	**Orthopaedic:** Treats conditions related to the musculoskeletal system, including joints, ligaments, bones, muscles, tendons and nerves. **Psychiatry:** The diagnosis, prevention, study, and treatment of mental disorders. **Neonatal ICU:** Closely linked with the hospital maternity department, provides critical care for new born babies who need advanced life support. **Neonatal High Care:** Closely linked with the hospital maternity department, provides care and support for babies who need close observation or monitoring than cannot safely be provided in a maternity ward. **Hospitals should report on applicable sub-categories.**				
	USE AND CONTEXT	These beds are the usual accommodation where the patient will spend most of his/her stay. The patient may temporarily leave this unit of accommodation for surgery or examinations or treatment, but the bed will be kept for them and they will return to it after the treatment or examination. This temporary accommodation is not counted as a bed.	FREQUENCY	Annual	TOOLS	Management

DAY PATIENTS	DEFINITION	The total count of inpatients admitted and separated on the same calendar day	INCLUSIONS	» INCLUDE surgical patients admitted intentionally for a day procedure and some medical patients » INCLUDE patients treated in the emergency unit who are held in the casualty/ emergency unit or elsewhere while waiting for a bed. The patients should be admitted based on the condition of the patient as determined by the treating doctor. » INCLUDE patients treated in the emergency unit that require inpatient care while waiting for an ambulance or require stabilisation before transported to a referral hospital » INCLUDE maternity patients assessed in OPD/Emergency for admission and later identified as being in false labour who only stay for a few hours in hospital and then leave the hospital before midnight on the same day	COLLECTED BY	Manager
	EXTENDED DEFINITION	These patients do not occupy a bed at midnight. Record all day patients also under the relevant separation data element: 'Inpatient discharges – all', 'Inpatient deaths – all', or 'Inpatient transferred out – all'. The total is specialities plus all others that do not appear on the identified specialities. **Intensive Care:** is a special department in a hospital where seriously ill patients, that require close observation and monitoring, are cared for by specially trained staff that provides intensive treatment for patients requiring advanced respiratory support, support of two or more failing organ systems or treatment of life threatening infections. **Surgery:** Patients are prepared for surgical procedures, where post-surgical care is provided as well as care of patients with surgical conditions that are treated by surgeons. **Medicine:** The diagnosis, treatment, and prevention of disease excluding surgery. *(continued on next page)*	EXCLUSIONS	» EXCLUDE patients on leave at midnight » EXCLUDE patients who were supposed to stay overnight, but left of their own accord » EXCLUDE patients who do not require inpatient care but who are waiting for an ambulance for transport to a referral hospital	COLLECTION POINTS	Inpatient facility

DAY PATIENTS (CONT.)	EXTENDED DEFINITION (CONT.)	**Paediatric:** The medical care of infants and children below 13 years of age. **Gynaecology:** Investigates and treats problems relating to the female urinary tract and reproductive organs. **Maternity:** Provides antenatal care, delivery of babies and care during childbirth, and postnatal support. **Orthopaedic:** Treats conditions related to the musculoskeletal system, including joints, ligaments, bones, muscles, tendons and nerves. **Psychiatry:** The diagnosis, prevention, study, and treatment of mental disorders. **Neonatal ICU:** Closely linked with the hospital maternity department, provides critical care for new born babies who need advanced life support. **Neonatal High Care:** Closely linked with the hospital maternity department, provides care and support for babies who need close observation or monitoring than cannot safely be provided in a maternity ward.				
	USE AND CONTEXT	Depending on the level of care and package of services rendered by the hospital, day patients can be sub- categorised as Gynaecology, ICU, Maternity, Medicine, Neonatal High Care, Neonatal ICU, Paediatric, Psychiatry, Surgery and Orthopaedic. **Hospitals should report on applicable sub-categories.**	FREQUENCY	Monthly	TOOLS	Midnight Census

INPATIENT DAYS	DEFINITION	Total number of days inpatients spent in the institution during the period. A day is the count of all patients occupying a bed at midnight.	INCLUSIONS	» INCLUDE Gynaecology, ICU, Maternity, Medicine, Neonatal High Care, Neonatal ICU, Paediatric, Psychiatry, Surgery and Orthopaedic inpatient days	COLLECTED BY	Manager
	EXTENDED DEFINITION	Inpatient days should exclusively be based on the Midnight Census, not on admissions or other administrative procedures. The following rules apply: » A patient discharged on the day following the date of admission has a stay of 1 (one) patient day. » A patient discharged 2 (two) days after the date of admission (e.g. admitted on Tuesday and discharged on Thursday) has a stay of 2 (two) patient days. Depending on level of care and package of service the hospital renders, Inpatient days can be categorised as Medicine, Surgery, Gynaecology, Orthopaedic, Paediatrics, Maternity, Psychiatry, Tuberculosis and Intensive Care. The total is specialities plus all others that do not appear on the identified specialities. **Intensive Care:** is a special department in a hospital where seriously ill patients, that require close observation and monitoring, are cared for by specially trained staff that provides intensive treatment for patients requiring advanced respiratory support, support of two or more failing organ systems or treatment of life threatening infections. **Surgery:** Patients are prepared for surgical procedures, where post-surgical care is provided as well as care of patients with surgical conditions that are treated by surgeons. **Medicine:** The diagnosis, treatment, and prevention of disease excluding surgery. **Paediatric:** The medical care of infants and children below 13 years of age. **Gynaecology:** Investigates and treats problems relating to the female urinary tract and reproductive organs. **Maternity:** Provides antenatal care, delivery of babies and care during childbirth, and postnatal support. **Orthopaedic:** Treats conditions related to the musculoskeletal system, including joints, ligaments, bones, muscles, tendons and nerves. *(continued on next page)*	EXCLUSIONS	» EXCLUDE lodgers/boarders » EXCLUDE live-in babies » EXCLUDE H1 clients on leave at midnight » EXCLUDE day patients	COLLECTION POINTS	Inpatient facility

INPATIENT DAYS (CONT.)	EXTENDED DEFINITION (CONT.)	**Psychiatry:** The diagnosis, prevention, study, and treatment of mental disorders. **Neonatal ICU:** Closely linked with the hospital maternity department, provides critical care for new born babies who need advanced life support. **Neonatal High Care:** Closely linked with the hospital maternity department, provides care and support for babies who need close observation or monitoring than cannot safely be provided in a maternity ward. **Hospitals should report on applicable sub-categories.**				
	USE AND CONTEXT	Patients who are discharged but should be accommodated by the hospital due to the unavailability of immediate transport should not be admitted as inpatients but as lodgers, unless the condition of the client warrants an admission. If a patient is scheduled for an OPD visit at another hospital and has to wait for some time at the hospital for transport reasons, the patient should not be admitted as an inpatient but as a boarder, unless the condition of the client warrants an admission. Boarder criteria: » The person should occupy a bed; » The person should receive meals; » The person should have access to bathroom facilities.	FREQUENCY	Monthly	TOOLS	Midnight Census

INPATIENT DEATHS	DEFINITION	An inpatient death is a death recorded against an admitted inpatient, including the death of a patient admitted earlier on the same day.	INCLUSIONS	» INCLUDE Gynaecology, ICU, Maternity, Medicine, Neonatal High Care, Neonatal ICU, Paediatric, Psychiatry, Surgery and Orthopaedic inpatient deaths » INCLUDE 0 to 7 days, 8 to 28 days, 29 days to 11 months, 12 to 59 months, 5 to 12 years, 13 years and older	COLLECTED BY	Manager
	EXTENDED DEFINITION	Count any death of an inpatient, even if the cause of death does not directly relate to the cause of admission or the specialty of the ward. For instance, a death due to an accident or an act of violence must be counted. The total is specialities plus all others that do not appear on the identified specialities **Intensive Care:** is a special department in a hospital where seriously ill patients, that require close observation and monitoring, are cared for by specially trained staff that provides intensive treatment for patients requiring advanced respiratory support, support of two or more failing organ systems or treatment of life threatening infections. **Surgery:** Patients are prepared for surgical procedures, where post-surgical care is provided as well as care of patients with surgical conditions that are treated by surgeons. **Medicine:** The diagnosis, treatment, and prevention of disease excluding surgery. **Paediatric:** The medical care of infants and children below 13 years of age. **Gynaecology:** Investigates and treats problems relating to the female urinary tract and reproductive organs. **Maternity:** Provides antenatal care, delivery of babies and care during childbirth, and postnatal support. **Orthopaedic:** Treats conditions related to the musculoskeletal system, including joints, ligaments, bones, muscles, tendons and nerves. **Psychiatry:** The diagnosis, prevention, study, and treatment of mental disorders. *(continued on next page)*	EXCLUSIONS	» EXCLUDE those dead on arrival » EXCLUDE stillbirths » EXCLUDE deaths in the casualty department and where the patient was not admitted as an inpatient before the death	COLLECTION POINTS	Inpatient facility

INPATIENT DEATHS (CONT.)	EXTENDED DEFINITION (CONT.)	**Neonatal ICU:** Closely linked with the hospital maternity department, provides critical care for new born babies who need advanced life support. **Neonatal High Care:** Closely linked with the hospital maternity department, provides care and support for babies who need close observation or monitoring than cannot safely be provided in a maternity ward.				
	USE AND CONTEXT	Depending on the level of care and the package of service the hospital renders, hospitals must report on the sub-categories of inpatient deaths like Medicine, Surgery, Gynaecology, Orthopaedic, Paediatrics, Maternity, Psychiatry and ICU. **Hospitals should report on applicable sub-categories.**	FREQUENCY	Monthly	TOOLS	Midnight Census
INPATIENT DISCHARGES	DEFINITION	An inpatient discharge is a patient admitted to a ward who completes an inpatient stay and is discharged to their usual residence such as home, family, correctional centre, hostel, etc. – including the discharge of a patient on the same day of admission.	INCLUSIONS	» INCLUDE Gynaecology, ICU, Maternity, Medicine, Neonatal High Care, Neonatal ICU, Paediatric, Psychiatry, Surgery and Orthopaedic inpatient discharges » INCLUDE those who self-discharge (client absconding) » INCLUDE discharges of day patients	COLLECTED BY	Manager
	EXTENDED DEFINITION	Avoid using admissions as a proxy for discharges, except as a last resort. The total is specialities plus all others that do not appear on the identified specialities. **Hospitals should report on applicable sub-categories.**	EXCLUSIONS	» EXCLUDE deaths » EXCLUDE transfers to other hospitals and step-down facilities	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Note that while there might seem to be a fine line between sub-acute facilities, frail care centres and old age homes, the difference lies in the term 'discharged to their usual residence'. If the patient is sent to a health facility other than his/her usual residence – presumably for a limited time before the client will continue to his/her usual residence – it is regarded as a transfer out. If an inpatient at one facility attends a second facility as an outpatient and the patient is then admitted after the OPD visit, the referring facility must not discharge or suspend the patient until the second facility has informed the referring facility to discharge the patient from their facility. Weekend leave: Facilities should discharge clients on weekend leave except H1 clients	FREQUENCY	Monthly	TOOLS	Midnight Census

INPATIENT TRANSFERS OUT	DEFINITION	Admitted patients who were transferred to another hospital for immediate admission there, including the transfer out of a patient admitted earlier on the same day	INCLUSIONS	» INCLUDE Gynaecology, ICU, Maternity, Medicine, Neonatal High Care, Neonatal ICU, Paediatric, Psychiatry, Surgery and Orthopaedic inpatient transfers out » INCLUDE patients transferred out to another public or private hospital or sub-acute facility » INCLUDE patients transferred out on the same day as admission	COLLECTED BY	Manager
	EXTENDED DEFINITION	Patients are directed or physically transferred to another hospital, maternity obstetric unit (MOU) with inpatient facilities, or sub-acute facility for immediate admission there. The total is specialities plus all others that do not appear on the identified specialities. **Intensive Care:** is a special department in a hospital where seriously ill patients, that require close observation and monitoring, are cared for by specially trained staff that provides intensive treatment for patients requiring advanced respiratory support, support of two or more failing organ systems or treatment of life threatening infections. **Surgery:** Patients are prepared for surgical procedures, where post-surgical care is provided as well as care of patients with surgical conditions that are treated by surgeons. **Medicine:** The diagnosis, treatment, and prevention of disease excluding surgery. **Paediatric:** The medical care of infants and children below 13 years of age. **Gynaecology:** Investigates and treats problems relating to the female urinary tract and reproductive organs. **Maternity:** Provides antenatal care, delivery of babies and care during childbirth, and postnatal support. **Orthopaedic:** Treats conditions related to the musculoskeletal system, including joints, ligaments, bones, muscles, tendons and nerves. **Psychiatry:** The diagnosis, prevention, study, and treatment of mental disorders. *(continued on next page)*	EXCLUSIONS	» EXCLUDE patients referred to a primary health care clinic, community health centre, community day centre, maternity obstetric unit without inpatient facilities, etc. » EXCLUDE patients referred to other hospitals for a specialist OPD visit and similar	COLLECTION POINTS	Inpatient facility

INPATIENT TRANSFERS OUT (CONT.)	EXTENDED DEFINITION (CONT.)	**Neonatal ICU:** Closely linked with the hospital maternity department, provides critical care for new born babies who need advanced life support. **Neonatal High Care:** Closely linked with the hospital maternity department, provides care and support for babies who need close observation or monitoring than cannot safely be provided in a maternity ward. **Hospitals should report on applicable sub-categories.**				
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Midnight Census
SEXUAL ASSAULT CASES SEEN - NEW	DEFINITION	Sexual assault case seen at health facility	INCLUSIONS	Includes unwanted or coercive sexual contact and sexual exploitation	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Male and female sexual assault cases should be counted. Use ICD10 codes for sexual assault.	EXCLUSIONS	None	COLLECTION POINTS	Hospitals
	USE AND CONTEXT	Sexual assault encompasses a range of acts to which consent has not been given and is usually accompanied by emotional or physical violence.	FREQUENCY	Monthly	TOOLS	Casualty Register

PHC HEADCOUNT UNDER 5 YEARS	DEFINITION	All individual clients who have not yet reached five years (60 months) seen for Primary Health Care	INCLUSIONS	» INCLUDE any child given individual service(s) during a visit to a health facility » INCLUDE children seen by health professionals of an outreach team » INCLUDE a DOTS visit to the facility (but not to the community, workplace or home) » INCLUDE clients who bypass the registration unit and go directly to the pharmacy to collect their own chronic medication (including CDU medication) or clients using fast lines or those to be seen by outreach teams NW: ADD: in the facility , or those to be seen by HIV counsellors; » INCLUDE clients seen by a professional nurse appointed by a NPO and seconded to a clinic/ service	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each client once for each day they appear at the facility, regardless of the number of services provided on that day. If the client is recorded as a headcount at the registration point, then leaves the facility without receiving the service that he/she is supposed to receive, a headcount can be recorded.	EXCLUSIONS	» EXCLUDE national campaign headcounts » EXCLUDE cases where a client visits a facility just to make an appointment (e.g. transport/OPD or specialist appointments) » EXCLUDE cases where a client visits a facility only to submit sputum samples produced outside of the facility; (sputum samples produced at facility with assistance of health professional should be recorded as a headcount) » EXCLUDE cases where a client visits a facility to get feedback regarding examination results without seeing a professional person » EXCLUDE medication collected by a Home/ Community-based Care (HCBC) worker or family member/friend as a headcount registered at the registration point » EXCLUDE cases where a professional nurse appointed by a HCBC NPO renders a service to individuals or to support groups, including the distribution of medication collected at PHC facilities on behalf of clients; the headcount should not be counted by the PHC facility from where the medication is collected, BUT as part of HCBC data.	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	If a delivery occurs at a CHC or MOU that does not admit clients as inpatients, ONLY count the mother as a PHC headcount and count the baby as a headcount ONLY if immunisation services are provided to the baby. In cases where a PHC service is rendered on an individual basis to a client at a place other than a PHC facility by a team consisting of professional practitioners and HCBC workers, all data for the relevant data elements should be recorded by the professional practitioners and not by the HCBC worker/s. If medication for a client is collected by a Home/ Community-based Care (HCBC) worker or family member/friend, a headcount of the client whose medication is collected CANNOT be registered as a headcount at the registration point.	FREQUENCY	Monthly	TOOLS	Daily Reception Headcount Register or HPRS

PHC HEADCOUNT 5–9 YEARS						
	DEFINITION	Adolescents and young clients 5–9 years seen for Primary Health Care	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each client once for each day they appear at the facility, regardless of the number of services provided on that day. If the client is recorded as a headcount at the registration point, the observations of the client are done and the client then leaves the facility without receiving the service that he/she is supposed to receive, a headcount can be recorded.	EXCLUSIONS	» EXCLUDE national campaign headcounts » EXCLUDE cases where a client visits a facility just to make an appointment (e.g. transport / OPD or specialist appointments) » EXCLUDE cases where a client visits a facility only to submit sputum samples produced outside of the facility; (sputum samples produced at facility with assistance of health professional should be recorded as a headcount) » EXCLUDE cases where a client visits a facility to gets feedback regarding examination results without seeing a professional person » EXCLUDE telephonic consultations with clients » EXCLUDE medication collected by a Home/ Community-based Care (HCBC) worker or family member/friend as a headcount registered at the registration point » EXCLUDE cases where a professional nurse appointed by a HCBC NPO renders a service to individuals or to support groups, including the distribution of medication collected at PHC facilities on behalf of clients; the headcount should not be counted by the PHC facility from where the medication is collected, BUT as part of HCBC data.	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	If a delivery occurs at a CHC or MOU that does not admit clients as inpatients, ONLY count the mother as a PHC headcount and count the baby as a headcount ONLY if immunisation services are provided to the baby. In cases where a PHC service is rendered on an individual basis to a client at another place than a PHC facility by a team consisting of professional practitioners and HCBC workers, all data for the relevant data elements should be recorded by the professional practitioners and not by the HCBC worker/s. If medication for a client is collected by a Home/ Community-based Care (HCBC) worker or family member/friend, a headcount of the client whose medication is collected CANNOT be registered as a headcount at the registration point.	FREQUENCY	Monthly	TOOLS	Daily Reception Headcount Register or HPRS

PHC HEADCOUNT 20 YEARS AND OLDER						
	DEFINITION	All individual clients 20 years and older seen for Primary Health Care	INCLUSIONS	» INCLUDE clients seen by health professionals of an outreach team » INCLUDE a DOTS visit to the facility (but not to the community, workplace or home) » INCLUDE clients who bypass the registration unit and go directly to the pharmacy to collect their own chronic medication (including CDU medication) or clients using fast lines or those to be seen by outreach teams, or those to be seen by HIV counsellors » INCLUDE clients seen by a professional nurse appointed by a NPO and seconded to a clinic/ service	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each client once for each day they appear at the facility, regardless of the number of services provided on that day. If the client is recorded as a headcount at the registration point, and the client then leaves the facility without receiving the service that he/she is supposed to receive, a headcount can be recorded.	EXCLUSIONS	» EXCLUDE cases where a client visits a facility only to submit sputum samples produced outside of the facility; (sputum samples produced at facility with assistance of health professional should be recorded as a headcount) » EXCLUDE cases where a client visits a facility to gets feedback regarding examination results without seeing a professional person » EXCLUDE telephonic consultations with clients » EXCLUDE medication collected by a Home/ Community-based Care (HCBC) worker or family member/friend as a headcount registered at the registration point » EXCLUDE cases where a professional nurse appointed by a HCBC NPO renders a service to individuals or to support groups, including the distribution of medication collected at PHC facilities on behalf of clients; the headcount should not be counted by the PHC facility from where the medication is collected, BUT as part of HCBC data.	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	If a delivery occurs at a CHC or MOU that does not admit clients as inpatients, ONLY count the mother as a PHC headcount and count the baby as a headcount ONLY if immunisation services are provided to the baby. In cases where a PHC service is rendered on an individual basis to a client at another place than a PHC facility by a team consisting of professional practitioners and HCBC workers, all data for the relevant data elements should be recorded by the professional practitioners and not by the HCBC worker/s. If medication for a client is collected by a Home/ Community-based Care (HCBC) worker or family member/friend, a headcount of the client whose medication is collected CANNOT be registered as a headcount at the registration point.	FREQUENCY	Monthly	TOOLS	Daily Reception Headcount Register or HPRS

PHC CLIENT SEEN BY PROFESSIONAL NURSE	DEFINITION	A PHC client of any age consulted and/or treated by a professional nurse for a PHC service	INCLUSIONS	» INCLUDE clients seen for curative and preventative services	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Any client seen by a Professional Nurse should be counted. Each client is counted once for each day they appear at the facility, regardless of the number of services provided on the day.	EXCLUSIONS	» EXCLUDE telephonic consultations with clients » EXCLUDE other categories of staff	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Each client is counted every time they are seen by a Professional Nurse.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
PHC PROFESSIONAL NURSE CLINICAL WORK DAYS	DEFINITION	The actual work days by Professional Nurses, irrespective of rank, used to perform clinical services	INCLUSIONS	» INCLUDE work days of roving nurses, e.g. specialised Psychiatric Nurses, Ophthalmic Nurses, etc. » INCLUDE work days of PHC staff working in the antenatal clinics ONLY in the case of MOUs NW: » INCLUDE work days of staff working in the maternity unit in the case of CHCs/MOUs	COLLECTED BY	Manager
	EXTENDED DEFINITION	Days spent on PHC clinical and normal daily administrative work related to client care, are counted	EXCLUSIONS	» EXCLUDE days spend on training courses » EXCLUDE days spend at meetings » EXCLUDE days spend on administrative issues only (not dealing with clients in that time) » EXCLUDE days not worked (e.g. leave, public holidays) » EXCLUDE workdays of nurses working in the pharmacy on behalf of pharmacy staff » EXCLUDE work days of staff working in the maternity unit in the case of CHCs/MOUs » EXCLUDE travelling time for mobile services	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	One actual work day is normally equivalent to an 8-hour shift (40 hours). Professional Nurses can be full-time or part-time employed. Work day equivalents: » 2 hours or less = 0.25 clinical work day » 3–4 hours = 0.5 clinical work day » 5–6 hours = 0.75 work day » 7–8 hours = 1 clinical work day » 12-hour shift = 1.5 work days The work day equivalent should be multiplied by the number of days worked per Professional Nurse. The clinical work days put in by each Professional Nurse must be added up. Do not confuse this data element with 'Clinical days open during month' – the two would only be equal when there is only ONE nurse in the facility. Only days PRIMARILY used to handle clients are included, NOT days primarily used for training courses, for example.	FREQUENCY	Monthly	TOOLS	Management

PHC CLIENT SEEN BY PUBLIC DOCTOR	**DEFINITION**	A PHC client of any age consulted and/or treated by a doctor employed in the public sector for a Primary Health Care curative and preventative service	**INCLUSIONS**	» INCLUDE clients seen by a medical doctor for the completion of a J88 form	**COLLECTED BY**	Clinician
	EXTENDED DEFINITION	Curative services entail the diagnosis and/or treatment of clients. This data element should be collected in all PHC facilities with full or part-time doctors. Clients might originally be seen by a Professional Nurse for a PHC service or may be seen directly by the doctor.	**EXCLUSIONS**	» EXCLUDE clients seen by a medical doctor for the completion of grant forms; » EXCLUDE cases where a medical doctor just renews prescriptions without seeing such clients » EXCLUDE interpretation of TB X-rays or laboratory results without seeing such clients » EXCLUDE client seen by a dentist » EXCLUDE clients seen by a psychologist » EXCLUDE telephonic consultations with regard to clients between Professional Nurses and doctors	**COLLECTION POINTS**	PHC facility
	USE AND CONTEXT	Each client is counted every time they are seen by a doctor. ONLY clients seen for curative and preventative services must be counted.	**FREQUENCY**	Monthly	**TOOLS**	PHC Comprehensive Tick Register
PHC CLIENT SEEN BY PRIVATE DOCTOR	**DEFINITION**	A PHC client of any age consulted and/or treated by a private doctor on contract for a Primary Health Care curative and preventative service. The service is provided in a public health facility.	**INCLUSIONS**	None	**COLLECTED BY**	Clinician
	EXTENDED DEFINITION	Monitoring of services rendered by private doctors employed on contract to consult PHC clients in public health facilities in accordance with the NHI objectives to increase doctor coverage.	**EXCLUSIONS**	None	**COLLECTION POINTS**	PHC facility
	USE AND CONTEXT	Each client is counted every time they are seen by a doctor. ONLY clients seen for curative and preventative services must be counted.	**FREQUENCY**	Monthly	**TOOLS**	PHC Comprehensive Tick Register

PHC PUBLIC DOCTOR CLINICAL WORK DAYS	DEFINITION	The actual work days by doctors employed in the public sector rendering a general clinical service	INCLUSIONS	» INCLUDE curative and preventative services » INCLUDE other roving doctor work days	COLLECTED BY	Manager
	EXTENDED DEFINITION	This figure should include ONLY clinical work (i.e. handling clients/clients), including administrative record-keeping related to clients/clients	EXCLUSIONS	» EXCLUDE days spend on training courses » EXCLUDE days spend at meetings » EXCLUDE days spend on administrative issues only (not dealing with clients in that time) » EXCLUDE days not worked (e.g. leave, public holidays) » EXCLUDE dentist work days » EXCLUDE psychologist work days » EXCLUDE work days by specialist doctors, e.g. psychiatrist	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Work day equivalents: » 2 hours or less = 0.25 clinical work day » 3–4 hours = 0.5 clinical work day » 5–6 hours = 0.75 work day » 7–8 hours = 1 clinical work day » 12-hour shift = 1.5 work days The work day equivalent should be multiplied by the days worked per doctor. The clinical work days put in by each doctor must be added up.	FREQUENCY	Monthly	TOOLS	Management
PHC PRIVATE DOCTOR CLINICAL WORK DAYS	DEFINITION	The actual work days by private doctors employed on contract in the public sector rendering a general clinical service	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	Monitoring of services rendered by private doctors employed on contract to consult PHC clients in public health facilities in accordance with the NHI objectives to increase doctor coverage	EXCLUSIONS	None	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Work day equivalents: » 2 hours or less = 0.25 clinical work day » 3–4 hours = 0.5 clinical work day » 5–6 hours = 0.75 work day » 7–8 hours = 1 clinical work day » 12-hour shift = 1.5 work days The work day equivalent should be multiplied by the days worked per doctor. The clinical work days put in by each doctor must be added up.	FREQUENCY	Monthly	TOOLS	Management

Any tracer item drug stock-out (clinic/CHC/CDC)	DEFINITION	Any item on the PHC tracer item list in current use that has been out of stock for ANY period during the month	INCLUSIONS	» INCLUDE ONLY fixed clinic/CHC/CDC	COLLECTED BY	Manager
	EXTENDED DEFINITION	Stock-out refers to a situation where there was no medication available to issue to a client. This data element will always be a 0 (if no tracer drug stock-out) or a 1 (if tracer drug stock-out). No blanks are allowed in the input form, nor in the data capture screen.	EXCLUSIONS	None	COLLECTION POINTS	Fixed clinic/ CHC/CDC
	USE AND CONTEXT	Current list includes: » Nevirapine 10mg/ml solution » Zidovudine 300mg tablets » DTaP-IPV-Hib-HBV (Hexavalent) Vaccine (EPI) » Adrenaline 1mg/ml injection » Co-trimoxazole 50 or 100ml suspension » Metformin 500mg or 850mg tablets » Norethisterone or Medroxyprogesterone injection » Paracetamol 500mg tablets » Sodium Chloride .09% 1L » RHZE 150/75/400/275mg tablets	FREQUENCY	Monthly	TOOLS	Management
Fixed clinic	DEFINITION	A fixed clinic renders comprehensive integrated PHC services using one-stop approach for at least 5 days per week, 8 hours per day	INCLUSIONS	None	COLLECTED BY	N/A
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	N/A
	USE AND CONTEXT	This semi-permanent data element will have the value 1 for each clinic that fulfils the criteria.	FREQUENCY	On Change	TOOLS	N/A
Fixed CHC/CDC	DEFINITION	A fixed CHC that is open 24 hours/7 days per week. A fixed CDC with extended services that is open at least 5 days per week for at least 8 hours per day	INCLUSIONS	None	COLLECTED BY	N/A
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	N/A
	USE AND CONTEXT	This semi-permanent data element will have the value 1 for each CHC or CDC that fulfils the criteria.	FREQUENCY	On change	TOOLS	N/A

ANTENATAL 1ST VISIT BEFORE 20 WEEKS	DEFINITION	A first visit by a pregnant woman to a health facility that occurs before 20 weeks after conception	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The first visit by a pregnant woman within 20 weeks after conception to primarily receive antenatal care according to BANC. The first antenatal visit is often referred to as a 'booking visit'.	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	The actual protocol followed during the visit might vary, but it should include: » Relevant screening procedures, » Laboratory tests (e.g. for syphilis), » Counselling and health promotion (often done in groups).	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
ANTENATAL 1ST VISIT 20 WEEKS OR LATER	DEFINITION	A first visit by a pregnant woman to a health facility that occurs 20 weeks after conception or later	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The first visit by a pregnant woman to a health facility 20 weeks or more after conception to primarily receive antenatal care according to BANC. The first antenatal visit is often referred to as a 'booking visit'	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	The actual protocol followed during the visit might vary but it should include: » Relevant screening procedures, » Laboratory tests (e.g. for syphilis), » Counselling and health promotion (often done in groups).	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
ANTENATAL CLIENT ALREADY ON ART AT 1ST VISIT	DEFINITION	HIV-positive antenatal client who is on ART at the time of her first antenatal visit	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
ANTENATAL CLIENT KNOWN HIV-POSITIVE BUT NOT ON ART AT 1ST VISIT	DEFINITION	Antenatal clients with known HIV-positive status but not on ART at their first antenatal visit. In the absence of documented proof, verbal confirmation of HIV status is acceptable and a CD4 count test must be done.	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD

ANTENATAL CLIENT HIV 1ST TEST	DEFINITION	Antenatal client who was tested for the first time during her current pregnancy	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	Each antenatal client who is not known HIV-positive should be tested during her 1st antenatal visit	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
ANTENATAL CLIENT HIV 1ST TEST POSITIVE	DEFINITION	Antenatal clients who tested positive for the first HIV test done during the current pregnancy	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	Count ONLY once on the day the HIV test was confirmed positive.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
ANTENATAL CLIENT HIV RE-TEST	DEFINITION	Antenatal clients who tested negative for HIV during an earlier antenatal visit and were re-tested for HIV during the pregnancy	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Each antenatal client whose previous HIV test was negative should be re-tested every 12 weeks during pregnancy to detect late sero-converters.	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
ANTENATAL CLIENT HIV RE-TEST POSITIVE	DEFINITION	Antenatal client who was tested positive for HIV after testing negative for HIV during an earlier antenatal visit	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	Count ONLY once on the day the HIV test was done.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD

Maternal death in facility	DEFINITION	Maternal death is death occurring during pregnancy, childbirth and puerperium within 42 days of termination of pregnancy, irrespective of the duration and site of pregnancy and the cause of death (obstetric and non-obstetric).	INCLUSIONS	» INCLUDE the additional category "pregnancy-related death" to facilitate identification of maternal deaths in circumstances in which cause of death attribution is inadequate » INCLUDE maternal deaths to infants born alive before arrival at health facilities	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Inpatient Facility and Casualty/OPD
	USE AND CONTEXT	This should be collected in all units of a health facility.	FREQUENCY	Monthly	TOOLS	Tick Register Ward; Tick Register OPD
Antenatal client start on ART	DEFINITION	HIV-positive antenatal clients who were initiated on ART during their current pregnancy	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	This may be viewed as an ART data element but is crucial for monitoring effective implementation of the PMTCT programme. Collect ONLY at facility where ART is initiated.	EXCLUSIONS	None	COLLECTION POINTS	ART site
	USE AND CONTEXT	Up until 1 April 2013 the criteria for ART initiation for antenatal clients were: HIV-positive antenatal client with a CD4 count under the specified threshold and/or a WHO staging of 4. From 1 April 2013 all HIV-positive antenatal clients who are not already on ART will be initiated on the ART Fixed-dose Combination (FDC)	FREQUENCY	Monthly	TOOLS	ART Register
Mother postnatal visit within 6 days after delivery	DEFINITION	Postnatal visits by a mother within 6 days after delivery	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The visit can be at a PHC facility or a postnatal home visit by facility staff and includes mother who are hospitalised within 6 days. The purpose of the visit is for a postnatal check-up.	EXCLUSIONS	None	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Count only the first visit after delivery. The postnatal protocol should be followed. Although there may be slight differences, this element serves as a proxy for infant postnatal visit within 6 days after delivery.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

BORN ALIVE BEFORE ARRIVAL AT FACILITY	DEFINITION	Live infant born to a woman who had intended/booked a facility delivery but delivered before arrival and reached a health facility within 72 hours for normal post-delivery care (BBAs)	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The assumption is that the mother will proceed to the (nearest) facility for medical care as soon as possible after delivering, and in general arrive within a few hours. Live birth is the complete expulsion or extraction from its mother of a product of conception, irrespective of the duration of the pregnancy, which, after such separation, breathes or shows any other evidence of life, such as beating of the heart, pulsation of the umbilical cord, or definite movement of involuntary muscles, whether or not the umbilical cord has been cut or the placenta is attached. Live births (BBA) should only be counted when the foetus is of 26 or more weeks' gestational age and/or weighs 500g or more.	EXCLUSIONS	» EXCLUDE infants weighing less than 500g	COLLECTION POINTS	Delivery
	USE AND CONTEXT	Multiple births are counted as several live births.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Delivery Register; Tick Register Casualty
DELIVERY BY CAESAREAN SECTION	DEFINITION	A Caesarean section delivery in facility is the removal of the foetus, placenta and membranes by means of an incision through the abdominal and uterine walls – obviously only done in health facilities by doctors	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	This can be further divided into: i) Caesarean section in labour and ii) Caesarean section, no labour – also known as an elective Caesarean section. Multiple births should be counted as 1 delivery and 2 (or more) births irrespective of the method of delivery. In the case of multiple births, where 1 (or more) infant is a vaginal delivery and 1 (or more) by Caesarean section – count/report the more complex method, i.e. Caesarean delivery.	EXCLUSIONS	» EXCLUDE hysterectomies	COLLECTION POINTS	Delivery
	USE AND CONTEXT	This might be collected directly as a separate data element, or it could be a calculated data element based on other sub-elements collected. The typical cases are: » Caesarean section in labour; » Caesarean section, no labour also known as an elective Caesarean section	FREQUENCY	Monthly	TOOLS	Delivery Register

Delivery in facility – total	DEFINITION	Any delivery taking place in a health facility under the supervision of trained medical/nursing staff	INCLUSIONS	» INCLUDE vaginal breech deliveries » INCLUDE Caesarean sections » INCLUDE assisted deliveries	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Multiple births should be counted as 1 delivery and 2 (or more) births irrespective of the method of delivery. In the case of multiple births, where 1 (or more) infant is a vaginal delivery and 1 (or more) by Caesarean section – count/report the more complex method, i.e. Caesarean delivery.	EXCLUSIONS	» EXCLUDE deliveries taking place before arrival at the facility (BBAs) or home deliveries » EXCLUDE miscarriages and terminations of pregnancy (ToPs)	COLLECTION POINTS	Delivery
	USE AND CONTEXT	On average, expect multiple births to be around 15 per cent of the total – usually higher in referral hospitals and lower in Midwife Obstetric Units due to referral of clients expecting twins or triplets.	FREQUENCY	Monthly	TOOLS	Delivery Register
Live birth under 2 500g in facility	DEFINITION	Infant born alive in health facility who weighs less than 2 500g	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Live birth is the complete expulsion or extraction from its mother of a product of conception, irrespective of the duration of the pregnancy, which, after such separation, breathes or shows any other evidence of life, such as beating of the heart, pulsation of the umbilical cord, or definite movement of involuntary muscles, whether or not the umbilical cord has been cut or the placenta is attached. The weight range relates to the weight of the infant immediately after delivery. Count the number of infants born when twins or multiple births occur.	EXCLUSIONS	» EXCLUDE live births less than 500g » EXCLUDE stillbirths	COLLECTION POINTS	Delivery
	USE AND CONTEXT	Live births should only be counted when the foetus is of 26 or more weeks' gestational age and/or weighs 500g or more.	FREQUENCY	Monthly	TOOLS	Delivery Register
Live birth in facility	DEFINITION	Live birth resulting from a delivery in a facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Live birth is the complete expulsion or extraction from its mother of a product of conception, irrespective of the duration of the pregnancy, which, after such separation, breathes or shows any other evidence of life, such as beating of the heart, pulsation of the umbilical cord, or definite movement of involuntary muscles, whether or not the umbilical cord has been cut or the placenta is attached. The weight range relates to the weight of the infant immediately after delivery. Count the number of infants born when twins or multiple births occur.	EXCLUSIONS	» EXCLUDE infants weighing less than 500g » EXCLUDE stillbirths	COLLECTION POINTS	Delivery
	USE AND CONTEXT	A live birth should not be admitted as an inpatient but as a live-in infant. When live-in infants require treatment, they must be admitted as inpatients. Live births should only be counted when the foetus is of 26 or more weeks' gestational age and/or weighs 500g or more.	FREQUENCY	Monthly	TOOLS	Delivery Register

STILLBIRTH IN FACILITY	DEFINITION	Stillborn infants delivered in a health facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Stillbirth is death prior to the complete expulsion or extraction from its mother of a product of conception. The death is indicated by the fact that after such separation, the foetus does not breathe or show any evidence of life such as beating of the heart, pulsation of the umbilical cord or definite movement of the involuntary muscles. Stillbirths should only be counted when the foetus is of 26 or more weeks' gestational age and/or weighs 500g or more. A stillborn foetus might have been dead for a while (macerated) or died just before or during expulsion or extraction (fresh) from its mother.	EXCLUSIONS	» EXCLUDE still births less than 500g	COLLECTION POINTS	Delivery
	USE AND CONTEXT	Multiple births are counted as several stillbirths (or as a combination of live and stillbirths, if one foetus is alive and another not).	FREQUENCY	Monthly	TOOLS	Delivery Register
LIVE BIRTH TO HIV-POSITIVE WOMAN	DEFINITION	Live birth to a woman known to be HIV-positive at delivery	INCLUSIONS	» INCLUDE infants born before arrival (BBA) at health facilities and infants born outside health facilities if they arrive within 72 hours of birth	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Includes infants born before arrival (BBA) at health facilities and infants born outside health facilities	EXCLUSIONS	None	COLLECTION POINTS	Delivery
	USE AND CONTEXT	Live births should only be counted when the foetus is of 26 or more weeks' gestational age and/or weighs 500g or more. Live births of women with unknown HIV status at delivery and tested during or after delivery and found to be HIV-positive should also be counted.	FREQUENCY	Monthly	TOOLS	Delivery Register
DEATH IN FACILITY 0–7 DAYS	DEFINITION	The death of a live-born infant within 7 completed days after birth is known as an early neonatal death.	INCLUSIONS	» INCLUDE only early neonatal deaths when the foetus is of 26 or more weeks' gestational age and/or weighs 500g or more » INCLUDE all infants born in the facility who die within 7 completed days after birth, even if they are not admitted separate from their mothers.	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The neonatal period is exactly 4 weeks or 28 completed days, with the first 7 completed days called the early neonatal period.	EXCLUSIONS	» EXCLUDE infants who died outside the facility (e.g. dead on arrival) » EXCLUDE stillbirths	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Midnight Census

Death in facility 8–28 days	DEFINITION	The death of a live-born infant between 8 and 28 completed days after birth is known as a late neonatal death.	INCLUSIONS	» INCLUDE infants who died inside a health facility	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The neonatal period is exactly 4 weeks or 28 completed days, with the period from 8–28 completed days called the late neonatal period.	EXCLUSIONS	» EXCLUDE infants who died outside the facility (e.g. dead on arrival)	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Midnight Census
Infant given Nevirapine within 72 hours after birth	DEFINITION	Infant born to HIV-positive woman who received Nevirapine within 72 hours after birth	INCLUSIONS	» INCLUDE BBAs	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Also count infants not delivered in health facilities (BBAs and known home deliveries) who were given Nevirapine within 72 hours after birth	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register OPD; Delivery Register; PHC Comprehensive Tick Register
Infant initiated on CPT around 6 weeks	DEFINITION	Infants born to HIV-positive women who were initiated on Co-trimoxazole around 6 weeks after birth to prevent opportunistic infections	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Health facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
Infant PCR test at birth	DEFINITION	Infants born to HIV-positive mothers who are PCR-tested at birth	INCLUSIONS	» INCLUDE first PCR tests done before 6 weeks after birth	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Ideally HIV-exposed infants must be tested within the first 7 days after birth. In cases where infants have been missed, any test done before 6 weeks is still counted as a birth PCR test.	EXCLUSIONS	» EXCLUDE confirmatory PCR tests done	COLLECTION POINTS	Delivery
	USE AND CONTEXT	This data element will fast track diagnosis of HIV exposed infants and initiation into treatment as recommended by the latest National Consolidated Guidelines for PMTCT and management of HIV in children, adolescents and adults.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Delivery Register

INFANT PCR TEST POSITIVE AT BIRTH	**DEFINITION**	Infants born to HIV-positive mothers who tested PCR-positive at birth	**INCLUSIONS**	» INCLUDE PCR-positive results from first PCR tests done before 6 weeks after birth	**COLLECTED BY**	Clinician
	EXTENDED DEFINITION	Ideally HIV-exposed infants must be tested within the first 7 days after birth. In cases where infants have been missed, positive results on tests done before 6 weeks are still counted as PCR-positive tests at birth.	**EXCLUSIONS**	» EXCLUDE confirmatory PCR-positive results	**COLLECTION POINTS**	Delivery
	USE AND CONTEXT	Infants who tested positive need to be linked to care to ensure that they are initiated on treatment as per the latest National consolidated HIV policy guidelines.	**FREQUENCY**	Monthly	**TOOLS**	PHC Comprehensive Tick Register; Delivery Register
INFANT PCR TEST AROUND 10 WEEKS	**DEFINITION**	Infants born to HIV-positive woman who are PCR tested around 10 weeks	**INCLUSIONS**	» INCLUDE PCR tests done from 6 to 12 weeks after birth	**COLLECTED BY**	Clinician
	EXTENDED DEFINITION	All PCR tests done on HIV-exposed infants from 6 to 12 weeks after birth can be counted.	**EXCLUSIONS**	» EXCLUDE confirmatory PCR tests done » EXCLUDE tests for infants who had positive results in the birth test	**COLLECTION POINTS**	All facilities
	USE AND CONTEXT	Until May 2015, the PCR test was only done around 6 weeks. The criteria for PCR testing were revised to test infants at birth with a follow-up test around 10 weeks on infants who tested negative at birth. PCR tests done around 10 weeks allow sufficient time to test HIV-exposed infants as a follow-up test from birth. The time period extension will ensure that infants on a 12-week course of Nevirapine can be counted when tested at 18 weeks.	**FREQUENCY**	Monthly	**TOOLS**	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net
INFANT PCR TEST POSITIVE AROUND 10 WEEKS	**DEFINITION**	Infants born to HIV-positive woman with PCR-positive results around 10 weeks	**INCLUSIONS**	» INCLUDE PCR-positive results from PCR tests done from 6 to 12 weeks after birth	**COLLECTED BY**	Clinician
	EXTENDED DEFINITION	All PCR-positive results on tests done on HIV-exposed infants from 6 to 12 weeks after birth can be counted.	**EXCLUSIONS**	» EXCLUDE confirmatory PCR positive results	**COLLECTION POINTS**	All facilities
	USE AND CONTEXT	Infants who tested positive need to be linked to care to ensure they are initiated on treatment as per the latest National consolidated HIV policy guidelines.	**FREQUENCY**	Monthly	**TOOLS**	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net

HIV TEST AROUND 18 MONTHS	DEFINITION	Children born to HIV-positive women who were tested for HIV antibodies around 18 months after birth	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE ALL children who previously tested HIV-positive and on ART	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Count ONLY once on the day the HIV test was done.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net

HIV TEST POSITIVE AROUND 18 MONTHS	DEFINITION	Children born to HIV-positive women who tested positive for HIV antibodies around 18 months after birth	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Count ONLY once on the day the HIV test was done.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; HTS Register (HIV Testing Services) or HCT module in TIER.Net

MENTAL HEALTH CLIENTS UNDER 18 YEARS *	DEFINITION	All clients under 18 years who attended ambulatory (non-inpatient) services for mental health conditions	INCLUSIONS	None	COLLECTED BY	Clinician including mental health care practitioners
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	PHC and OPD
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
MENTAL HEALTH CLIENT 18 YEARS AND OLDER *	DEFINITION	All clients 18 years and older who attended ambulatory (non-inpatient) services for mental health conditions	INCLUSIONS	None	COLLECTED BY	Clinician including mental health care practitioners
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	PHC and OPD
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
MENTAL HEALTH ADMISSION INVOLUNTARY *	DEFINITION	Client admitted for mental health conditions in terms of section 33 of the Mental Health Care Act (17 of 2002)	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	These are clients who are incapable of making informed decisions due to their mental health status. The data elements should be collected at all hospitals with designated psychiatric beds or wards.	EXCLUSIONS	» EXCLUDE voluntary and assisted admissions	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Exclude voluntary and assisted admissions for mental health conditions. Typical examples are mood disorders, anxiety, post-traumatic stress disorder, schizophrenia, organic brain disease, dementia, substance abuse disorders, psychosis, mental handicap.	FREQUENCY	Monthly	TOOLS	Midnight Census
MENTAL HEALTH SEPARATIONS UNDER 18 YEARS *	DEFINITION	Client under 18 years admitted for mental health conditions	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Midnight Census
MENTAL HEALTH SEPARATIONS 18 YEARS AND OLDER *	DEFINITION	Client 18 years and older admitted for mental health conditions	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Midnight Census

PHC CLIENT SCREENED FOR MENTAL DISORDERS *						
	DEFINITION	Client of all ages screened for mental disorders	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	PHC and OPD
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD

Dental headcount – total	DEFINITION	All clients attending the facility who received dental or oral health services	INCLUSIONS	» INCLUDE clients seen at a facility by a doctor or Professional Nurse with a tooth abscess	COLLECTED BY	Oral Health
	EXTENDED DEFINITION	Count the client only once for each day they appear at the facility.	EXCLUSIONS	None	COLLECTION POINTS	Oral Health
	USE AND CONTEXT	Include any clients given individual services, e.g. at a school or crèche visit. These services must then be reported against the nearest health facility with a comment that the service was rendered outside the health facility and the data collection tools must be sent to the health facility for recordkeeping.	FREQUENCY	Monthly	TOOLS	Tick Register Oral Health
Tooth extraction	DEFINITION	The actual number of teeth extracted by a health worker	INCLUSIONS	» INCLUDE tooth extracted by a doctor in the absence of an oral health worker » INCLUDE tooth extractions done in theatres at hospitals	COLLECTED BY	Oral Health
	EXTENDED DEFINITION	Count each tooth that is extracted, not the number of clients seen.	EXCLUSIONS	None	COLLECTION POINTS	Oral Health
	USE AND CONTEXT	Count the number of teeth extracted, NOT the number of clients.	FREQUENCY	Monthly	TOOLS	Tick Register Oral Health
Tooth restoration	DEFINITION	The actual number of teeth restored by a health worker	INCLUSIONS	» INCLUDE tooth restorations done in theatres at hospitals	COLLECTED BY	Oral Health
	EXTENDED DEFINITION	Count each tooth that is restored, not the number of clients seen.	EXCLUSIONS	None	COLLECTION POINTS	Oral Health
	USE AND CONTEXT	Count the number of teeth restored, NOT the number of clients.	FREQUENCY	Monthly	TOOLS	Tick Register Oral Health
Tooth fissure sealant 1st or 2nd permanent molar (child)	DEFINITION	Children (normally 6 and 12 years old) who received fissure sealant applications on their first and second permanent molar teeth by a health worker	INCLUSIONS	» INCLUDE tooth fissure sealant applied in children of other ages. Applications are normally needed at age 6 and 12, but can also occur before or after.	COLLECTED BY	Oral Health
	EXTENDED DEFINITION	Count the number of children and not tooth fissure sealant applications.	EXCLUSIONS	None	COLLECTION POINTS	Oral Health
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Tick Register Oral Health

OHH HEADCOUNT UNDER 5 YEARS	DEFINITION	Children under 5 years to whom care was provided by Ward-based PHC Outreach household teams	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Household (HH)
	USE AND CONTEXT	The community health worker (CHW) must enter the total number of children under 5 years of age to whom support was provided in each household visited, on the CHW Household Visit form.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH HEADCOUNT 5 YEARS AND OLDER	DEFINITION	Clients 5 years and older to whom care was provided by Ward-based PHC Outreach household teams	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	The community health worker (CHW) must enter the total number of clients 5 years and older to whom support was provided in each household visited, on the CHW Household Visit form.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH REGISTRATION VISIT	DEFINITION	Outreach households visited by Ward-based PHC Outreach household teams to complete a Household Registration form	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Each household allocated to a community health worker (CHW) must be registered during the first outreach household visit, using the standardised Household Registration form.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH ALLOCATED TO TEAM	DEFINITION	Households allocated to Ward-based PHC Outreach household teams	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Each PHC outreach household team is allocated a number of households to support.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH FOLLOW-UP VISIT	DEFINITION	Outreach households visited to provide follow-up health care support	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Community health workers (CHW) are expected to visit each household for follow-up support monthly. At-risk households should be visited more frequently.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH SUPERVISED VISIT	DEFINITION	Ward-based PHC Outreach household team visits accompanied by a supervisor	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Monitors supervision of ward-based community health workers (CHW)	FREQUENCY	Monthly	TOOLS	WBOT Register

OHH WITH PREGNANCY CARE	DEFINITION	PHC outreach household (OHH) visit where care was provided to pregnant women	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Do not count individual pregnant women.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH WITH POSTNATAL CARE	DEFINITION	PHC outreach household (OHH) visit where care was provided to woman and/or new-born baby within 6 days after delivery	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Do not count individual postnatal women.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH WITH CHILD UNDER 5 YEARS CARE	DEFINITION	Outreach household (OHH) visit where care was provided to child(ren) under 5 years of age	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Do not count individual children.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH WITH ADHERENCE SUPPORT	DEFINITION	Outreach household (OHH) visit where adherence support was provided	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Do not count individual clients.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH WITH HOME-BASED CARE	DEFINITION	Outreach household (OHH) visit where home-based care was provided	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Do not count individual clients.	FREQUENCY	Monthly	TOOLS	WBOT Register
OHH CLIENT REFER TO FACILITY	DEFINITION	Clients of all ages referred to health facilities by Ward-based PHC Outreach household teams	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Monitors referral to health facilities. Count individual clients.	FREQUENCY	Monthly	TOOLS	WBOT Register

OHH CLIENT REFER TO SOCIAL SERVICES	DEFINITION	A client who was referred to social services by a CHW during an household outreach visit	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Monitors referral to social services. Count individual clients.	FREQUENCY	Monthly	TOOLS	WBOT Register

OHH CLIENT REFER TO HOME-BASED CARE	DEFINITION	A client who was referred to home-based care by a CHW during an household outreach visit	INCLUSIONS	None	COLLECTED BY	WBOT
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	HH
	USE AND CONTEXT	Monitors referral to home-based care. Count individual clients.	FREQUENCY	Monthly	TOOLS	WBOT Register

OHH HOUSEHOLDS IN POPULATION	DEFINITION	Households in the population that should be visited and registered by Ward-based Outreach Teams	INCLUSIONS	None	COLLECTED BY	StatsSA
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Census
	USE AND CONTEXT	Monitors implementation of PHC re-engineering in terms of Ward-based Outreach Teams. Will be auto-calculated when StatsSA releases break-downs of 2011 Census data (the Small Area Layer)	FREQUENCY	Yearly	TOOLS	None

Complaint resolved within 25 working days *	DEFINITION	Total number of complaints resolved during the reporting month within 25 working days from the complaint logged	INCLUSIONS	» INCLUDE complaints logged by family members on behalf of clients	COLLECTED BY	Manager
	EXTENDED DEFINITION	Although all complaints should be resolved, count only those that were resolved within 25 days (even if reported the previous month).	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	This is a subset of complaints resolved.	FREQUENCY	Monthly	TOOLS	Complaints Register
Complaints received *	DEFINITION	Total number of complaints received from clients	INCLUSIONS	» INCLUDE complaints logged by family members on behalf of clients	COLLECTED BY	Manager
	EXTENDED DEFINITION	Count the number of written complaints as well as the verbal complaints transcribed by staff members or other clients	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Complaints Register
Complaints resolved *	DEFINITION	Total number of complaints resolved	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	Complaints register
Hospital achieved 75% and more on National Core Standards self-assessment *	DEFINITION	Number of hospitals that achieved 75% or more on National Core Standards self-assessment	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	The printed plan should be filed as evidence.	EXCLUSIONS	None	COLLECTION POINTS	Hospitals
	USE AND CONTEXT	This data element will always be a 0 (if 75% is not achieved) or a 1 (if 75% or more is achieved). No blanks are allowed in the input form, nor in the data-capture screen.	FREQUENCY	Monthly	TOOLS	Management

Ideal Clinic (IC) *	DEFINITION	Fixed PHC health facilities that have Ideal Clinic status	INCLUSIONS	None	COLLECTED BY	Facility Manager
	EXTENDED DEFINITION	The self-assessment must be captured on the Ideal Clinic dashboard software.	EXCLUSIONS	None	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Monitors whether PHC health establishments are measuring their own level of compliance with standards in order to close gaps in preparation for an external assessment by the Office of Health Standards Compliance	FREQUENCY	Monthly	TOOLS	Ideal Clinic Monitoring Tools
Ideal Clinic status determinations conducted by PPTICRM (fixed clinic/CHC/CDC) *	DEFINITION	Number of Ideal Clinic status determinations conducted by PPTICRM according to the Ideal Clinic manual	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	A determination conducted according to the Ideal Clinic manual. Record 1 (one) regardless of the number of determinations conducted during the reporting period and 0 (zero) if no determinations were conducted. Do not leave this field blank.	EXCLUSIONS	None	COLLECTION POINTS	Fixed clinic/CHC/CDC
	USE AND CONTEXT	Monitors whether PHC health establishments are measuring their own level of compliance with standards in order to close gaps in preparation for an external assessment by the Office of Health Standards Compliance	FREQUENCY	Monthly	TOOLS	Management
Facility Patient Experience of Care Survey done *	DEFINITION	Number of PHC fixed facilities (Clinic/CHC/CDC) and Hospitals (public) that have conducted a Patient Experience of Care Survey	INCLUSIONS	None	COLLECTED BY	DoH
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Fixed facilities
	USE AND CONTEXT	Monitors the quality of care as experienced by patients	FREQUENCY	Yearly	TOOLS	Questionnaires
Patient Experience of Care satisfied responses *	DEFINITION	Number of patient responses that indicated satisfaction with experience of care	INCLUSIONS	None	COLLECTED BY	DoH
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Fixed facilities
	USE AND CONTEXT	Monitors the quality of care as experienced by patients	FREQUENCY	Yearly	TOOLS	Questionnaires

PATIENT EXPERIENCE OF CARE TOTAL RESPONSES *	DEFINITION	Patient Safety Incident (PSI) case closed within the reporting month	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	A patient safety incident is an event or circumstance that could have resulted, or did result, in unnecessary harm to a patient. The use of the word 'unnecessary' in this definition recognises that errors, violation, patient abuse and deliberately unsafe acts occur in health care. These are considered incidents. **Incident:** An incident can be a reportable circumstance, near miss, no-harm incident or harmful incident (adverse event). **Reportable circumstance:** a situation in which there was significant potential for harm, but no incident occurred. **Near miss:** an incident which did not reach the patient. **No-harm incident:** is one in which an event reached a patient but no discernible harm resulted. **Harmful incident:** is an incident that results in harm to a patient. **Adverse event:** harm related to medical management, in contrast to disease complications or underlying disease. Medical management includes all aspects of care from interaction with health care provider to discharge of a patient from medical treatment or health care facility.	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents	FREQUENCY	Yearly	TOOLS	Patient Safety Incident Register

PATIENT SAFETY INCIDENT (PSI) CASE REPORTED *	DEFINITION	Patient Safety Incident (PSI) case reported	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	Patient safety incident: is an event or circumstance that could have resulted, or did result, in unnecessary harm to a patient. The use of the word 'unnecessary' in this definition recognizes that errors, violation, patient abuse and deliberately unsafe acts occur in health care. These are considered incidents. **Incident:** An incident can be a reportable circumstance, near miss, no harm incident or harmful incident (adverse event). **Reportable circumstance:** is a situation in which there was significant potential for harm, but no incident occurred. **Near miss:** an incident which did not reach the patient. **No-harm incident:** in which an event reached a patient but no discernible harm resulted. **Harmful incident:** an incident that results in harm to a patient. **Adverse event:** harm related to medical management, in contrast to disease complications or underlying disease. Medical management includes all aspects of care from interaction with healthcare provider to discharge of a patient from medical treatment or healthcare facility.	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents	FREQUENCY	Yearly	TOOLS	Patient Safety Incident Register

Patient Safety Incident (PSI) case closed *	DEFINITION	Patient Safety Incident (PSI) case closed within the reporting month	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	Patient safety incident: is an event or circumstance that could have resulted, or did result, in unnecessary harm to a patient. The use of the word 'unnecessary' in this definition recognises that errors, violation, patient abuse and deliberately unsafe acts occur in health care. These are considered incidents. Incident: An incident can be a reportable circumstance, a near miss, a no-harm incident or a harmful incident (adverse event). Reportable circumstance: a situation in which there was significant potential for harm, but no incident occurred. Near miss: an incident which did not reach the patient. No-harm incident: an event reached a patient but no discernible harm resulted. Harmful incident: an incident that results in harm to a patient. Adverse event: harm related to medical management, in contrast to disease complications or underlying disease. Medical management includes all aspects of care from interaction with a healthcare provider to discharge of a patient from medical treatment or health care facility.	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents	FREQUENCY	Monthly	TOOLS	Patient Safety Incident Register
Severity assessment code (SAC) 1 incidents reported within 24 hours *	DEFINITION	Severity assessment code (SAC) 1 incidents reported within 24 hours after the occurrence of the incident. This allows for speedy follow-up and intervention after the incident.	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	SAC 1 include incidents where serious harm or death occurred. SAC1 incidents are classified according to the National Policy to Manage Patient Safety Incidents (Annexure E)	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents	FREQUENCY	Monthly	TOOLS	Patient Safety Incident Register
Severity assessment code (SAC) 1 incident reported *	DEFINITION	SAC1 incident reported within the reporting month	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	SAC1 include incidents where serious harm or death occurred. SAC1 incidents are classified according to the National Policy to Manage Patient Safety Incidents (Annexure E)	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents	FREQUENCY	Monthly	TOOLS	Patient Safety Incident Register

Patient Safety Incident (PSI) case closed within 60 days *						
	DEFINITION	Patient Safety Incident (PSI) case closed within 60 days after being reported	INCLUSIONS	None	COLLECTED BY	Manager
	EXTENDED DEFINITION	SAC1 include incidents where serious harm or death occurred. SAC1 incidents are classified according to the National Policy to Manage Patient Safety Incidents (Annexure E)	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents	FREQUENCY	Monthly	TOOLS	Patient Safety Incident Register

Wheelchair required	DEFINITION	All wheelchair requests received at the facility	INCLUSIONS	» INCLUDE all types of chairs, self-propelled or pushed and buggies for children	COLLECTED BY	Rehab staff
	EXTENDED DEFINITION	Count all wheelchair requests received by the facility.	EXCLUSIONS	None	COLLECTION POINTS	CHCs and hospitals
	USE AND CONTEXT	Also recorded as clients on register for requiring wheelchairs	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
Wheelchair issued	DEFINITION	Wheelchair issued to a client in need of a wheelchair	INCLUSIONS	» INCLUDE all types of chairs, self-propelled or pushed and buggies for children	COLLECTED BY	Rehab staff
	EXTENDED DEFINITION	Count all the wheelchairs issued by the facility, to new clients or as replacement, new or refurbished	EXCLUSIONS	» EXCLUDE replacements of existing wheelchairs due to them being broken or not functioning as expected	COLLECTION POINTS	CHCs and hospitals
	USE AND CONTEXT	These data must be collected by the facility that issues the wheelchair to the client – which may not necessarily be the facility that is ordering the wheelchairs.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
Hearing aid issued	DEFINITION	New hearing aid issued to patients	INCLUSIONS	» All hearing aids issued should be recorded, new or replacement	COLLECTED BY	Rehab staff
	EXTENDED DEFINITION	Count all hearing aids issued to all clients who need them, whether new or replacement. All issued devices have budget implications.	EXCLUSIONS	None	COLLECTION POINTS	PHC Comprehensive Tick Register; Tick Register OPD
	USE AND CONTEXT	Access to hearing aids	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
Hearing aid required	DEFINITION	All hearing aids required	INCLUSIONS	» All hearing aids required should be recorded.	COLLECTED BY	Rehab staff
	EXTENDED DEFINITION	Count all clients who are in need of hearing aids on register of requests.	EXCLUSIONS	None	COLLECTION POINTS	PHC Comprehensive Tick Register; Tick Register OPD
	USE AND CONTEXT	Access to hearing aids	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD

SCHOOL GRADE R – LEARNERS SCREENED	DEFINITION	Grade R learners in the school screened by a nurse in line with the ISHP service package	INCLUSIONS	» Quintile 1 (Q1) to Quintile 5 (Q5)	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE private schools and independent schools	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical, emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
SCHOOL GRADE 1 – LEARNERS SCREENED	DEFINITION	Grade 1 learners in the school screened by a nurse in line with the ISHP service package	INCLUSIONS	» Quintile 1 (Q1) to Quintile 5 (Q5)	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE private schools and independent schools	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical, emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health

School Grade 4 – Learners screened	DEFINITION	Grade 4 learners in the school screened by a nurse in line with the ISHP service package	INCLUSIONS	» Quintile 1 (Q1) to Quintile 5 (Q5)	COLLECTED BY	School Health
	EXTENDED DEFINITION	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments.	EXCLUSIONS	» EXCLUDE private schools and independent schools	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical, emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
School Grade 8 – Learners screened	DEFINITION	Grade 8 learners in the school screened by a nurse in line with the ISHP service package	INCLUSIONS	» Quintile 1 (Q1) to Quintile 5 (Q5)	COLLECTED BY	School Health
	EXTENDED DEFINITION	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments.	EXCLUSIONS	» EXCLUDE private schools and independent schools	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical, emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health

SCHOOL GRADE 10 – LEARNERS SCREENED	DEFINITION	Grade 10 learners in the school screened by a nurse in line with the ISHP service package	INCLUSIONS	» Quintile 1 (Q1) to Quintile 5 (Q5)	COLLECTED BY	School Health
	EXTENDED DEFINITION	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments.	EXCLUSIONS	» EXCLUDE private schools and independent schools	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical, emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
SCHOOL GRADE OTHER – LEARNERS SCREENED	DEFINITION	Other Grade learners in the school screened by a nurse in line with the ISHP service package (not in Grade s1/4/8/10). This excludes follow-up visits and referrals, and each learner should be counted only once per school year.	INCLUSIONS	» Quintile 1 (Q1) to Quintile 5 (Q5)	COLLECTED BY	School Health
	EXTENDED DEFINITION	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments.	EXCLUSIONS	» EXCLUDE private schools and independent schools	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical, emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health

School learner underweight	DEFINITION	A learner with weight diagnosed below the -2 SD line but above the -3 SD line for the first time	INCLUSIONS	None	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical and emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
School learner overweight	DEFINITION	A learner with weight recorded above the +2 SD line for the first time	INCLUSIONS	None	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical and emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / nofee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
School learner referred for Oral Health	DEFINITION	A learner referred for oral health issues	INCLUSIONS	None	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical and emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health

SCHOOL LEARNER REFERRED FOR EYE CARE	DEFINITION	A learner referred for eye care	INCLUSIONS	None	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical and emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
SCHOOL LEARNER REFERRED FOR HEARING PROBLEMS	DEFINITION	A learner referred for hearing problems	INCLUSIONS	None	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical and emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
SCHOOL LEARNER REFERRED FOR SPEECH PROBLEMS	DEFINITION	A learner referred for speech problems	INCLUSIONS	None	COLLECTED BY	School Health
	EXTENDED DEFINITION	The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	EXCLUSIONS	None	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical and emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health

School learner referred for suspected TB	DEFINITION	A learner referred for suspected TB	INCLUSIONS	None	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical and emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
School learner given deworming	DEFINITION	A learner given deworming tablets	INCLUSIONS	» INCLUDE deworming medication given routinely » INCLUDE deworming medication given during a provincial campaign	COLLECTED BY	School Health
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE deworming medication given during a national campaign	COLLECTION POINTS	Schools
	USE AND CONTEXT	Health screening includes oral health, vision, hearing, speech, height and weight, physical assessment, mental health, tuberculosis, chronic illnesses, and psychosocial support. On-site services include deworming, immunisation, oral health, and minor ailments. Health education includes hand-washing, personal and environmental hygiene, nutrition, tuberculosis, road safety, poisoning, 'know your body', and abuse (sexual, physical and emotional). Quintile 1 (Q1) and Quintile 2 (Q2) schools are the poorest / no-fee schools. The Integrated School Health Programme (ISHP) includes health screening, on-site services, and health education.	FREQUENCY	Monthly	TOOLS	Tick Register School Health
School learners total	DEFINITION	Total learners in the school	INCLUSIONS	None	COLLECTED BY	DoE
	EXTENDED DEFINITION	None	EXCLUSIONS	None	COLLECTION POINTS	None
	USE AND CONTEXT	Planning and monitoring. Schools including their quintile classification are imported from the Department of Basic Education (DBE) database into the DHIS, or if not available, from the provincial/local DBE	FREQUENCY	Yearly	TOOLS	None

School Grade 1 – Learners Total	**DEFINITION**	Grade 1 learners in the school	**INCLUSIONS**	None	**COLLECTED BY**	DoE
	EXTENDED DEFINITION	None	**EXCLUSIONS**	None	**COLLECTION POINTS**	None
	USE AND CONTEXT	Planning and monitoring. Schools including their quintile classification are imported from the Department of Basic Education (DBE) database into the DHIS, or if not available, from the provincial/local DBE	**FREQUENCY**	Yearly	**TOOLS**	None
School Grade 4 – Learners Total	**DEFINITION**	Grade 4 learners in the school	**INCLUSIONS**	None	**COLLECTED BY**	DoE
	EXTENDED DEFINITION	None	**EXCLUSIONS**	None	**COLLECTION POINTS**	None
	USE AND CONTEXT	Planning and monitoring. Schools including their quintile classification are imported from the Department of Basic Education (DBE) database into the DHIS, or if not available, from the provincial/local DBE	**FREQUENCY**	Yearly	**TOOLS**	None
School Grade 8 – Learners Total	**DEFINITION**	Grade 8 learners in the school	**INCLUSIONS**	None	**COLLECTED BY**	DoE
	EXTENDED DEFINITION	None	**EXCLUSIONS**	None	**COLLECTION POINTS**	None
	USE AND CONTEXT	Planning and monitoring. Schools including their quintile classification are imported from the Department of Basic Education (DBE) database into the DHIS, or if not available, from the provincial/local DBE	**FREQUENCY**	Yearly	**TOOLS**	None
School Grade 10 – Learners Total	**DEFINITION**	Grade 10 learners in the school	**INCLUSIONS**	None	**COLLECTED BY**	DoE
	EXTENDED DEFINITION	None	**EXCLUSIONS**	None	**COLLECTION POINTS**	None
	USE AND CONTEXT	Planning and monitoring. Schools including their quintile classification are imported from the Department of Basic Education (DBE) database into the DHIS, or if not available, from the provincial/local DBE	**FREQUENCY**	Yearly	**TOOLS**	None

MALE URETHRITIS SYNDROME TREATED – NEW EPISODES						
	DEFINITION	A new episode of Male urethritis syndrome (MUS)	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The data element counts the episode, not the client.	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD

Screen for TB symptoms under 5 years	DEFINITION	Children under 5 years who were screened in health facilities using the symptom screening tool	INCLUSIONS	» INCLUDE HIV-positive and HIV-negative clients » INCLUDE contacts » INCLUDE neonates	COLLECTED BY	Clinician, Data Capturer, CHW
	EXTENDED DEFINITION	Count each child once for each day they are seen at the facility. Screening should be done once regardless of the number of services provided on that day.	EXCLUSIONS	» Children on TB treatment » Children on IPT » Children screened outside health facilities	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
Screen for TB symptoms 5 years and older	DEFINITION	Clients 5 years and older who were screened in health facilities using the TB symptom screening tool	INCLUSIONS	» INCLUDE HIV-positive and HIV-negative clients » INCLUDE miners » INCLUDE diabetic clients » INCLUDE contacts » INCLUDE pregnant women » INCLUDE neonates	COLLECTED BY	Clinician; Data Capturer, CHW
	EXTENDED DEFINITION	each person once for each day they are seen at the facility. Screening should be done once regardless of the number of services provided on that day.	EXCLUSIONS	» Clients already on TB treatment. » EXCLUDE clients screened outside health facilities	COLLECTION POINTS	All facilities, communities, ISHP
	USE AND CONTEXT	None	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
TB symptomatic child under 5 years	DEFINITION	Client under 5 years with a positive symptoms screen	INCLUSIONS	» INCLUDE children referred to the next level of care for investigations	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Positive symptom screen is defined as the presence of one or more of the symptoms in the screening tool.	EXCLUSIONS	» EXCLUDE clients screened outside health facilities	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Count each child once for each day they are seen at the facility. Screening should be done once regardless of the number of services.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register

TB SYMPTOMATIC CLIENT 5 YEARS AND OLDER	DEFINITION	Client 5 years and older with a positive symptoms screen	INCLUSIONS	» INCLUDE children referred to the next level of care for investigations	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Positive symptom screen is defined as the presence of one or more of the symptoms in the screening tool.	EXCLUSIONS	» EXCLUDE clients screened outside health facilities	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Count each person once for each day they are seen at the facility. Screening should be done once regardless of the number of services provided on that day.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
TB SYMPTOMATIC CLIENT 5 YEARS AND OLDER WITH SPUTUM SENT	DEFINITION	TB symptomatic client 5 years and older who had sputum collected and sent for TB testing	INCLUSIONS	» INCLUDE all where gastric aspirates / lavage / sputum induction was done	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Count each client once, regardless of the number of sputa samples sent for TB testing.	EXCLUSIONS	» EXCLUDE follow-up sputum tests while client is on TB treatment	COLLECTION POINTS	All facilities
	USE AND CONTEXT	TB must be excluded in all those who have a cough.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
TB SYMPTOMATIC CLIENT 5 YEARS AND OLDER TEST POSITIVE	DEFINITION	Any client 5 years and older who had at least one sputum specimen sent for TB testing and the result was positive	INCLUSIONS	» INCLUDE rifampicin-susceptible TB	COLLECTED BY	Clinician, Data
	EXTENDED DEFINITION	Positive test is when *Mycobacterium TB* is detected. This could be rifampicin-resistant or rifampicin-susceptible. Count each client only once.	EXCLUSIONS	» EXCLUDE rifampicin-resistant TB results	COLLECTION POINTS	All facilities
	USE AND CONTEXT	The National TB Guidelines must be considered.	FREQUENCY	Monthly	TOOLS	TB identification (suspect) register/ TIER.Net
TB CLIENT 5 YEARS AND OLDER START ON TREATMENT	DEFINITION	Any client 5 years and older who after being tested for TB is confirmed as a pulmonary TB and is starting treatment. It can be a new or a retreatment case.	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Count each case only once.	EXCLUSIONS	None	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	The client must also be entered into the electronic TB register or TIER.Net. The TB Guidelines should be followed in determining clients who are eligible for starting TB treatment.	FREQUENCY	Monthly	TOOLS	TB identification (suspect) register/ TIER.Net

TB CLIENT UNDER 5 YEARS START ON TREATMENT	DEFINITION	Any client under 5 years who – based on clinical, bacteriological, or other investigations – is started on TB treatment.	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Count each case only once.	EXCLUSIONS	» EXCLUDE children started on Isoniazid preventive therapy	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	The client must also be entered into the electronic TB register or TIER.Net. The TB Guidelines should be followed in determining clients who are eligible for starting TB treatment.	FREQUENCY	Monthly	TOOLS	TB identification (suspect) register/ TIER.Net
TB CONTACT UNDER 5 YEARS START ON IPT	DEFINITION	Contact under 5 years who – based on negative symptom screen – is started on Isoniazid preventive therapy (IPT)	INCLUSIONS	» INCLUDE HIV-positive » INCLUDE HIV-negative » INCLUDE HIV unknown » INCLUDE contacts of DS-TB and DR-TB clients	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Negative symptom screen is defined as the absence of all the symptoms in the screening tool; all these contacts must be started on treatment to prevent TB disease.	EXCLUSIONS	» EXCLUDE clients already on IPT » EXCLUDE clients on TB treatment	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Count each case only once.	FREQUENCY	Monthly	TOOLS	TB identification (suspect) register/ TIER.Net
TB ASYMPTOMATIC CONTACT UNDER 5 YEARS	DEFINITION	Contact under 5 years with a negative symptom screen	INCLUSIONS	» INCLUDE HIV-positive » INCLUDE HIV-negative » INCLUDE HIV unknown » INCLUDE contacts of DS-TB and DR-TB clients	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Negative symptom screen is defined as the absence of all the symptoms in the screening tool.	EXCLUSIONS	» EXCLUDE clients already on IPT » EXCLUDE clients on TB treatment	COLLECTION POINTS	PHC facility
	USE AND CONTEXT	Count each case only once.	FREQUENCY	Monthly	TOOLS	TB identification (suspect) register/ TIER.Net

TB CLIENT START ON TREATMENT	DEFINITION	All clients started on TB treatment during the reporting period	INCLUSIONS	» INCLUDE all client categories (new, retreatment, other) » INCLUDE all disease classifications (pulmonary, extra-pulmonary)	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	All clients for whom a decision to treat for TB was made based on bacteriological or clinical diagnosis	EXCLUSIONS	» EXCLUDE MDR-TB clients » EXCLUDE clients started on IPT	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
TB CLIENT LOST TO FOLLOW-UP	DEFINITION	All TB clients who were initiated on treatment but were lost to follow-up during TB treatment	INCLUSIONS	» INCLUDE all client categories (new, retreatment, other) » INCLUDE all disease classifications (pulmonary, extra-pulmonary)	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	'Lost to follow-up' is defined as interruption of treatment for a period of two consecutive months or longer at any time during the treatment period.	EXCLUSIONS	» EXCLUDE clients who are known to have died during TB treatment » EXCLUDE clients who were transferred out and treatment outcome is not known	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Cohort of clients initiated on treatment at least 6 months prior	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
TB CLIENT SUCCESSFULLY COMPLETED TREATMENT	DEFINITION	All TB clients who completed treatment; this includes clients who were cured and those without proof of cure at the end of the treatment period	INCLUSIONS	» INCLUDE all client categories (new, retreatment, other) » INCLUDE all disease classifications (pulmonary, extra-pulmonary)	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Count of all clients (new and retreatment, pulmonary and extra-pulmonary) who were cured and those who completed treatment.	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Cohort of clients started on treatment at least 6 months prior	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
TB CLIENT DEATH	DEFINITION	All TB clients started on treatment but died during the treatment period	INCLUSIONS	» INCLUDE all client categories (new, retreatment, other) » INCLUDE all disease classifications (pulmonary, extra-pulmonary); » INCLUDE deaths that occurred in health facilities and in the community	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	The cause of death may not necessarily be due to TB but death occurred during TB treatment from any other cause	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Cohort of clients started on treatment at least 6 months prior	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net

NEW PULMONARY TB START TREATMENT	DEFINITION	New pulmonary tuberculosis (PTB) clients started on treatment	INCLUSIONS	» INCLUDE all new pulmonary TB clients, irrespective of smear results	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	New clients who have never been on TB treatment or have taken antituberculosis drugs for less than 4 weeks	EXCLUSIONS	» EXCLUDE retreatment TB cases » EXCLUDE extra-pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
NEW EXTRA-PULMONARY TB START TREATMENT	DEFINITION	New extra-pulmonary TB client started on treatment	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	New clients who have never been on TB treatment or have taken antituberculosis drugs for less than 4 weeks	EXCLUSIONS	» EXCLUDE retreatment TB cases » EXCLUDE pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
RETREATMENT PULMONARY TB START AFTER RELAPSE	DEFINITION	Pulmonary TB client started on TB retreatment who was previously taking TB treatment for 4 weeks or more and relapsed	INCLUSIONS	» INCLUDE clients who have taken TB treatment for 4 weeks or more in the past and relapsed	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	A relapsed client who received treatment and was declared cured or treatment completed at the end of the treatment period and developed TB again. These patients could be true relapses or have a new episode.	EXCLUSIONS	» EXCLUDE extra-pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
RETREATMENT PULMONARY TB START AFTER DEFAULT	DEFINITION	After default pulmonary TB client started on treatment	INCLUSIONS	» INCLUDE clients who have taken TB treatment for 4 weeks or more in the past and defaulted	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	A default client who completed at least one month of treatment and returned after interrupting treatment for two months or more	EXCLUSIONS	» EXCLUDE extra-pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net

RETREATMENT PULMONARY TB START AFTER FAILURE	DEFINITION	After failure pulmonary TB client started on treatment	INCLUSIONS	» INCLUDE clients who have taken TB treatment for 4 weeks or more in the past and failed on treatment	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	A failed client who received treatment and remained or became smear- or culture-positive at the end of the treatment period	EXCLUSIONS	» EXCLUDE extra-pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
RETREATMENT EXTRA-PULMONARY TB START AFTER RELAPSE	DEFINITION	Relapsed extra-pulmonary TB client started on treatment	INCLUSIONS	» INCLUDE clients who have taken TB treatment for 4 weeks or more in the past and failed on treatment	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	A relapsed client who received treatment and was declared cured or treatment completed at the end of the treatment period and developed TB again. These patients could be true relapses or have a new episode.	EXCLUSIONS	» EXCLUDE extra-pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
RETREATMENT EXTRA-PULMONARY TB START AFTER DEFAULT	DEFINITION	After default extra-pulmonary TB client started on treatment	INCLUSIONS	» INCLUDE clients who have taken TB treatment for 4 weeks or more in the past and defaulted	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	A default client who completed at least one month of treatment and returned after interrupting treatment for two months or more	EXCLUSIONS	» EXCLUDE pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
RETREATMENT EXTRA-PULMONARY TB AFTER FAILURE	DEFINITION	After failure extra-pulmonary TB client started on treatment	INCLUSIONS	» INCLUDE clients who have taken TB treatment for 4 weeks or more in the past and failed on treatment	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	A failed client who received treatment and remained or became smear- or culture-positive at the end of the treatment period	EXCLUSIONS	» EXCLUDE pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net

Other pulmonary TB start on treatment	DEFINITION	Other pulmonary TB client started on treatment	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	'Other' is classified as previously treated but the outcome of previous TB treatment is unknown	EXCLUSIONS	» EXCLUDE extra-pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
Other extra-pulmonary TB start on treatment	DEFINITION	Other extra-pulmonary TB client started on treatment	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Other' is classified as previously treated but the outcome of previous TB treatment is unknown	EXCLUSIONS	» EXCLUDE extra-pulmonary TB cases	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
New smear-positive pulmonary TB client start on treatment	DEFINITION	New smear-positive pulmonary TB client started on TB treatment during the reporting period	INCLUSIONS	» INCLUDE Xpert-positive and clients without an Xpert result but had an AFB-positive baseline smear result	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	New smear-positive PTB client (irrespective of Xpert result) started on TB treatment	EXCLUSIONS	» EXCLUDE extra-pulmonary TB » EXCLUDE smear-negative PTB » EXCLUDE PTB without smear result	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
New smear-positive pulmonary TB client lost to follow-up	DEFINITION	New smear-positive pulmonary TB client started on treatment but was loss to follow up during TB treatment	INCLUSIONS	» INCLUDE All clients with a pre-treatment smear AFB-positive result in the cohort	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	'Lost to follow-up' is defined as interruption of treatment for a period of two consecutive months or longer at any time during the treatment period	EXCLUSIONS	» EXCLUDE extra-pulmonary TB » EXCLUDE smear-negative PTB	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Cohort of clients started on treatment at least 6 months prior	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net

NEW SMEAR-POSITIVE PULMONARY TB CLIENT CURED	DEFINITION	New smear-positive pulmonary TB client started on treatment and was cured at the end of the treatment period	INCLUSIONS	» INCLUDE all clients with a pre-treatment smear AFB-positive result in the cohort	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Smear-positive (irrespective of Xpert result) pre-treatment and is smear-negative in the last month of treatment and on at least one previous occasion more than 30 days prior	EXCLUSIONS	» EXCLUDE extra-pulmonary TB » EXCLUDE smear-negative PTB » EXCLUDE PTB without smear result	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Cohort of clients started on treatment at least 6 months prior	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
NEW SMEAR-POSITIVE PULMONARY TB CLIENT DEATH	DEFINITION	New smear-positive pulmonary tuberculosis clients who were started on treatment but died during the treatment	INCLUSIONS	» INCLUDE all clients with a smear AFB-positive result in the cohort; INCLUDE all deaths even if not due to TB » INCLUDE deaths that occurred in health facilities and in the community	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	The cause of death may not necessarily be due to TB but death occurred during TB treatment period	EXCLUSIONS	» EXCLUDE extra-pulmonary TB clients » EXCLUDE smear-negative PTB clients » EXCLUDE pulmonary TB clients without smear result	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Cohort of clients initiated on TB treatment at least 6 months prior	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
NEW SMEAR-POSITIVE PULMONARY TB CONVERSION AT 2 MONTHS	DEFINITION	New smear-positive pulmonary TB (PTB) patients who completed intensive phase of treatment at 2 months with a negative smear and changed to continuation phase	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE patients on previous TB treatment » EXCLUDE extra-pulmonary TB patients	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net

New smear-positive pulmonary TB conversion at 3 months	DEFINITION	New smear-positive pulmonary TB (PTB) patients who completed intensive phase of treatment at 3 months with a negative smear and changed to continuation phase	INCLUSIONS	None	COLLECTED BY	Clinician; Data capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE patients on previous TB treatment » EXCLUDE extra-pulmonary TB patients	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/Tier.Net
Retreatment smear-positive pulmonary/other TB conversion at 2 months	DEFINITION	Retreatment smear-positive pulmonary TB (PTB) or other TB patients who completed intensive phase of treatment at 2 months with a negative smear and changed to continuation phase	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE extra-pulmonary TB patients	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
Retreatment smear-positive pulmonary/other TB conversion at 3 months	DEFINITION	Retreatment smear-positive pulmonary TB (PTB) or other TB patients who completed intensive phase of treatment at 3 months with a negative smear and changed to continuation phase	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE extra-pulmonary TB patients	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net

TB CLIENT KNOWN HIV-POSITIVE	DEFINITION	All TB clients known to be HIV-positive in the case-finding period	INCLUSIONS	» INCLUDE TB clients who had a positive HIV status prior to TB treatment » INCLUDE TB clients who tested HIV-positive whilst on TB treatment » INCLUDE TB clients already on ART » INCLUDE all TB client categories (new, retreatment, other)	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	This includes TB clients already known to be HIV-positive and those who test HIV-positive during TB treatment	EXCLUSIONS	» EXCLUDE clients with HIV-negative and unknown HIV status	COLLECTION POINTS	All facilities
	USE AND CONTEXT	TB and HIV co-infection. Clients with more than one episode of TB in a year will be counted with each episode in the TB register	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
TB/HIV CO-INFECTED CLIENT ON ART	DEFINITION	HIV-positive TB client on ART in the case finding reporting period	INCLUSIONS	» INCLUDE clients already on ART at start of TB treatment » INCLUDE clients started on ART during TB treatment	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	This includes clients who were on ART prior to TB treatment and those started on ART during TB treatment.	EXCLUSIONS	None	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Clients with more than one episode of TB in a year will be counted with each episode in the TB Register.	FREQUENCY	Quarterly	TOOLS	ETR/TIER.Net
TB MDR CONFIRMED CLIENT	DEFINITION	Client diagnosed with Multidrug-resistant TB (MDR-TB)	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Clients confirmed with resistance to at least both isoniazid and rifampicin	EXCLUSIONS	» EXCLUDE clients with Rifampicin mono resistance » EXCLUDE clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	NICD

TB MDR CONFIRMED CLIENT START ON TREATMENT	DEFINITION	Confirmed MDR-TB client started on treatment during the reporting period	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Clients confirmed with resistance to at least both isoniazid and rifampicin started on treatment	EXCLUSIONS	» EXCLUDE clients with rifampicin mono-resistance » EXCLUDE TB MDR confirmed but not started on treatment » EXCLUDE TB rifampicin-resistant on treatment » EXCLUDE clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net
TB MDR CLIENT LOST TO FOLLOW-UP	DEFINITION	Confirmed MDR-TB client who is lost to follow-up during the treatment period	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	A client whose treatment was interrupted for 2 consecutive months or more	EXCLUSIONS	» EXCLUDE TB MDR confirmed but not started on treatment » EXCLUDE TB rifampicin-resistant on treatment » EXCLUDE clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net
TB MDR CLIENT SUCCESSFULLY COMPLETE TREATMENT	DEFINITION	Confirmed MDR-TB client successfully treated at the end of the treatment period	INCLUSIONS	» INCLUDE clients who were cured, i.e. 3 or more consecutive cultures taken at least 30 days apart are negative after the intensive phase and those who completed treatment without evidence of failure and no record of cure	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE TB MDR confirmed but not started on treatment » EXCLUDE TB rifampicin-resistant on treatment » EXCLUDE clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net

TB MDR DEATH	DEFINITION	Confirmed MDR-TB client who died during treatment period	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Clients who die from any cause during the course of treatment. The cause of death may not necessarily be due to TB MDR. Includes deaths that occurred in health facilities and in the community.	EXCLUSIONS	» EXCLUDE TB MDR confirmed but not started on treatment » EXCLUDE clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net
TB XDR CONFIRMED CLIENT	DEFINITION	Client diagnosed with extensive drug-resistant TB (XDR-TB)	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Clients confirmed with resistance to any fluoroquinolone and to at least one of three second-line injectable drugs (capreomycin, kanamycin and amikacin), in addition to multidrug resistance (MDR)	EXCLUSIONS	» EXCLUDE clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	These clients may already be on MDR-TB treatment on diagnosis.	FREQUENCY	Quarterly	TOOLS	NICD
TB XDR CONFIRMED CLIENT START ON TREATMENT	DEFINITION	Confirmed XDR-TB client started on treatment during the reporting period	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	These clients may already be on MDR-TB treatment on diagnosis and treatment changed/ adjusted accordingly.	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net

TB XDR CLIENT LOST TO FOLLOW-UP	DEFINITION	Confirmed XDR-TB client who is lost to follow-up during the treatment period	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	A client whose treatment was interrupted for 2 consecutive months or more	EXCLUSIONS	» EXCLUDE TB XDR clients started on treatment based on clinical suspicion and resistance pattern of index case » EXCLUDE clients who are known to have died during TB treatment. » EXCLUDE clients who were transferred out	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net
TB XDR CLIENT SUCCESSFULLY COMPLETE TREATMENT	DEFINITION	Confirmed XDR-TB client successfully treated at the end of the treatment period	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	This includes clients who were cured, i.e. 3 or more consecutive cultures taken at least 30 days apart are negative after the intensive phase, and those who completed treatment without evidence of failure and no record of cure.	EXCLUSIONS	» EXCLUDE TB XDR clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net
TB XDR DEATH	DEFINITION	Confirmed XDR-TB client who died during treatment period	INCLUSIONS	» INCLUDE deaths that occurred in health facilities and in the community	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Clients who die from any cause during the course of treatment. The cause of death may not necessarily be due to TB XDR.	EXCLUSIONS	» EXCLUDE TB XDR clients started on treatment based on clinical suspicion and resistance pattern of index case	COLLECTION POINTS	All facilities
	USE AND CONTEXT	None	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net

TB Rifampicin-Resistant confirmed client	DEFINITION	Rifampicin-resistant TB clients diagnosed using Xpert during the reporting period	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	This could be accompanied by resistance to other drugs or mono-resistance, but only rifampicin resistance is confirmed during the reporting period.	EXCLUSIONS	» EXCLUDE confirmed TB MDR and XDR » EXCLUDE clients diagnosed using other methods – culture and DST, LPA	COLLECTION POINTS	All facilities
	USE AND CONTEXT	These clients are recorded in the TB Identification Register but not in the TB Register.	FREQUENCY	Quarterly	TOOLS	NICD
TB Rifampicin-resistant confirmed client start on treatment	DEFINITION	Rifampicin-resistant TB clients diagnosed using Xpert started on treatment during the reporting period	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	This could be accompanied by resistance to other drugs or mono-resistance, but only rifampicin resistance is confirmed during the reporting period.	EXCLUSIONS	» EXCLUDE confirmed TB MDR and XDR	COLLECTION POINTS	All facilities
	USE AND CONTEXT	These clients are recorded in the DR-TB Register and captured in EDR.Web and/or TIER.Net at the DR-TB treatment initiation site. Treatment should be started within 5 days of diagnosis.	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net
TB Rifampicin-resistant confirmed client sputum sent for culture and DST	DEFINITION	Rifampicin-resistant TB clients who had sputum collected for baseline culture and second line DST	INCLUSIONS	None	COLLECTED BY	Clinician, Data Capturer
	EXTENDED DEFINITION	Confirmatory culture and drug susceptibility testing must be conducted on all rifampicin-resistant TB for MDR-TB.	EXCLUSIONS	» EXCLUDE clients confirmed with TB MDR and XDR upfront » EXCLUDE other methods of diagnosis – culture and DST, LPA	COLLECTION POINTS	All facilities
	USE AND CONTEXT	These clients are recorded in the DR-TB Register and captured in EDR.Web.	FREQUENCY	Quarterly	TOOLS	DR-TB Register and captured in EDR.Web/ TIER.Net

CERVICAL CANCER SCREENING 20 YEARS AND OLDER	DEFINITION	A cervical cancer screening includes Pap smear, visual inspection with acetic acid (VIA) and liquid-based cytology for women 20 years and older	INCLUSIONS	» INCLUDE smears done during antenatal care » INCLUDE smears done during postnatal care » INCLUDE smears done on HIV-positive women	COLLECTED BY	Clinician
	EXTENDED DEFINITION	The policy states that high-risk women, e.g. HIV-positive, should be screened every 3 years starting at the age of 20. All other women should be screened every 10 years, with at least 3 screenings in a lifetime, starting at the age of 20 years.	EXCLUSIONS	» EXCLUDE repeat smears	COLLECTION POINTS	PHC OPD
	USE AND CONTEXT	Only smears, visual inspections and liquid-based screening done for women in the specified age category should be counted here. The smear must be sufficient to enable quality screening (e.g. include endo-cervical cells).	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD
IUCD INSERTED	DEFINITION	Intrauterine Contraceptive Device (IUCD) inserted into a woman aged 15–49 years	INCLUSIONS	» INCLUDE referrals to a General Practitioner to insert the IUCD when the IUCD is supplied by the facility	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each IUCD inserted.	EXCLUSIONS	» EXCLUDE IUCD inserted to women younger than 15 years of age and older than 49 years of age	COLLECTION POINTS	All facilities
	USE AND CONTEXT	IUCDs are relatively uncommon in developing countries, and the numbers are small compared to e.g. injectable or oral contraceptives. Facility numbers above 10 during one reporting period should be verified.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register
ORAL PILL CYCLE	DEFINITION	A packet (cycle) of oral contraceptives issued to a woman aged 15–49 years	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each packet issued. Each packet contains pills for one cycle (28 days).	EXCLUSIONS	» EXCLUDE oral contraceptive pills given for the treatment of breakthrough bleedings on medroxyprogesterone injection » EXCLUDE oral cycle given to women younger than 15 years of age and older than 49 years of age	COLLECTION POINTS	All Facilities
	USE AND CONTEXT	The packets issued per client would normally range from around 3 given to e.g. new/young users who need closer monitoring for side effects, and up to 6 given to older women who have used pills for many years without known side effects.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD; Delivery Register

NORETHISTERONE ENANTHATE INJECTION	DEFINITION	Norethisterone enanthate injection given to a woman aged 15–49 years	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each injection given. This injection provides protection against pregnancy for 2 months.	EXCLUSIONS	» EXCLUDE norethisterone enanthate injection given to women younger than 15 years of age and older than 49 years of age	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Do not mix up medroxyprogesterone and norethisterone enanthate injections.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD; Delivery Register
MEDROXYPROGESTERONE INJECTION	DEFINITION	Medroxyprogesterone acetate (Depo Provera/ Petogen) injection given to a woman aged 15–49 years	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each injection given. This injection provides protection against pregnancy for 3 months.	EXCLUSIONS	» EXCLUDE medroxyprogesterone acetate (Depo Provera/Petogen) injection given to women younger than 15 years of age and older than 49 years of age	COLLECTION POINTS	All facilities
	USE AND CONTEXT	Do not mix up medroxyprogesterone and norethisterone enanthate injections.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD; Delivery Register
SUB-DERMAL IMPLANT INSERTED	DEFINITION	Sub-dermal contraceptive implant inserted just under the skin of a woman aged 15–49 years upper arm	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Count each contraceptive implant inserted.	EXCLUSIONS	» EXCLUDE sub-dermal contraceptive implants inserted to women younger than 15 years of age and older than 49 years of age	COLLECTION POINTS	All facilities
	USE AND CONTEXT	The sub-dermal contraceptive implant is active for 3 years.	FREQUENCY	Monthly	TOOLS	PHC Comprehensive Tick Register; Tick Register OPD; Delivery Register
STERILISATION – FEMALE	DEFINITION	Surgical procedure to protect a woman from further pregnancy	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Interval and post-partum sterilisations should be counted. The sterilisation can be performed by means of a mini-laparotomy or laparoscopic procedure.	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Count each case only in the facility where the operation is actually performed.	FREQUENCY	Monthly	TOOLS	Threatre Register

STERILISATION – MALE	DEFINITION	Surgical procedure to prevent a man from being fertile	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	Also called vasectomy	EXCLUSIONS	None	COLLECTION POINTS	Inpatient facility
	USE AND CONTEXT	Count each case when the operation is actually performed.	FREQUENCY	Monthly	TOOLS	Threatre Register
TERMINATION OF PREGNANCY 0–12 WEEKS	DEFINITION	Termination of pregnancy in the first 12 weeks of pregnancy performed under safe conditions in a health facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE therapeutic TOPs	COLLECTION POINTS	Facilities where ToPs are performed
	USE AND CONTEXT	Count each case ONLY in the facility (designated facility) where the termination is actually performed.	FREQUENCY	Monthly	TOOLS	Threatre Register
TERMINATION OF PREGNANCY 13–20 WEEKS	DEFINITION	Termination of pregnancy at 13–20 weeks of pregnancy performed under safe conditions in a health facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE therapeutic TOPs	COLLECTION POINTS	Facilities where ToPs are performed
	USE AND CONTEXT	Count each case ONLY in the facility (designated facility) where the termination is actually performed.	FREQUENCY	Monthly	TOOLS	Threatre Register
TERMINATION OF PREGNANCY 20 YEARS AND OLDER	DEFINITION	Termination of pregnancy 20 years and older performed under safe conditions in a health facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE therapeutic TOPs	COLLECTION POINTS	Facilities where ToPs are performed
	USE AND CONTEXT	Count each case ONLY in the facility (designated facility) where the termination is actually performed.	FREQUENCY	Monthly	TOOLS	Threatre Register
TERMINATION OF PREGNANCY 10–19 YEARS	DEFINITION	Termination of pregnancy 10 to 19 years performed under safe conditions in a health facility	INCLUSIONS	None	COLLECTED BY	Clinician
	EXTENDED DEFINITION	None	EXCLUSIONS	» EXCLUDE therapeutic TOPs	COLLECTION POINTS	Facilities where ToPs are performed
	USE AND CONTEXT	Count each case ONLY in the facility (designated facility) where the termination is actually performed.	FREQUENCY	Monthly	TOOLS	Threatre Register

DATA ELEMENTS

INDICATORS

Termination of pregnancy 0–12 weeks rate	NUMERATOR	Termination of pregnancy 0–12 weeks	DENOMINATOR FORMULA	SUM([Termination of pregnancy performed])	TYPE	%
	NUMERATOR FORMULA	SUM([Termination of pregnancy performed 0–12 weeks])	DEFINITION	Pregnancies terminated in health facilities in the first 12 weeks of pregnancy as a proportion of total termination of pregnancies	LEVEL	Process/Activity
	DENOMINATOR	Termination of pregnancy performed	USE AND CONTEXT	Monitors access to and utilisation of ToP services. High ToP rates may also indicate that contraceptive services are inadequate.	KEY REPORTS	None
Termination of pregnancy 13–20 weeks rate	NUMERATOR	Termination of pregnancy 13–20 weeks	DENOMINATOR FORMULA	SUM([Termination of pregnancy performed])	TYPE	%
	NUMERATOR FORMULA	SUM([Termination of pregnancy performed 13–20 weeks])	DEFINITION	Pregnancies terminated in health facilities at 13–20 weeks of pregnancy as a proportion of total termination of pregnancies	LEVEL	Process/Activity
	DENOMINATOR	Termination of pregnancy performed	USE AND CONTEXT	Monitors high-risk utilisation of ToP services. High ToP rates may also indicate poor access and utilisation to either contraceptive or ToP services and/or inadequate counselling.	KEY REPORTS	None
Termination of pregnancy 10–19 years rate	NUMERATOR	Termination of pregnancy 10–19 years	DENOMINATOR FORMULA	SUM([Termination of pregnancy performed])	TYPE	%
	NUMERATOR FORMULA	SUM([Termination of pregnancy performed 10–19 years])	DEFINITION	Termination of pregnancy under 20 years as a proportion of total termination of pregnancies in health facilities	LEVEL	Process/Activity
	DENOMINATOR	Termination of pregnancy performed	USE AND CONTEXT	Monitors access to youth friendly services, including contraceptive utilisation.	KEY REPORTS	SDG3 proxy
Termination of pregnancy – total	NUMERATOR	Termination of pregnancy – total	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	Sum of Termination of pregnancy (Termination of pregnancy 0–12 weeks, Termination of pregnancy 13–20 weeks)	DEFINITION	Total termination of pregnancy (ToP) clients	LEVEL	Process/Activity
	DENOMINATOR	None	USE AND CONTEXT	Monitors access to and utilisation of ToP services. High ToP rates may also indicate that contraceptive services are inadequate.	KEY REPORTS	None

CERVICAL CANCER SCREENING COVERAGE 20 YEARS AND OLDER	NUMERATOR	Cervical cancer screening in women 20 years and older	DENOMINATOR FORMULA	(SUM([Female 20-24 years]) + SUM([Female 25-29 years]) + SUM([Female 30-34 years]) + SUM([Female 35-39 years]) + SUM([Female 40-44 years]) + SUM([Female 45 years and older])) / 14.7	TYPE	%
	NUMERATOR FORMULA	SUM([Cervical cancer screening 20 years and older])	DEFINITION	Cervical smears in women 20 years and older as a proportion of the target for cervical cancer screening. According to the SA National HIV survey 2012 done by HSRC, 20.6% (used 20% in calculations) of females over 20 years of age are HIV-positive. These women must be screened for cervical cancer every 3 years. The other 80% must still be screened every 10 years.	LEVEL	Output
	DENOMINATOR	Target population 20 years and older females	USE AND CONTEXT	Monitors implementation of policy on cervical screening and ART policy.	KEY REPORTS	APP, MTSF 2014–2019; Strategic Plan 2015/16–2019/20
COUPLE YEAR PROTECTION RATE (INTERNATIONAL)	NUMERATOR	Couple year protection	DENOMINATOR FORMULA	(SUM([Female 15-44 years]) + SUM([Female 45-49 years]))	TYPE	%
	NUMERATOR FORMULA	(SUM([Oral pill cycle]) / 15) + (SUM([Medroxyprogesterone injection]) / 4) + (SUM([Norethisterone enanthate injection]) / 6) + (SUM([IUCD inserted]) * 4.5) + (SUM([Male condoms distributed]) / 120) + (SUM([Sterilisation - male]) * 10) + (SUM([Sterilisation - female]) * 10) + (SUM([Female condoms distributed]) / 120) + (SUM([Sub-dermal implant inserted]) * 2.5)	DEFINITION	Women protected against pregnancy by using modern contraceptive methods, including sterilisations, as proportion of female population 15–49 years. Couple year protection are the total of (Oral pill cycles / 15) + (Medroxyprogesterone injection / 4) + (Norethisterone enanthate injection / 6) + (IUCD x 4.5) +) + (Sub dermal implant x 2.5) + Male condoms distributed / 120) + (Female condoms distributed / 120) + (Male sterilisation x 10) + (Female sterilisation x 10	LEVEL	Outcome
	DENOMINATOR	Population 15–49 years females	USE AND CONTEXT	Monitors access to and utilisation of modern contraceptives to prevent unplanned pregnancies. Serves as proxy for the indicator Contraceptive prevalence rate by monitoring trends between official surveys.	KEY REPORTS	MTSF 2014–2019, 2016/17–2018/19 APP; SDG3 Proxy

TB/HIV CO-INFECTED CLIENT ON ART RATE						
	NUMERATOR	TB/HIV co-infected client on ART	DENOMINATOR FORMULA	SUM([TB client known HIV-positive])	TYPE	%
	NUMERATOR FORMULA	SUM([TB/HIV co-infected client on ART])	DEFINITION	TB/HIV co-infected clients on ART as a proportion of ALL HIV-positive TB clients	LEVEL	Outcome
	DENOMINATOR	HIV-positive TB client	USE AND CONTEXT	All co-infected clients must be on ART to reduce mortality. This includes clients already on ART at TB treatment initiation and those started on ART during TB treatment. Monitors ART coverage for TB clients.	KEY REPORTS	2016/17–2018/19, MTSF 2014–2019, Strategic Plan 2015/16–2019/20

TB RIFAMPICIN-RESISTANT CONFIRMED TREATMENT START RATE	**NUMERATOR**	TB rifampicin-resistant confirmed clients start on treatment	**DENOMINATOR FORMULA**	SUM([TB rifampicin-resistant confirmed client])	**TYPE**	%
	NUMERATOR FORMULA	SUM([TB rifampicin-resistant confirmed client start on treatment])	**DEFINITION**	TB rifampici-rResistant confirmed clients started on treatment as a proportion of TB rifampicin-resistant confirmed clients	**LEVEL**	Outcome
	DENOMINATOR	TB rifampicin-resistant confirmed client	**USE AND CONTEXT**	Monitors initial loss to follow-up and the effectiveness of linkage to care strategies for clients diagnosed using Xpert as rifampicin-resistant TB. Treatment should be initiated within 5 days of diagnosis.	**KEY REPORTS**	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
TB XDR TREATMENT SUCCESS RATE	**NUMERATOR**	TB XDR client successfully complete treatment	**DENOMINATOR FORMULA**	SUM([TB XDR confirmed client start on treatment])	**TYPE**	%
	NUMERATOR FORMULA	SUM([TB XDR client successfully complete treatment])	**DEFINITION**	TB XDR clients successfully completing treatment as a proportion of TB XDR confirmed clients started on treatment	**LEVEL**	Outcome
	DENOMINATOR	TB XDR confirmed client start on treatment	**USE AND CONTEXT**	Monitors success of XDR TB treatment.	**KEY REPORTS**	2016/17–2018/19, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
TB XDR LOST TO FOLLOW-UP RATE	**NUMERATOR**	TB XDR client lost to follow-up	**DENOMINATOR FORMULA**	SUM([TB XDR confirmed client start on treatment])	**TYPE**	%
	NUMERATOR FORMULA	SUM([TB XDR client lost to follow-up])	**DEFINITION**	TB XDR clients lost to follow-up as a proportion of TB XDR confirmed clients started on treatment	**LEVEL**	Outcome
	DENOMINATOR	TB XDR confirmed client start on treatment	**USE AND CONTEXT**	Monitors the effectiveness of the retention in care strategies during MDR TB treatment period.	**KEY REPORTS**	2016/17–2018/19, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
TB XDR DEATH RATE	**NUMERATOR**	TB XDR died during treatment	**DENOMINATOR FORMULA**	SUM([TB XDR confirmed client start on treatment])	**TYPE**	%
	NUMERATOR FORMULA	SUM([TB XDR death])	**DEFINITION**	TB XDR clients who died as a proportion of TB XDR confirmed clients who started on treatment	**LEVEL**	Impact
	DENOMINATOR	TB XDR confirmed client start on treatment	**USE AND CONTEXT**	Monitors death during XDR TB treatment period. The cause of death may not necessarily be due to TB.	**KEY REPORTS**	2016/17–2018/19, MTSF 2014–2019, Strategic Plan 2015/16–2019/20

TB RIFAMPICIN-RESISTANT DST TESTING RATE	NUMERATOR	TB rifampicin-resistant confirmed client sputum sent for culture and DST	DENOMINATOR FORMULA	SUM([TB rifampicin-resistant confirmed client])	TYPE	%
	NUMERATOR FORMULA	SUM([TB rifampicin-resistant confirmed client sputum sent for culture and DST])	DEFINITION	TB rifampicin-resistant confirmed client sputum sent for culture and DST as a proportion of TB rifampicin-resistant confirmed client	LEVEL	Output
	DENOMINATOR	TB rifampicin-resistant confirmed client	USE AND CONTEXT	Monitors the confirmatory testing for MDR-TB. Monitors the rifampicin non-resistance or discordant results.	KEY REPORTS	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
TB MDR TREATMENT SUCCESS RATE	NUMERATOR	TB MDR client successfully complete treatment	DENOMINATOR FORMULA	SUM([TB MDR confirmed client start on treatment])	TYPE	%
	NUMERATOR FORMULA	SUM([TB MDR client successfully complete treatment])	DEFINITION	TB MDR client successfully completing treatment as a proportion of TB MDR confirmed clients started on treatment	LEVEL	Outcome
	DENOMINATOR	TB MDR confirmed client start on treatment	USE AND CONTEXT	Monitors success of MDR TB treatment.	KEY REPORTS	APP; MTSF 2014–2019; NSP; WHO; Strategic Plan 2015/16–2019/20
TB MDR LOST TO FOLLOW-UP RATE	NUMERATOR	TB MDR client lost to follow-up	DENOMINATOR FORMULA	SUM([TB MDR confirmed client start on treatment])	TYPE	%
	NUMERATOR FORMULA	SUM([TB MDR client lost to follow-up])	DEFINITION	TB MDR clients lost to follow-up as a proportion of TB MDR confirmed clients started on treatment	LEVEL	Outcome
	DENOMINATOR	TB MDR confirmed client start on treatment	USE AND CONTEXT	Monitors the effectiveness of the retention in care strategies during MDR TB treatment period.	KEY REPORTS	2016/17–2018/19, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
TB MDR DEATH RATE	NUMERATOR	TB MDR died during treatment	DENOMINATOR FORMULA	SUM([TB MDR confirmed client start on treatment])	TYPE	%
	NUMERATOR FORMULA	SUM([TB MDR death])	DEFINITION	TB MDR clients who died as a proportion of TB MDR confirmed clients who started on treatment	LEVEL	Impact
	DENOMINATOR	TB MDR confirmed client start on treatment	USE AND CONTEXT	Monitors death during MDR TB treatment period. The cause of death may not necessarily be due to TB.	KEY REPORTS	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20

TB CLIENT TREATMENT SUCCESS RATE	**NUMERATOR**	TB client successfully completed treatment	**DENOMINATOR FORMULA**	SUM([New smear positive pulmonary TB client start on treatment])	**TYPE**	%
	NUMERATOR FORMULA	SUM([TB client successfully completed treatment])	**DEFINITION**	TB clients successfully completed treatment (both cured and treatment completed) as a proportion of ALL TB clients started on treatment. This applies to ALL TB clients (New, Retreatment, Other, pulmonary and extra-pulmonary).	**LEVEL**	Outcome
	DENOMINATOR	TB client start on treatment	**USE AND CONTEXT**	Monitors success of TB treatment for ALL types of TB. This follows a cohort analysis, therefore the clients would have been started on treatment at least 6 months prior.	**KEY REPORTS**	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
TB CLIENT LOST TO FOLLOW-UP RATE	**NUMERATOR**	TB client lost to follow-up	**DENOMINATOR FORMULA**	SUM([TB client start on treatment])	**TYPE**	%
	NUMERATOR FORMULA	SUM([TB client lost to follow-up])	**DEFINITION**	TB clients who are lost to follow-up (missed two months or more of treatment) as a proportion of TB clients started on treatment. This applies to ALL TB clients (New, Retreatment, Other, pulmonary and extra-pulmonary)	**LEVEL**	Outcome
	DENOMINATOR	TB client start on treatment	**USE AND CONTEXT**	Monitors success of TB treatment for ALL types of TB. This follows a cohort analysis, therefore the clients would have been started on treatment at least 6 months prior.	**KEY REPORTS**	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
TB CLIENT DEATH RATE	**NUMERATOR**	TB client died during treatment	**DENOMINATOR FORMULA**	SUM([New smear-positive pulmonary TB client start on treatment])	**TYPE**	%
	NUMERATOR FORMULA	SUM([TB client died during treatment])	**DEFINITION**	TB clients who died during treatment as a proportion of TB clients started on treatment. This applies to ALL TB clients (New, Retreatment, Other, pulmonary and extra-pulmonary).	**LEVEL**	Impact
	DENOMINATOR	TB client start on treatment	**USE AND CONTEXT**	Monitors death during TB treatment period. The cause of death may not necessarily be due to TB. This follows a cohort analysis, therefore the clients would have been started on treatment at least 6 months prior.	**KEY REPORTS**	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20

New smear-positive PTB cure rate						
NUMERATOR	New smear-positive pulmonary TB client cured	DENOMINATOR FORMULA	SUM([New smear-positive pulmonary TB client start on treatment])	TYPE	%
NUMERATOR FORMULA	SUM([New smear positive pulmonary TB client cured])	DEFINITION	New smear-positive pulmonary TB clients cured as a proportion of new smear-positive pulmonary TB clients who started on treatment	LEVEL	Output
DENOMINATOR	New smear-positive pulmonary TB client start on treatment	USE AND CONTEXT	Monitors cure of new infectious TB cases. The aim is to effectively treat and cure all new infectious cases at first attempt. This follows a cohort analysis, therefore the clients would have been started on treatment at least 6 months prior.	KEY REPORTS	2016/17–2018/19; MTSF 2014–2019, Strategic Plan 2015/16–2019/20

New smear-positive PTB loss to follow up rate						
NUMERATOR	New smear-positive pulmonary TB client loss to follow up	DENOMINATOR FORMULA	SUM([New smear positive pulmonary TB client start on treatment])	TYPE	%
NUMERATOR FORMULA	SUM([New smear-positive pulmonary TB client loss to follow up])	DEFINITION	New smear-positive pulmonary TB clients loss to follow up as a proportion of new smear-positive pulmonary TB clients started on treatment	LEVEL	Outcome
DENOMINATOR	New smear-positive pulmonary TB client start on treatment	USE AND CONTEXT	Monitors cure of new infectious TB cases. The aim is to effectively treat and cure all new infectious cases at first attempt. This follows a cohort analysis, therefore the clients would have been started on treatment at least 6 months prior.	KEY REPORTS	2016/17–2018/19; MTSF 2014–2019, Strategic Plan 2015/16–2019/20

New smear-positive PTB death rate						
NUMERATOR	New smear-positive pulmonary TB client death	DENOMINATOR FORMULA	SUM([New smear-positive pulmonary TB client start on treatment])	TYPE	%
NUMERATOR FORMULA	SUM([New smear-positive pulmonary TB client death])	DEFINITION	New smear-positive pulmonary TB clients who died during TB treatment as a proportion of new smear-positive pulmonary TB clients started on treatment	LEVEL	Outcome
DENOMINATOR	New smear-positive pulmonary TB client start on treatment	USE AND CONTEXT	Monitors cure of new infectious TB cases. The aim is to effectively treat and cure all new infectious cases at first attempt. This follows a cohort analysis, therefore the clients would have been started on treatment at least 6 months prior.	KEY REPORTS	2016/17–2018/19; MTSF 2014–2019, Strategic Plan 2015/16–2019/20

RETREATMENT TB START RATE	NUMERATOR	Retreatment TB start	DENOMINATOR FORMULA	SUM([TB client start on treatment])	TYPE	%
	NUMERATOR FORMULA	SUM([Retreatment pulmonary TB start after relapse]) + SUM([Retreatment pulmonary TB start after default]) + SUM([Retreatment pulmonary TB start after failure]) + SUM([Retreatment extra-pulmonary TB start after relapse]) + SUM([Retreatment extra-pulmonary TB start after default]) + SUM([Retreatment extra-pulmonary TB start after failure])	DEFINITION	Retreatment TB clients started as a proportion of total TB patients who started treatment	LEVEL	Process/Activity
	DENOMINATOR	TB client start on treatment	USE AND CONTEXT	Monitors cure of new infectious TB cases	KEY REPORTS	None
NEW SMEAR-POSITIVE PTB CONVERSION AT 2 OR 3 MONTHS RATE	NUMERATOR	New smear-positive pulmonary TB conversion at 2 or 3 months	DENOMINATOR FORMULA	SUM([New smear-positive pulmonary TB client start on treatment])	TYPE	%
	NUMERATOR FORMULA	SUM([New smear-positive pulmonary TB conversion at 2 months]) + SUM([New smear-positive pulmonary TB conversion at 3 months])	DEFINITION	New smear-positive pulmonary TB conversion at 2 or 3 months as a proportion of new smear-positive pulmonary TB clients who started on treatment	LEVEL	Process/Activity
	DENOMINATOR	New smear positive pulmonary TB client start on treatment	USE AND CONTEXT	Monitors cure of new infectious TB cases.	KEY REPORTS	None
RETREATMENT SMEAR-POSITIVE PULMONARY/OTHER TB CONVERSION AT 2 OR 3 MONTHS RATE	NUMERATOR	Retreatment smear-positive pulmonary/other TB conversion at 2 or 3 months	DENOMINATOR FORMULA	SUM([Retreatment pulmonary TB start after relapse]) + SUM([Retreatment pulmonary TB start after default]) + SUM([Retreatment pulmonary TB start after failure]) + SUM([Other pulmonary TB start on treatment])	TYPE	%
	NUMERATOR FORMULA	Sum([Retreatment smear-positive pulmonary/other TB conversion at 2 months]) + Sum([Retreatment smear-positive pulmonary/other TB conversion at 3 months])	DEFINITION	Retreatment smear-positive pulmonary/other TB conversion at 2 or 3 months as a proportion of Retreatment smear-positive pulmonary/other TB cases started treatment	LEVEL	Process/Activity
	DENOMINATOR	Retreatment smear-positive pulmonary/other TB started	USE AND CONTEXT	Monitors cure of new infectious TB cases.	KEY REPORTS	None

TB POSITIVE CLIENT 5 YEARS AND OLDER RATE	NUMERATOR	TB symptomatic client 5 years and older tested positive	DENOMINATOR FORMULA	SUM([TB symptomatic client 5 years and older with sputum sent])	TYPE	%
	NUMERATOR FORMULA	SUM([TB symptomatic client 5 years and older tested positive])	DEFINITION	TB symptomatic clients who tested positive as a proportion of TB symptomatic clients 5 years and older with sputum sent	LEVEL	Input
	DENOMINATOR	TB symptomatic client 5 years and older with sputum sent	USE AND CONTEXT	Monitors early detection of TB clients.	KEY REPORTS	NSP; WHO, SGD3 Proxy
TB CLIENT 5 YEARS AND OLDER START ON TREATMENT RATE	NUMERATOR	TB client 5 years and older start on treatment	DENOMINATOR FORMULA	SUM([TB symptomatic client 5 years and older tested positive])	TYPE	%
	NUMERATOR FORMULA	SUM([TB client 5 years and older start on treatment])	DEFINITION	TB client 5 years and older start on treatment as a proportion of TB symptomatic client 5 years and older test positive	LEVEL	Output
	DENOMINATOR	TB symptomatic client 5 years and older tested positive	USE AND CONTEXT	Monitors initial loss to follow-up and the effectiveness of linkage to treatment and care strategies.	KEY REPORTS	APP; NSP; WHO
TB CONTACT UNDER 5 YEARS IPT UPTAKE RATE	NUMERATOR	TB asymptomatic contact under 5 years start on IPT	DENOMINATOR FORMULA	SUM([TB asymptomatic contact under 5 years])	TYPE	%
	NUMERATOR FORMULA	SUM([TB asymptomatic contact under 5 years start on IPT])	DEFINITION	TB contacts under 5 years started on Isoniazid Preventive therapy (IPT) as a proportion of asymptomatic TB contacts under 5 years	LEVEL	Output
	DENOMINATOR	TB asymptomatic contact under 5 years	USE AND CONTEXT	Monitors TB preventive therapy in children under 5 years.	KEY REPORTS	NSP
HIV-POSITIVE NEW CLIENT START ON IPT RATE	NUMERATOR	HIV-positive eligible client initiated on IPT	DENOMINATOR FORMULA	SUM([HIV-positive client eligible for IPT])	TYPE	%
	NUMERATOR FORMULA	DHIS Formula	DEFINITION	HIV-positive IPT eligible clients who started on IPT as a proportion of HIV positive clients who are eligible for IPT	LEVEL	Process/ Activity
	DENOMINATOR	HIV-positive client eligible for IPT	USE AND CONTEXT	Monitors initiation of Isoniazid preventive therapy (IPT) to prevent TB.	KEY REPORTS	

TB SYMPTOM CHILD UNDER 5 YEARS SCREENED IN FACILITY RATE	NUMERATOR	Child under 5 years screened for TB symptoms	DENOMINATOR FORMULA	SUM([PHC headcount under 5 years])	TYPE	%
	NUMERATOR FORMULA	SUM([Child under 5 years screened for TB symptoms])	DEFINITION	SUM([PHC headcount under 5 years])	LEVEL	Process/ Activity
	DENOMINATOR	PHC headcount under 5 years	USE AND CONTEXT	Children under 5 years screened for TB symptoms as a proportion of PHC headcount under 5 years	KEY REPORTS	MTSF 2014–2019, SDG3 Proxy
TB SYMPTOM 5 YEARS AND OLDER SCREENED IN FACILITY RATE	NUMERATOR	Client 5 years and older screened for TB	DENOMINATOR FORMULA	SUM([PHC headcount 5 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([Client 5 years and older screened for TB])	DEFINITION	Clients 5 years and older screened for TB symptoms as a proportion of PHC headcount 5 years and older	LEVEL	Process/ Activity
	DENOMINATOR	PHC headcount 5 years and older	USE AND CONTEXT		KEY REPORTS	APP; MTSF 2014–2019, SDG3 Proxy
TB SPUTUM 5 YEARS AND OLDER TEST RATE	NUMERATOR	TB symptomatic client 5 years and older with sputum sent	DENOMINATOR FORMULA	SUM([TB symptomatic client 5 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([TB symptomatic client 5 years and older with sputum sent])	DEFINITION	TB symptomatic clients 5 years and older with sputum sent to laboratory services as a proportion of clients 5 years and older with TB symptoms	LEVEL	Process/ Activity
	DENOMINATOR	TB symptomatic client 5 years and older	USE AND CONTEXT	Monitors testing of symptomatic clients 5 years and older. Symptoms include any one of the following symptoms according to symptom screening tool: cough, unintended weight loss, night sweats, contact with known TB client.	KEY REPORTS	NSP
TB CHILD UNDER 5 YEARS START ON TREATMENT RATE	NUMERATOR	TB client under 5 years start on treatment	DENOMINATOR FORMULA	SUM([TB symptomatic client under 5 years])	TYPE	%
	NUMERATOR FORMULA	SUM([TB client under 5 years start on treatment])	DEFINITION	TB client under 5 years started on treatment as a proportion of ALL symptomatic children under 5 years	LEVEL	Output
	DENOMINATOR	TB symptomatic client under 5 years	USE AND CONTEXT	Monitors the TB case detection among children under 5 years.	KEY REPORTS	NSP

MALE URETHRITIS SYNDROME INCIDENCE						
	NUMERATOR	Male urethritis syndrome treated – new episodes	DENOMINATOR FORMULA	SUM([Male population 15–49 years])	TYPE	%
	NUMERATOR FORMULA	SUM([Male urethritis syndrome treated – new episodes])	DEFINITION	Male urethritis syndrome cases reported per 1000 male population 15–49 years	LEVEL	Output
	DENOMINATOR	Male population 15–49 years	USE AND CONTEXT	Male urethritis syndrome is the most accurate way to reflect on newly acquired sexually transmitted infections.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

School learner referred for Hearing problems rate	NUMERATOR	School learner referred for Hearing problems	DENOMINATOR FORMULA	SUM([School learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School learner referred for Hearing problems])	DEFINITION	Proportion of learners screened by a nurse in line with the ISHP service package and referred for hearing problems	LEVEL	Process/ Activity
	DENOMINATOR	School learners screened – total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School learner referred for Speech problems rate	NUMERATOR	School learner referred for Speech problems	DENOMINATOR FORMULA	SUM([School learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School learner referred for Speech problems])	DEFINITION	Proportion of learners screened by a nurse in line with the ISHP service package and referred for Speech problems	LEVEL	Process/ Activity
	DENOMINATOR	School learners screened – total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School learner referred for Suspected TB rate	NUMERATOR	School learner referred for Suspected TB	DENOMINATOR FORMULA	SUM([School learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School learner referred for Suspected TB])	DEFINITION	Proportion of learners screened by a nurse in line with the ISHP service package and referred for Suspected TB	LEVEL	Process/ Activity
	DENOMINATOR	School learners screened – total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School learner deworming rate	NUMERATOR	School learner given deworming dose	DENOMINATOR FORMULA	SUM([School Grade 1 - learners screened]) + ([School Grade 4 - learners screened])	TYPE	%
	NUMERATOR FORMULA	SUM([School learner given deworming dose])	DEFINITION	Proportion of learners screened by a nurse in line with the ISHP service package and given deworming tablets	LEVEL	Process/ Activity
	DENOMINATOR	School Grade 1 learners screened plus Grade 4 learners screened	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None

School learner screening coverage	NUMERATOR	School learners screened	DENOMINATOR FORMULA	SUM([School learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School learners screened – total])	DEFINITION	Proportion of all learners screened by a nurse in line with the ISHP service package	LEVEL	Process/ Activity
	DENOMINATOR	School learners – total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School learner underweight rate	NUMERATOR	School learners underweight	DENOMINATOR FORMULA	SUM([School learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School learner underweight])	DEFINITION	Proportion of learners screened by a nurse in line with the ISHP service package diagnosed as underweight (below -2SD but above -3SD)	LEVEL	Process/ Activity
	DENOMINATOR	School learners screened – total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School learner overweight rate	NUMERATOR	School learners overweight	DENOMINATOR FORMULA	SUM([School learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School learner overweight])	DEFINITION	Proportion of learners screened by a nurse in line with the ISHP service package diagnosed as overweight (above +2SD)	LEVEL	Process/ Activity
	DENOMINATOR	School learners screened – total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School learner referred for Oral Health rate	NUMERATOR	School learner referred for Oral Health	DENOMINATOR FORMULA	SUM([School learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School learner referred for Oral Health])	DEFINITION	Proportion of learners screened by a nurse in line with the ISHP service package and referred for Oral Health	LEVEL	Process/ Activity
	DENOMINATOR	School learners screened – total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School learner referred for Eye Care rate	NUMERATOR	School learner referred for Eye Care	DENOMINATOR FORMULA	SUM([School learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School learner referred for Eye Care])	DEFINITION	Proportion of learners screened by a nurse in line with the ISHP service package and referred for Eye Care	LEVEL	Process/ Activity
	DENOMINATOR	School learners screened – total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None

School Grade R screening coverage	NUMERATOR	School Grade R – learners screened	DENOMINATOR FORMULA	SUM([School Grade R – learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School Grade R – learners screened])	DEFINITION	Proportion of Grade R learners screened by a nurse in line with the ISHP service package	LEVEL	Process/ Activity
	DENOMINATOR	School Grade R – learners total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School Grade 1 screening coverage	NUMERATOR	School Grade 1 – learners screened	DENOMINATOR FORMULA	SUM([School Grade 1 - learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School Grade 1 – learners screened])	DEFINITION	Proportion of Grade 1 learners screened by a nurse in line with the ISHP service package	LEVEL	Process/ Activity
	DENOMINATOR	School Grade 1 – learners total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	APP; Strategic Plan 2015/16–2019/20
School Grade 4 screening coverage	NUMERATOR	School Grade 4 – learners screened	DENOMINATOR FORMULA	SUM([School Grade 4 – learners total])	TYPE	%
	NUMERATOR FORMULA	SUM([School Grade 4 – learners screened])	DEFINITION	Proportion of Grade 4 learners screened by a nurse in line with the ISHP service package	LEVEL	Process/ Activity
	DENOMINATOR	School Grade 4 – learners total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None
School Grade 8 screening coverage	NUMERATOR	School Grade 8 – learners screened	DENOMINATOR FORMULA	SUM([School Grade 8 – learners screened])	TYPE	%
	NUMERATOR FORMULA	SUM([School Grade 8 – learners screened])	DEFINITION	Proportion of Grade 8 learners screened by a nurse in line with the ISHP service package	LEVEL	Process/ Activity
	DENOMINATOR	School Grade 8 – learners total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	2016/17–2018/19 APP, Strategic Plan 2015/16–2019/20
School Grade 10 screening coverage	NUMERATOR	School Grade 10 – learners screened	DENOMINATOR FORMULA	SUM([School Grade 10 – learners screened])	TYPE	%
	NUMERATOR FORMULA	SUM([School Grade 10 – learners screened])	DEFINITION	Proportion of Grade 10 learners screened by a nurse in line with the ISHP service package	LEVEL	Process/ Activity
	DENOMINATOR	School Grade 10 – learners total	USE AND CONTEXT	Monitors implementation of the Integrated School Health Programme (ISHP).	KEY REPORTS	None

WHEELCHAIR ISSUED RATE	NUMERATOR	Wheelchairs issued	DENOMINATOR FORMULA	SUM([Wheelchair applications])	TYPE	%
	NUMERATOR FORMULA	SUM([Wheelchair issued – new])	DEFINITION	Wheelchairs issued as a proportion of the applications for wheelchairs received	LEVEL	Outcome
	DENOMINATOR	All wheelchair applications received at the facility	USE AND CONTEXT	Planning and budgeting of services	KEY REPORTS	None

HEARING AIDS ISSUED RATE	NUMERATOR	Hearing aids issued – new	DENOMINATOR FORMULA	SUM([Hearing aid applications received])	TYPE	%
	NUMERATOR FORMULA	SUM([Hearing aids issued – new])	DEFINITION	Hearing aids issued as a proportion of the applications for hearing aids received	LEVEL	Process/ Activity
	DENOMINATOR	Hearing aid applications received	USE AND CONTEXT	Planning and budgeting of services	KEY REPORTS	None

Severity assessment code (SAC) 1 incidents reported within 24 hours rate *						
	NUMERATOR	Severity assessment code (SAC) 1 incidents reported within 24 hours	DENOMINATOR FORMULA	SUM([Severity assessment code (SAC) 1 incident reported])	TYPE	%
	NUMERATOR FORMULA	SUM([Severity assessment code (SAC) 1 incident reported within 24 hours])	DEFINITION	Severity assessment code (SAC) 1 incidents reported within 24 hours as a proportion of Severity assessment code (SAC) 1 incident reported	LEVEL	Process/ Activity
	DENOMINATOR	Severity assessment code (SAC) 1 incident reported	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents	KEY REPORTS	APP

Patient Safety Incident (PSI) cases closure within 60 working days rate *						
	NUMERATOR	Patient Safety Incident (PSI) case closed within 60 days	DENOMINATOR FORMULA	SUM([Patient Safety Incident (PSI) case closed])	TYPE	%
	NUMERATOR FORMULA	SUM([Patient Safety Incident (PSI) cases closed within 60 days])	DEFINITION	Patient Safety Incident (PSI) cases closed within 60 days as a proportion of Patient Safety Incident (PSI) cases closed	LEVEL	Process/ Activity
	DENOMINATOR	Patient Safety Incident (PSI) case closed	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents	KEY REPORTS	APP

Ideal clinic status determinations conducted by PPTICRM rate (fixed clinic/CHC/CDC) *	NUMERATOR	Ideal Clinic status determinations conducted by PPTICRM (fixed clinic/CHC/CDC)	DENOMINATOR FORMULA	SUM([Fixed clinic]) + SUM([Fixed CHC/CDC])	TYPE	%
	NUMERATOR FORMULA	SUM([Ideal Clinic status determinations conducted by PPTICRM (fixed clinic/CHC/CDC) (fixed clinic/CHC/CDC)])	DEFINITION	Fixed clinics, CHCs and CDCs where Ideal Clinic status determinations are conducted by PPTICRM as a proportion of Fixed clinics plus fixed CHCs/CDCs	LEVEL	Process/ Activity
	DENOMINATOR	Fixed clinics plus fixed CHCs/CDCs	USE AND CONTEXT	Monitors whether PHC health establishments are measuring their own level of compliance with standards in order to close gaps in preparation for an external assessment by the Office of Health Standards Compliance.	KEY REPORTS	APP; DHP
Patient Experience of Care survey rate *	NUMERATOR	Facility Patient Experience of Care survey conducted	DENOMINATOR FORMULA	SUM([Fixed PHC clinics/fixed CHCs/CDCs] + [public hospitals])	TYPE	%
	NUMERATOR FORMULA	SUM([Facility Patient Experience of Care survey done])	DEFINITION	Fixed health facilities that have conducted Patient Experience of Care Surveys as a proportion of fixed health facilities	LEVEL	Process/ Activity
	DENOMINATOR	Fixed PHC clinics/fixed CHCs/CDCs plus public hospitals	USE AND CONTEXT	Monitors whether public health establishments are conducting Patient Experience of Care surveys	KEY REPORTS	MTSF 2014-2019
Patient Experience of Care Satisfaction rate *	NUMERATOR	Patient Experience of Care survey satisfied responses	DENOMINATOR FORMULA	SUM([Patient Experience of Care total responses])	TYPE	%
	NUMERATOR FORMULA	SUM([Patient Experience of Care satisfied responses])	DEFINITION	Satisfied responses as a proportion of all responses from Patient Experience of Care survey questionnaires	LEVEL	Process/ Activity
	DENOMINATOR	Patient Experience of Care survey total responses	USE AND CONTEXT	Monitors patient satisfaction with health services.	KEY REPORTS	APP, MTSF 2014-2019
Patient Safety Incident (PSI) case closure rate *	NUMERATOR	Patient Safety Incident (PSI) case closed	DENOMINATOR FORMULA	SUM([Patient Safety Incident (PSI) case reported])	TYPE	%
	NUMERATOR FORMULA	SUM([Patient Safety Incident (PSI) case closed])	DEFINITION	Patient Safety Incident (PSI) case closed in the reporting month as a proportion of Patient Safety Incident (PSI) cases reported in the reporting month	LEVEL	Process/ Activity
	DENOMINATOR	Patient Safety Incident (PSI) case reported	USE AND CONTEXT	Monitors the implementation of the National Policy to Manage Patient Safety Incidents.	KEY REPORTS	APP

Complaint resolution rate *	NUMERATOR	Complaints resolved	DENOMINATOR FORMULA	SUM([Complaint received])	TYPE	%
	NUMERATOR FORMULA	SUM([Complaint resolved])	DEFINITION	Complaints resolved as a proportion of complaints received	LEVEL	Output
	DENOMINATOR	Complaints received	USE AND CONTEXT	Monitors public health system response to customer concerns.	KEY REPORTS	APP; DHP
Complaint resolution within 25 working days rate *	NUMERATOR	Complaints resolved within 25 working days	DENOMINATOR FORMULA	SUM([Complaint resolved])	TYPE	%
	NUMERATOR FORMULA	SUM([Complaint resolved within 25 working days])	DEFINITION	Complaints resolved within 25 working days as a proportion of all complaints resolved	LEVEL	Output
	DENOMINATOR	Complaints resolved	USE AND CONTEXT	Monitors the timeframe in which the public health system responds to complaints.	KEY REPORTS	APP; DHP
Hospital achieved 75% and more on National Core Standards (NCS) self-assessment rate *	NUMERATOR	Hospital achieved 75% and more on National Core Standards self-assessment	DENOMINATOR FORMULA	SUM([Hospitals conducted National Core Standards self-assessment])	TYPE	%
	NUMERATOR FORMULA	SUM([Hospital achieved 75% and more on National Core Standards self-assessment])	DEFINITION	Hospitals that achieved a performance of 75% or more on National Core Standards self-assessment	LEVEL	Output
	DENOMINATOR	National Core Standards self-assessment	USE AND CONTEXT	Monitors whether public hospital establishments are measuring their own level of compliance with standards in order to close gaps in preparation for an external assessment by the Office of Health Standards Compliance.	KEY REPORTS	MTSF 2014-2019
Ideal Clinic (IC) status rate *	NUMERATOR	Ideal Clinic status	DENOMINATOR FORMULA	SUM([Fixed PHC clinics/fixed CHCs/CDCs])	TYPE	%
	NUMERATOR FORMULA	SUM([Ideal Clinic status])	DEFINITION	Fixed PHC health facilities have obtained Ideal Clinic status	LEVEL	Process/ Activity
	DENOMINATOR	Fixed PHC clinics/fixed CHCs/CDCs	USE AND CONTEXT	Monitors outcomes of self-assessments (Ideal Clinics) to ensure that they are ready for inspections conducted by the Office of Health Standards Compliance.	KEY REPORTS	MTSF 2014-2019

OHH WITH PREGNANCY CARE RATE	NUMERATOR	OHH with pregnancy care	DENOMINATOR FORMULA	(SUM([OHH follow-up visit]) + SUM([OHH registration visit]))	TYPE	%
	NUMERATOR FORMULA	(SUM([OHH follow-up visit]) + SUM([OHH registration visit]))	DEFINITION	Outreach households (OHH) visits during which antenatal care was provided to pregnant women as proportion of households visited by the Ward-based Outreach Team	LEVEL	Output
	DENOMINATOR	OHH visits – total	USE AND CONTEXT	Monitors households where care was provided to pregnant women, irrespective of the number of pregnant women cared for.	KEY REPORTS	None
OHH SUPERVISED VISIT RATE	NUMERATOR	OHH supervised visit	DENOMINATOR FORMULA	(SUM([OHH follow-up visit]) + SUM([OHH registration visit]))	TYPE	%
	NUMERATOR FORMULA	SUM([OHH supervised visit])	DEFINITION	Outreach household (OHH) visits accompanied by a supervisor as proportion of all household visits by Ward-based Outreach Teams	LEVEL	Process/ Activity
	DENOMINATOR	OHH visits – total	USE AND CONTEXT	Monitors supervision of ward-based community health workers.	KEY REPORTS	None
OHH FOLLOW-UP VISIT RATE	NUMERATOR	OHH follow-up visit	DENOMINATOR FORMULA	(SUM([OHH follow-up visit]) + SUM([OHH registration visit]))	TYPE	%
	NUMERATOR FORMULA	SUM([OHH follow-up visit])	DEFINITION	Outreach household follow-up visits as proportion of all households visits by Ward-based Outreach Teams	LEVEL	Process/ Activity
	DENOMINATOR	OHH visits – total	USE AND CONTEXT	Monitors follow-up visits to registered households. Community health workers are expected to visit each household for follow-up support monthly. At-risk households need to be visited more frequently.	KEY REPORTS	None
OHH REGISTRATION VISIT COVERAGE	NUMERATOR	OHH registration visit	DENOMINATOR FORMULA	DHIS formula	TYPE	%
	NUMERATOR FORMULA	SUM([OHH registration visit])	DEFINITION	Outreach households registered by Ward-based Outreach Teams as a proportion of OHH in population	LEVEL	Output
	DENOMINATOR	OHH in population	USE AND CONTEXT	Monitors implementation of the PHC Re-engineering Strategy.	KEY REPORTS	None

OHH CLIENT REFERRED TO FACILITY RATE	NUMERATOR	OHH client refer to facility	DENOMINATOR FORMULA	SUM([OHH headcount under 5 years]) + SUM([OHH headcount 5 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([OHH client refer to facility])	DEFINITION	Proportion of outreach household clients referred to health facilities	LEVEL	Process/ Activity
	DENOMINATOR	OHH headcount – total	USE AND CONTEXT	Monitors referral by community health workers (CHW) to health facilities. Individual clients referred are counted.	KEY REPORTS	None
OHH WITH HOME-BASED CARE RATE	NUMERATOR	OHH with home-based care	DENOMINATOR FORMULA	(SUM([OHH follow-up visit]) + SUM([OHH registration visit]))	TYPE	%
	NUMERATOR FORMULA	SUM([OHH with home-based care])	DEFINITION	Outreach households (OHH) with home-based support provided as proportion of households visited by the Ward-based Outreach Team	LEVEL	Output
	DENOMINATOR	OHH visits – total	USE AND CONTEXT	Monitors households where home-based care was provided, irrespective of the number of clients who received home-based care.	KEY REPORTS	None
OHH WITH ADHERENCE SUPPORT RATE	NUMERATOR	OHH with adherence support	DENOMINATOR FORMULA	(SUM([OHH follow-up visit]) + SUM([OHH registration visit]))	TYPE	%
	NUMERATOR FORMULA	SUM([OHH with adherence support])	DEFINITION	Outreach households (OHH) with adherence support provided as proportion of households visited by the Ward-based Outreach Team	LEVEL	Output
	DENOMINATOR	OHH visits – total	USE AND CONTEXT	Monitors households where adherence support was provided, irrespective of the number of clients who received support.	KEY REPORTS	None
OHH WITH CHILD UNDER 5 YEARS CARE RATE	NUMERATOR	OHH with child under 5 care	DENOMINATOR FORMULA	(SUM([OHH follow-up visit]) + SUM([OHH registration visit]))	TYPE	%
	NUMERATOR FORMULA	SUM([OHH with child under 5 years care])	DEFINITION	Outreach households (OHH) with basic health care provided to children under 5 years as proportion of households visited by the Ward-based Outreach Team	LEVEL	Output
	DENOMINATOR	OHH visits – total	USE AND CONTEXT	Monitors households where care was provided to children under 5, irrespective of the number of children under 5 cared for.	KEY REPORTS	None
OHH WITH POSTNATAL CARE RATE	NUMERATOR	OHH with postnatal care	DENOMINATOR FORMULA	(SUM([OHH follow-up visit]) + SUM([OHH registration visit]))	TYPE	%
	NUMERATOR FORMULA	SUM([OHH with postnatal care])	DEFINITION	Outreach households (OHH) with postnatal care provided to a mother and/or neonate within 6 days after delivery as proportion of households visited by the Ward-based Outreach Team	LEVEL	Output
	DENOMINATOR	OHH visits – total	USE AND CONTEXT	Monitors households where care was provided to postnatal women, irrespective of the number of postnatal women cared for.	KEY REPORTS	None

OHH HEADCOUNT 5 YEARS AND OLDER COVERAGE	NUMERATOR	OHH headcount 5 years and older	DENOMINATOR FORMULA	(SUM([Female 05-14 years]) + SUM([Male 05-14 years]) + SUM([Female 15-44 years]) + SUM([Male 15-44 years]) + SUM([Female 45 years and older]) + SUM([Male 45 years and older]))	TYPE	%
	NUMERATOR FORMULA	SUM([OHH headcount 5 years and older])	DEFINITION	Children 5 years and older in the population who received care during Ward-based Outreach Team visits	LEVEL	Output
	DENOMINATOR	Population 5 years and older	USE AND CONTEXT	Monitors proportion of population 5 years and older attended to during household visits.	KEY REPORTS	None
OHH HEADCOUNT UNDER 5 YEARS COVERAGE	NUMERATOR	OHH headcount under 5 years	DENOMINATOR FORMULA	(SUM([Female under 5 years]) + SUM([Male under 5 years]))	TYPE	%
	NUMERATOR FORMULA	SUM([OHH headcount under 5 years])	DEFINITION	Children under 5 years in population who received care during Ward-based Outreach Team visits as a proportion of population under 5 years	LEVEL	Output
	DENOMINATOR	Population under 5 years	USE AND CONTEXT	Monitors proportion of population under 5 years attended to during household visits.	KEY REPORTS	None
OHH REGISTRATION VISIT RATE	NUMERATOR	OHH registration visit	DENOMINATOR FORMULA	SUM([OHH allocated to team])	TYPE	%
	NUMERATOR FORMULA	SUM([OHH registration visit])	DEFINITION	Outreach household (OHH) registration visits as proportion households allocated to Wardbased Outreach Teams	LEVEL	Output
	DENOMINATOR	OHH allocated to team	USE AND CONTEXT	Monitors household registration. Each household allocated to a community health worker must be registered during the first outreach household visit, using the standardised Household Registration form.	KEY REPORTS	None
OHH CLIENT REFERRED TO HOME BASED CARE RATE	NUMERATOR	OHH client referred to home based care	DENOMINATOR FORMULA	SUM([OHH headcount under 5 years]) + SUM([OHH headcount 5 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([OHH client refer to home based care])	DEFINITION	Outreach households (OHH) clients referred to home-based care as a proportion of the total number of clients supported (total headcount) during outreach team visits	LEVEL	Process/ Activity
	DENOMINATOR	OHH headcount – total	USE AND CONTEXT	Monitors referral by community health workers (CHW) to home-based care. Individual clients referred are counted.	KEY REPORTS	None
OHH CLIENT REFERRED TO SOCIAL SERVICES RATE	NUMERATOR	OHH client referred to social services	DENOMINATOR FORMULA	SUM([OHH headcount under 5 years]) + SUM([OHH headcount 5 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([OHH client refer to social services])	DEFINITION	Outreach households (OHH) clients referred to social services as a proportion of the total number of clients supported (total headcount) during outreach team visits	LEVEL	Process/ Activity
	DENOMINATOR	OHH headcount – total	USE AND CONTEXT	Monitors referral by community health workers (CHW) to social services. Individual clients referred are counted.	KEY REPORTS	None

DENTAL UTILISATION RATE	NUMERATOR	Dental headcount total	DENOMINATOR FORMULA	SUM([Total population]) - SUM([Total population (MedicAid)])	TYPE	%
	NUMERATOR FORMULA	SUM([Dental headcount])	DEFINITION	Dental visits per 1 000 uninsured population	LEVEL	Output
	DENOMINATOR	Population uninsured total	USE AND CONTEXT	Monitors access to dental/oral health services. Each person should visit a dentist at least once a year.	KEY REPORTS	None
DENTAL EXTRACTION TO RESTORATION RATIO	NUMERATOR	Tooth extraction	DENOMINATOR FORMULA	SUM([Tooth restoration])	TYPE	No
	NUMERATOR FORMULA	SUM([Tooth extraction])	DEFINITION	The ratio between the number of teeth extracted and the number of teeth restored. The Ratio is 10:1.	LEVEL	Output
	DENOMINATOR	Tooth restoration	USE AND CONTEXT	Monitors overall quality of dental services. Poor quality dental services if many extractions and few restorations are reflected.	KEY REPORTS	None

MENTAL HEALTH SEPARATION RATE *	NUMERATOR	Mental health separations	DENOMINATOR FORMULA	SUM([Inpatient deaths - total]) + SUM([Inpatient discharges - total]) + SUM([Inpatient transfers out - total])	TYPE	%
	NUMERATOR FORMULA	SUM([Mental health separations under 18 years]) + SUM([Mental health separations 18 years and older])	DEFINITION	Proportion of clients admitted for mental health problems. Inpatient separations is the total of inpatient discharges, inpatient deaths and inpatient transfers out	LEVEL	Output
	DENOMINATOR	Inpatient separations – total	USE AND CONTEXT	Monitors trends in mental health admissions in non-mental health institutions. The data element 'Inpatient separations' is used as a proxy for admissions. Monitor in general hospitals only and NOT in mental health institutions.	KEY REPORTS	NSP; WHO

MENTAL HEALTH INVOLUNTARY ADMISSION RATE *	NUMERATOR	Mental health admission involuntary	DENOMINATOR FORMULA	SUM([Mental health admission under 18 years]) + SUM([Mental health admission 18 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([Mental health admission involuntary])	DEFINITION	Involuntary admissions as a proportion of all mental health admissions	LEVEL	Output
	DENOMINATOR	Mental health admission – total	USE AND CONTEXT	Monitors admissions in terms of the Mental Health Care Act (17 of 2002). The data elements should be collected at all hospitals with designated psychiatric beds or wards. Because of long stay, the data element Inpatient separations cannot serve as a proxy for admissions in mental health institutions.	KEY REPORTS	UN; CRPD

MENTAL DISORDERS SCREENING RATE *	NUMERATOR	PHC client screened for mental disorders	DENOMINATOR FORMULA	SUM([PHC headcount under 5 years]) + SUM([PHC headcount 5 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([PHC client screened for mental disorders])	DEFINITION	Clients screened for mental disorders (depression, anxiety, dementia, psychosis, mania, suicide, developmental disorders, behavioural disorders and substance use disorders at PHC facilities	LEVEL	Process/Activity
	DENOMINATOR	PHC headcount – total	USE AND CONTEXT	Monitors access to and quality of mental health services in PHC facilities.	KEY REPORTS	2016/17–2018/19 APP, Strategic Plan 2015/16–2019/20
MENTAL DISORDERS TREATMENT RATE NEW *	NUMERATOR	PHC client treated for mental disorders	DENOMINATOR FORMULA	SUM([PHC headcount under 5 years]) + SUM([PHC headcount 5 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([PHC client treated for mental disorders])	DEFINITION	Clients treated for mental disorders (depression, anxiety, dementia, psychosis, mania, suicide, developmental disorders, behavioural disorders and substance use) as a proportion of total PHC headcount	LEVEL	Output
	DENOMINATOR	PHC headcount – total	USE AND CONTEXT	Monitors access to mental health services	KEY REPORTS	2016/17–2018/19 APP, Strategic Plan 2015/16–2019/20
MENTAL HEALTH CASE LOAD *	NUMERATOR	Mental health visits – total (This was clients before but changed to exclude possibility that clients on register will be counted and to be in line with hypertension and diabetes caseload definition.)	DENOMINATOR FORMULA	SUM([PHC headcount 5 years and older]) + SUM([PHC headcount under 5 years])	TYPE	%
	NUMERATOR FORMULA	SUM([Mental health client 18 years and older]) + SUM([Mental health client under 18 years])	DEFINITION	Mental health care visits as proportion of total PHC headcount	LEVEL	Process/Activity
	DENOMINATOR	PHC headcount – total	USE AND CONTEXT	Monitors access to and utilisation of mental health services.	KEY REPORTS	NSP; WHO

Infant initiated on CPT around 6 weeks uptake rate	NUMERATOR	Infant initiated on CPT around 6 weeks	DENOMINATOR FORMULA	SUM([Live birth to HIV-positive woman])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant initiated on CPT around 6 weeks])	DEFINITION	Infants initiated on Co-trimoxazole (CPT) around 6 weeks after birth (to prevent opportunistic infections) as proportion of live births to HIV-positive women	LEVEL	Output
	DENOMINATOR	Live birth to HIV-positive woman	USE AND CONTEXT	Monitors implementation of the PMTCT guidelines in terms of Co-trimoxazole prophylaxis (CPT) for HIV-exposed infants.	KEY REPORTS	DORA; NSP; GAR; UA
Infant 1st PCR test at birth uptake rate	NUMERATOR	Infant 1st PCR test at birth	DENOMINATOR FORMULA	SUM([Live birth to HIV-positive woman])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant PCR test at birth])	DEFINITION	Infants PCR tested for the first time at birth as proportion of live births to HIV-positive women	LEVEL	Output
	DENOMINATOR	Live birth to HI- positive woman	USE AND CONTEXT	Monitors implementation of PMTCT guidelines in terms of tracking PCR positivity in infants at birth.	KEY REPORTS	DORA; NSP; GAR; UA
Infant 1st PCR test positive at birth rate	NUMERATOR	Infant 1st PCR test positive at birth	DENOMINATOR FORMULA	SUM([Infant PCR test at birth])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant PCR test positive at birth])	DEFINITION	Infants tested PCR-positive for the first time at birth as proportion of infants PCR tested at birth	LEVEL	Outcome
	DENOMINATOR	Infant 1st PCR test at birth	USE AND CONTEXT	Monitors PCR positivity rate in HIV-exposed infants at birth.	KEY REPORTS	DORA; NSP; GAR; UA, SDG3 Proxy
Infant PCR test around 10 weeks rate	NUMERATOR	Infant PCR test around 10 weeks	DENOMINATOR FORMULA	SUM([Live birth to HIV-positive woman]) - SUM([Infant PCR test positive at birth])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant PCR test around 10 weeks])	DEFINITION	Infants PCR tested around 10 weeks as a proportion of HIV-exposed infants	LEVEL	Output
	DENOMINATOR	Live birth to HIV-positive woman - Infant 1st PCR test positive at birth (10 weeks lag)	USE AND CONTEXT	Monitors implementation of PMTCT guidelines in terms of follow-up PCR testing of HIV-exposed Infants around 10 weeks (between 8 and 10 weeks).	KEY REPORTS	DORA; NSP; GAR; UA, SDG3 Proxy
Infant PCR test positive around 10 weeks rate	NUMERATOR	Infant PCR test positive around 10 weeks	DENOMINATOR FORMULA	SUM([Infant PCR test around 10 weeks])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant PCR test positive around 10 weeks])	DEFINITION	Infants tested PCR positive for follow-up test as a proportion of Infants PCR tested around 10 weeks	LEVEL	Outcome
	DENOMINATOR	Infant PCR test around 10 weeks	USE AND CONTEXT	Monitors PCR positivity rate in HIV-exposed infants around 10 weeks.	KEY REPORTS	APP; DORA; NSP; GAR; UA, SDG3 Proxy

Perinatal mortality in facility rate	**NUMERATOR**	Stillbirth plus Death 0-7 days in facility	**DENOMINATOR FORMULA**	(SUM([Female under 1 year]) + SUM([Male under 1 year])) * 1.03	**TYPE**	%
	NUMERATOR FORMULA	SUM([Stillbirth in facility]) + SUM([Inpatient death 0-7 days])	**DEFINITION**	Stillbirths and deaths in facility within the first 7 days of life (Early Neonatal Death) per 1 000 births	**LEVEL**	Impact
	DENOMINATOR	Stillbirth in facility + Live birth in facility	**USE AND CONTEXT**	Monitors trends in neonatal mortality in health facilities between official surveys. Indication of health system results in terms of antenatal, delivery and neonatal care.	**KEY REPORTS**	SGD3 proxy
Early neonatal death in facility rate	**NUMERATOR**	Death in facility 0-7 days	**DENOMINATOR FORMULA**	(SUM([Female under 1 year]) + SUM([Male under 1 year])) * 1.03	**TYPE**	%
	NUMERATOR FORMULA	SUM([Inpatient death 0-7 days])	**DEFINITION**	Early neonatal deaths (0-7 days) as a proportion of infants who were born alive in health facilities	**LEVEL**	Impact
	DENOMINATOR	Live birth in facility	**USE AND CONTEXT**	Monitors trends in early neonatal deaths in health facilities. Indication of health system results in terms of antenatal, delivery and early neonatal care.	**KEY REPORTS**	SGD3 proxy; MTSF 2014–2019
Neonatal death in facility rate	**NUMERATOR**	Neonatal 0-28 days death in facility	**DENOMINATOR FORMULA**	SUM([Live birth in facility])	**TYPE**	%
	NUMERATOR FORMULA	SUM([Inpatient death 0–7 days]) + SUM([Inpatient death 8–28 days])	**DEFINITION**	Infants 0–28 days who died during their stay in the facility as a proportion of live births in facility	**LEVEL**	Impact
	DENOMINATOR	Live birth in facility	**USE AND CONTEXT**	None	**KEY REPORTS**	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20, SDG3 Proxy
Infant given NVP within 72 hours after birth uptake rate	**NUMERATOR**	Infant given NVP within 72 hours after birth	**DENOMINATOR FORMULA**	SUM([Live birth to HIV-positive woman])	**TYPE**	%
	NUMERATOR FORMULA	SUM([Infant given Nevirapine within 72 hours after birth])	**DEFINITION**	Infants given Nevirapine (NVP) within 72 hours of birth as proportion of live births to HIV-positive women	**LEVEL**	Output
	DENOMINATOR	Live birth to HIV-positive woman	**USE AND CONTEXT**	Monitors implementation of the PMTCT guidelines in terms of NVP for HIV-exposed infants.	**KEY REPORTS**	DORA; NSP; GAR; UA

Delivery in 10 to 19 years in facility rate	NUMERATOR	Delivery 10 to 19 years years in facility	DENOMINATOR FORMULA	SUM([Delivery in facility total])	TYPE	%
	NUMERATOR FORMULA	[Delivery 10–14 years in facility] + [Delivery 15–19 years in facility]	DEFINITION	Deliveries to women under the age of 20 years as proportion of total deliveries in health facilities	LEVEL	Outcome
	DENOMINATOR	Delivery in facility – total	USE AND CONTEXT	Monitors the proportion of deliveries in facility by teenagers (young women under 20 years).	KEY REPORTS	SDG3 Proxy; MTSF 2014–2019
Delivery by Caesarean section rate	NUMERATOR	Delivery by Caesarean section	DENOMINATOR FORMULA	SUM([Delivery in facility total])	TYPE	%
	NUMERATOR FORMULA	SUM([Delivery by Caesarean section])	DEFINITION	Delivery by Caesarean section as proportion of total deliveries in health facilities	LEVEL	Output
	DENOMINATOR	Delivery in facility – total	USE AND CONTEXT	Monitors access to Caesarean section (and other surgery) as well as use of resources at delivery facilities. Differs between types of hospitals, for example 2011/12 values for DH:15% RH:25% TH: 30% CH: 50%.	KEY REPORTS	SDG3 proxy
Stillbirth in facility rate	NUMERATOR	Stillbirth in facility	DENOMINATOR FORMULA	SUM([Live birth in facility]) + SUM([Stillbirth in facility])	TYPE	%
	NUMERATOR FORMULA	SUM([Stillbirth in facility])	DEFINITION	Infants born still as proportion of total infants born in health facilities	LEVEL	Impact
	DENOMINATOR	Total births in facility	USE AND CONTEXT	Monitors stillbirth trends.	KEY REPORTS	WHO World Health Statistics
Live birth under 2 500g in facility rate	NUMERATOR	Live birth under 2 500g in facility	DENOMINATOR FORMULA	SUM([Live birth in facility])	TYPE	%
	NUMERATOR FORMULA	SUM([Live birth under 2 500g in facility])	DEFINITION	Infants born alive weighing less than 2 500g as proportion of total Infants born alive in health facilities (Low birth weight)	LEVEL	Outcome
	DENOMINATOR	Live birth in facility	USE AND CONTEXT	Monitors trends in low birth weight.	KEY REPORTS	SDG3 Proxy
Born alive before arrival at facility rate	NUMERATOR	Born alive before arrival at facility	DENOMINATOR FORMULA	SUM([Live birth in facility]) + SUM([Born alive before arrival at facility])	TYPE	%
	NUMERATOR FORMULA	SUM([Born alive before arrival at facility])	DEFINITION	Infants born alive before arriving at facility as a proportion of total live births (infants born in the facility and before arriving at the facility)	LEVEL	Outcome
	DENOMINATOR	Live birth in facility plus Born alive before arrival at facility	USE AND CONTEXT	None	KEY REPORTS	None

MATERNAL MORTALITY IN FACILITY RATIO	NUMERATOR	Maternal death in facility	DENOMINATOR FORMULA	SUM([Live birth in facility]) + SUM([Born alive before arrival at facility])	TYPE	%
	NUMERATOR FORMULA	SUM([Maternal death in facility])	DEFINITION	Maternal death is death occurring during pregnancy, childbirth and the puerperium of a woman while pregnant or within 42 days of termination of pregnancy, irrespective of the duration and site of pregnancy and irrespective of the cause of death (obstetric and non-obstetric) per 100 000 live births in facility	LEVEL	Impact
	DENOMINATOR	Live births known to facility (Live birth in facility plus Born alive before arrival at facility)	USE AND CONTEXT	This is a proxy for the population-based maternal mortality ratio, aimed at monitoring trends in health facilities between official surveys. Focuses on obstetric causes (around 30% of all maternal mortality). Provides indication of health system results in terms of prevention of unplanned pregnancies, antenatal care, delivery and postnatal services.	KEY REPORTS	APP; MDG; MTSF 2014–2019; WHO World Health Statistics; Strategic Plan 2015/16–2019/20, SDG3 proxy
MOTHER POSTNATAL VISIT WITHIN 6 DAYS RATE	NUMERATOR	Mother postnatal visit within 6 days after delivery	DENOMINATOR FORMULA	SUM([Delivery in facility total])	TYPE	%
	NUMERATOR FORMULA	SUM([Mother postnatal visit within 6 days after delivery])	DEFINITION	Mothers who received postnatal care within 6 days after delivery as proportion of deliveries in health facilities	LEVEL	Process/Activity
	DENOMINATOR	Delivery in facility – total	USE AND CONTEXT	Monitors access to and utilisation of postnatal services. May be more than 100% in areas with low delivery in facility rates if many mothers who delivered outside health facilities used postnatal visits within 6 days after delivery.	KEY REPORTS	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
DELIVERY IN FACILITY RATE	NUMERATOR	Delivery in facility - total	DENOMINATOR FORMULA	(SUM([Female under 1 year]) + SUM([Male under 1 year])) * 1.07	TYPE	%
	NUMERATOR FORMULA	SUM([Delivery in facility total])	DEFINITION	Deliveries in health facilities as proportion of expected deliveries in the population. Expected deliveries are estimated as population under 1 year multiplied by 1.025 to compensate for stillbirths and infant mortality.	LEVEL	Outcome
	DENOMINATOR	Population estimated deliveries	USE AND CONTEXT	Monitors accessibility of maternity and obstetric services. Born Before Arrivals (BBAs) are not included here.	KEY REPORTS	SDG3 proxy
TOTAL BIRTHS IN FACILITY	NUMERATOR	Sum of live births in facility and stillbirths in facility	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([Live birth in facility]) + SUM([Stillbirth in facility])	DEFINITION	Sum of live births in facility and stillbirths in facility	LEVEL	Process/Activity
	DENOMINATOR	None	USE AND CONTEXT	None	KEY REPORTS	SDG3 proxy

Antenatal client HIV re-test rate	NUMERATOR	Antenatal client HIV re-test	DENOMINATOR FORMULA	SUM([Antenatal client HIV 1st test]) - SUM([Antenatal client HIV 1st test positive])	TYPE	%
	NUMERATOR FORMULA	SUM([Antenatal client HIV re-test])	DEFINITION	Antenatal clients re-tested for HIV as proportion of antenatal clients tested negative for 1st HIV tests done during current pregnancy	LEVEL	Process/Activity
	DENOMINATOR	Antenatal client HIV 1st test negative	USE AND CONTEXT	Monitors implementation of PMTCT guidelines in terms of HIV re-testing. Used as a proxy indicator as it is not cost-effective to monitor cohorts with paper-based systems. Note: Antenatal client HIV 1st test negative used in the denominator has been reported 3 months (12 weeks) earlier.	KEY REPORTS	DORA; NSP; GAR; UA; HADACC:36, SDG3 proxy
Antenatal client HIV re-test positive rate	NUMERATOR	Antenatal client HIV re-test positive	DENOMINATOR FORMULA	SUM([Antenatal client HIV re-test])	TYPE	%
	NUMERATOR FORMULA	SUM([Antenatal client HIV re-test positive])	DEFINITION	Antenatal clients re-tested positive for HIV as proportion of antenatal clients re-tested for HIV	LEVEL	Outcome
	DENOMINATOR	Antenatal client HIV re-test	USE AND CONTEXT	Monitors HIV infection during pregnancy.	KEY REPORTS	DORA; NSP; GAR; UAS,DG3 proxy
Antenatal client start on ART rate	NUMERATOR	Antenatal client start on ART	DENOMINATOR FORMULA	Antenatal client known HIV-positive but NOT on ART at 1st visit + Antenatal client HIV 1st test positive + Antenatal client HIV re-test positive	TYPE	%
	NUMERATOR FORMULA	SUM([Antenatal client start on ART])	DEFINITION	Antenatal clients who started on ART as a proportion of the total number of antenatal clients who are HIV-positive and not previously on ART	LEVEL	Output
	DENOMINATOR	Antenatal client known HIV-positive but NOT on ART at 1st visit	USE AND CONTEXT	Monitors implementation of PMTCT guidelines in terms of ART initiation of eligible HIV-positive antenatal clients. Until 1 April 2013, the criteria for ART initiation for antenatal clients were: HIV-positive antenatal client with a CD4 count under the specified threshold and/or a WHO staging of 4. Since 1 April 2013, all HIV-positive antenatal clients who are not already on ART are eligible for the ART Fixed-dose Combination (FDC). Since 1 January 2015, all HIV-positive antenatal clients go onto lifelong treatment regardless of their CD4 status.	KEY REPORTS	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20, SDG3 Proxy

ANTENATAL 1ST VISIT BEFORE 20 WEEKS RATE	NUMERATOR	Antenatal 1st visit before 20 weeks	DENOMINATOR FORMULA	SUM([Antenatal 1st visit 20 weeks or later]) + SUM([Antenatal 1st visit before 20 weeks])	TYPE	%
	NUMERATOR FORMULA	SUM([Antenatal 1st visit before 20 weeks])	DEFINITION	Women who have a booking visit (first visit) before they are 20 weeks into their pregnancy as proportion of all antenatal 1st visits	LEVEL	Process/Activity
	DENOMINATOR	Antenatal 1st visit – total	USE AND CONTEXT	Monitors early utilisation of antenatal services.	KEY REPORTS	APP, MTSF 2014-2019, Strategic Plan 2015/16-2019/20
ANTENATAL 1ST VISIT COVERAGE	NUMERATOR	Antenatal 1st visit - total	DENOMINATOR FORMULA	(SUM([Female under 1 year]) + SUM([Male under 1 year])) * 1.15	TYPE	%
	NUMERATOR FORMULA	SUM([Antenatal 1st visit 20 weeks or later]) + SUM([Antenatal 1st visit before 20 weeks])	DEFINITION	The proportion of potential antenatal clients coming for at least one (booking) antenatal visit. The census number of children under one year factorised by 1.15 is used as a proxy denominator – the extra 0.15 (15%) is a rough estimate to cater for late miscarriages (~10 to 26 w), stillbirths (after 26 weeks gestation) and infant mortality. Pregnant women are regarded as potential antenatal clients from around 10 weeks' gestation, i.e. spontaneous abortions before that as well as ToP cases are excluded.	LEVEL	Process/Activity
	DENOMINATOR	Estimated pregnant women ~10 weeks' gestation	USE AND CONTEXT	Monitors access to and utilisation of antenatal services.	KEY REPORTS	None
ANTENATAL CLIENT ALREADY ON ART AT 1ST VISIT RATE	NUMERATOR	Antenatal client already on ART at 1st visit	DENOMINATOR FORMULA	Antenatal client HIV-positive	TYPE	%
	NUMERATOR FORMULA	SUM([Antenatal client already on ART at 1st visit])	DEFINITION	Antenatal clients already on ART at first visit as a proportion of antenatal clients HIV-positive	LEVEL	Process/Activity
	DENOMINATOR	Antenatal client HIV positive	USE AND CONTEXT	This element indicates the women who fell pregnant while on ART.	KEY REPORTS	SDG3 Proxy
ANTENATAL CLIENT HIV 1ST TEST POSITIVE RATE	NUMERATOR	Antenatal client HIV 1st test positive	DENOMINATOR FORMULA	SUM([Antenatal client HIV 1st test])	TYPE	%
	NUMERATOR FORMULA	SUM([Antenatal client HIV 1st test positive])	DEFINITION	Antenatal clients tested HIV-positive as proportion of antenatal clients HIV tested for the first time during current pregnancy	LEVEL	Outcome
	DENOMINATOR	Antenatal client HIV 1st test	USE AND CONTEXT	Monitors trends in HIV test positivity of antenatal clients and initiation of HIV-positive antenatal clients on ART.	KEY REPORTS	DORA; NSP; GAR; UA, SDG3 Proxy

PHC PROFESSIONAL NURSE CLINICAL WORKLOAD	NUMERATOR	PHC clients seen by professional nurse	DENOMINATOR FORMULA	SUM([PHC professional nurse clinical work days])	TYPE	No
	NUMERATOR FORMULA	SUM([PHC client seen by professional nurse])	DEFINITION	Average number of clients seen per professional nurse per professional nurse clinical work day	LEVEL	Input
	DENOMINATOR	PHC professional nurse clinical work days	USE AND CONTEXT	Monitors staff allocation/management and provides indirect indication of quality of care. A low workload may be due to inefficient staff utilisation, and a very high workload indicates too little time spent per client. Do not include delivery service time in numerator or denominator.	KEY REPORTS	None

PHC DOCTOR CLINICAL WORKLOAD	NUMERATOR	PHC clients seen by doctor	DENOMINATOR FORMULA	SUM([PHC doctor clinical work days – total])	TYPE	No
	NUMERATOR FORMULA	SUM([PHC client seen by doctor])	DEFINITION	Average number of clients seen per doctor per clinical work day. This includes doctors employed in the public and private sector	LEVEL	Input
	DENOMINATOR	PHC doctor clinical work days	USE AND CONTEXT	Monitors staff allocation/management and provides indirect indication of quality of client care. A low workload may be due to inefficient staff utilisation, and a very high workload indicates too little time spent per client. Do not include delivery service time in numerator or denominator.	KEY REPORTS	None

PHC UTILISATION UNDER 5 YEARS RATE	NUMERATOR	PHC headcount under 5 years	DENOMINATOR FORMULA	SUM([Female under 5 years]) + SUM([Male under 5 years])	TYPE	No
	NUMERATOR FORMULA	SUM([PHC headcount under 5 years])	DEFINITION	Average number of PHC visits per year per person under 5 years of age in the population	LEVEL	Output
	DENOMINATOR	Population under 5 years	USE AND CONTEXT	Monitors PHC access and utilisation by children under 5 years of age.	KEY REPORTS	APP; DHP
PHC PRIVATE DOCTOR CLINICAL WORK LOAD	NUMERATOR	PHC client seen by private doctor	DENOMINATOR FORMULA	SUM([PHC private doctor clinical work days])	TYPE	No
	NUMERATOR FORMULA	SUM([PHC client seen by private doctor])	DEFINITION	Average number of clients seen per private doctor per clinical work day. This includes doctors employed in the public and private sector. Monitors the extent of effective utilisation of private doctors appointed on contract to consult clients in public sector facilities in accordance with NHI objectives to increase doctor coverage.	LEVEL	Input
	DENOMINATOR	PHC private doctor clinical work days	USE AND CONTEXT	Monitors staff allocation/management and provides indirect indication of quality of client care. Low workload may be due to inefficient staff utilisation, and very high workload indicates too little time spent per client. Do not include delivery service time in numerator or denominator.	KEY REPORTS	APP; DHP
TRACER ITEMS STOCK-OUT RATE (FIXED CLINIC/CHC/CDC)	NUMERATOR	Any tracer item drug stock-out (clinic/CHC/CDC)	DENOMINATOR FORMULA	SUM([Fixed clinic]) + SUM([Fixed CHC/CDC])	TYPE	%
	NUMERATOR FORMULA	SUM([Any tracer item drug stock-out (clinic/CHC/CDC)])	DEFINITION	The proportion of all fixed clinics, CHCs and CDCs that had stock-out of ANY tracer item for any period as a proportion of fixed clinics plus fixed CHCs/CDCs	LEVEL	Process/ Activity
	DENOMINATOR	Fixed clinics plus fixed CHCs/CDCs	USE AND CONTEXT	Fixed clinics plus fixed CHCs/CDCs	KEY REPORTS	None
PHC UTILISATION RATE – TOTAL	NUMERATOR	Sum of PHC headcount breakdowns (under 5 years, 5–9 years, 10–19 years, 20 years and older)	DENOMINATOR FORMULA	Sum([Population – Total)]	TYPE	No
	NUMERATOR FORMULA	SUM ([PHC headcount under 5 years] + [PHC headcount 5–9 years] + [PHC headcount 10–19 years] + [PHC headcount 20 years and older])	DEFINITION	Average number of PHC visits per year per person in the population	LEVEL	Output
	DENOMINATOR	Population – Total	USE AND CONTEXT	Monitors PHC access and utilisation	KEY REPORTS	None

Inpatient bed utilisation rate						
	NUMERATOR	Inpatient days + 1/2 Day patients	DENOMINATOR FORMULA	DHIS formula	TYPE	%
	NUMERATOR FORMULA	Sum ([Inpatient days total x 1]) + ([Day patient total x 0.5])	DEFINITION	Inpatient bed days used as proportion of maximum Inpatient bed days (inpatient beds x days in period) available. Include all specialities.	LEVEL	Output
	DENOMINATOR	Inpatient bed days available	USE AND CONTEXT	Monitors effectiveness and efficiency of inpatient management.	KEY REPORTS	APP; DHP

Inpatient separations	NUMERATOR	Sum of inpatient deaths, inpatient discharges and inpatient transfers out	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([inpatient deaths-total]) + ([inpatient discharges-total]) + ([inpatient transfers out-total])	DEFINITION	Sum of inpatient deaths, inpatient discharges and inpatient transfers out. Include all specialities. The total is specialities plus others that do not appear on the identified specialities.	LEVEL	Outcome
	DENOMINATOR	None	USE AND CONTEXT	None	KEY REPORTS	None
Patient Day Equivalent	NUMERATOR	The sum of Inpatient days total x 1, Day patient total x 0.5, and OPD/Emergency total headcount x 0.3333333	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	Sum ([Inpatient days total x 1]) + ([Day patient total x 0.5]) + ([OPD headcount not referred new x 0.3333333]) + SUM([OPD headcount referred new x 0.3333333]) + ([OPD headcount follow-up x 0.3333333]) + ([Emergency headcount - total x 0.3333333])	DEFINITION	The sum of Inpatient days total x 1, Day patient total x 0.5, and OPD/Emergency total headcount x 0.3333333. The total is specialities plus others that do not appear on the identified specialities.	LEVEL	Process/ Activity
	DENOMINATOR	None	USE AND CONTEXT	Weighted data element as proxy for estimating resources for all types of patients in terms of inpatient days	KEY REPORTS	None
Average length of stay	NUMERATOR	Inpatient days + 1/2 Day patients	DENOMINATOR FORMULA	SUM([inpatient deaths-total]) + ([inpatient discharges-total]) + ([inpatient transfers out-total])	TYPE	No
	NUMERATOR FORMULA	Sum ([Inpatient days total x 1]) + ([Day patient total x 0.5])	DEFINITION	The average number of client days an admitted client spends in hospital before separation. Inpatient separation is the total of day clients, Inpatient discharges, Inpatient deaths and Inpatient transfers out. Include all specialities.	LEVEL	Outcome
	DENOMINATOR	Inpatient separations	USE AND CONTEXT	Monitors effectiveness and efficiency of inpatient management. Proxy indicator because ideally it should only include inpatient days for those clients separated during the reporting month. Use in all hospitals and CHCs with inpatient beds.	KEY REPORTS	APP; DHP; OECD

OPD NEW CLIENT NOT REFERRED RATE	NUMERATOR	OPD headcount not referred new	DENOMINATOR FORMULA	SUM([OPD headcount not referred new]) + SUM([OPD headcount referred new])	TYPE	%
	NUMERATOR FORMULA	SUM([OPD headcount not referred new])	DEFINITION	New OPD clients not referred as a proportion of OPD new clients – total	LEVEL	Output
	DENOMINATOR	OPD new clients – total	USE AND CONTEXT	Monitors utilisation trends of clients by-passing PHC facilities and the effect of PHC re-engineering on OPD utilisation. Do not include OPD follow-up and emergency clients in the denominator.	KEY REPORTS	None
OPD HEADCOUNT – TOTAL	NUMERATOR	Sum of OPD headcount not referred new, referred new and follow up	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([OPD headcount not referred new]) + SUM([OPD headcount referred new]) + ([OPD headcount follow-up])	DEFINITION	Total clients attending general or specialist Outpatient clinics including new and follow-up visits	LEVEL	Output
	DENOMINATOR	None	USE AND CONTEXT	None	KEY REPORTS	None
EXPENDITURE PER PDE	NUMERATOR	Expenditure – total	DENOMINATOR FORMULA	Sum ([Inpatient days total x 1]) + ([Day patient total x 0.5]) + ([OPD headcount not referred new x 0.3333333]) + SUM([OPD headcount referred new x 0.3333333]) + ([OPD headcount follow-up x 0.3333333]) + ([Emergency headcount - total x 0.3333333])	TYPE	%
	NUMERATOR FORMULA	SUM([Expenditure - total])	DEFINITION	Average cost per patient day equivalent (PDE). PDE is the Inpatient days total + Day Patients * 0.5 + (Emergency headcount + OPD headcount total) * 0.33333333	LEVEL	Outcome
	DENOMINATOR	Patient day equivalent	USE AND CONTEXT	Monitors effective and efficient management of inpatient facilities. Note that multiplied by 0.5 is the same as division by 2, and multiplied by 0.33333333 is the same as division by 3.	KEY REPORTS	None
INPATIENT CRUDE DEATH RATE	NUMERATOR	Inpatient deaths – total	DENOMINATOR FORMULA	SUM([inpatient deaths-total]) + ([inpatient discharges-total]) + ([inpatient transfers out-total])	TYPE	%
	NUMERATOR FORMULA	SUM([Inpatient deaths – total])	DEFINITION	Clients who died during hospital stay as a proportion of inpatient separations – total. Inpatient separations is the total of day clients, Inpatient discharges, Inpatient deaths and Inpatient transfers out. Include all specialities.	LEVEL	Outcome
	DENOMINATOR	Inpatient separations – total	USE AND CONTEXT	Monitors trends in inpatient deaths. Provides an indication of the quality of inpatient care.	KEY REPORTS	None

MALARIA CASES REPORTED	NUMERATOR	Malaria case reported	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([Malaria case reported])	DEFINITION	Parasitological-confirmed malaria cases reported in South Africa (includes both local transmission and imported cases)	LEVEL	Outcome
	DENOMINATOR	None	USE AND CONTEXT	Monitors the progress of South Africa towards elimination of malaria cases.	KEY REPORTS	APP, WHO, SADC, Strategic Plan 2015/16–2019/20, SDG3 Proxy

MALARIA DEATHS REPORTED	NUMERATOR	Malaria death reported	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([Malaria death reported])	DEFINITION	Malaria deaths reported in South Africa. The death resulting from primary malaria diagnosis at the time of death.	LEVEL	Outcome
	DENOMINATOR	None	USE AND CONTEXT	Monitors the progress of South Africa towards elimination of malaria cases.	KEY REPORTS	APP, WHO, SADC, Strategic Plan 2015/16–2019/20, SDG3 Proxy

HIV-POSITIVE CHILD 12–59 MONTHS START ART RATE	NUMERATOR	Child 12-59 months naive started ART	DENOMINATOR FORMULA	SUM([Infant rapid HIV test positive around 18 months]) + SUM([HIV test positive child 19-59 months])	TYPE	%
	NUMERATOR FORMULA	SUM([[ART child 1 to under 5 years naïve started ART])	DEFINITION	Children 12 to 59 months naive started ART as a proportion of children 12 to 59 months who tested HIV-positive	LEVEL	Output
	DENOMINATOR	HIV test positive 12-59 months	USE AND CONTEXT	Monitors what proportion of children who tested HIV-positive have been started on ART.	KEY REPORTS	None
HIV TEST POSITIVE 5-14 YEARS RATE	NUMERATOR	HIV test positive 5–14 years	DENOMINATOR FORMULA	SUM([HIV test 5–14 years])	TYPE	%
	NUMERATOR FORMULA	SUM([HIV test positive 5–14 years])	DEFINITION	Children 5 to 14 years who tested HIV-positive as a proportion of children who were tested for HIV in this age group	LEVEL	Outcome
	DENOMINATOR	HIV test child 5–14 years	USE AND CONTEXT	HIV counselling and testing services for children 5 to 14 years as stipulated in the Integrated Management of Childhood Illness (IMCI) guidelines. It also monitors the impact of the pandemic on children to assist in planning for effective strategies in combating HIV and AIDS.	KEY REPORTS	MDG; MTSF 2014–2019
HIV-POSITIVE CHILD 5–14 YEARS START ART RATE	NUMERATOR	Child 5–14 years naive started ART	DENOMINATOR FORMULA	SUM([HIV test positive child 5-14 years])	TYPE	%
	NUMERATOR FORMULA	SUM([ART child 5 to 14 years naïve started ART])	DEFINITION	Children 5 to 14 years naive started ART as a proportion of children 5 to 14 years who tested HIV-positive	LEVEL	Output
	DENOMINATOR	HIV test positive child 5–14 years	USE AND CONTEXT	Monitors what proportion of children who tested HIV-positive have been started on ART.	KEY REPORTS	None
HIV TEST POSITIVE CLIENT 15 YEARS AND OLDER RATE (INCLUDING ANC)	NUMERATOR	HIV test positive client 15 years and older (incl ANC)	DENOMINATOR FORMULA	SUM([HIV test client 15 years and older (incl ANC)])	TYPE	%
	NUMERATOR FORMULA	SUM([HIV test positive client 15 years and older (incl ANC)])	DEFINITION	Clients 15 years and older (including antenatal) who tested HIV-positive as a proportion of all clients 15 years and older (including antenatal) on whom a HIV test was done	LEVEL	Outcome
	DENOMINATOR	HIV test client 15 years and older (incl ANC)	USE AND CONTEXT	Monitors the impact of the pandemic and assists in better planning for effective combating of HIV and AIDS and decreasing the burden of disease from TB.	KEY REPORTS	SDG3 Proxy

HIV-POSITIVE CHILD UNDER 1 YEAR START ART RATE	NUMERATOR	Child under 1 year naive started ART	DENOMINATOR FORMULA	SUM([Infant PCR test positive at birth]) + ([Infant PCR test positive around 10 weeks])	TYPE	%
	NUMERATOR FORMULA	sum([Child under 1 year naive started ART])	DEFINITION	Children under 1 year naive started ART as a proportion of infants who tested positive on the 1st PCR test	LEVEL	Output
	DENOMINATOR	PCR test positive under 1 year: Infant PCR test positive at birth PLUS Infant PCR test positive around 10 weeks	USE AND CONTEXT	Monitors what proportion of children who tested HIV-positive have been started on ART	KEY REPORTS	None
HIV TEST AROUND 18 MONTHS UPTAKE RATE	NUMERATOR	Infant rapid HIV test around 18 months	DENOMINATOR FORMULA	SUM([Live birth to HIV-positive woman])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant rapid HIV test around 18 months])	DEFINITION	Infant rapid HIV test around 18 months after birth as the proportion of infants under 18 months. The denominator is collected 18 months before the numerator thus the indicator formula uses Live birth to HIV-positive woman recorded 18 months ago in order to provide reliable output.	LEVEL	Output
	DENOMINATOR	Live birth to HIV-positive woman	USE AND CONTEXT	Monitors the HIV sero-conversion of HIV-exposed infants after 18 months. Note: Live birth to HIV-positive woman used in the denominator has been reported 18 months earlier.	KEY REPORTS	None
HIV TEST AROUND 18 MONTHS POSITIVE RATE	NUMERATOR	Infant rapid HIV test positive around 18 months	DENOMINATOR FORMULA	SUM([Infant rapid HIV test around 18 months])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant rapid HIV test positive around 18 months])	DEFINITION	Infants tested positive for HIV antibodies around 18 months after birth as the proportion of Infants tested for HIV antibodies around 18 months	LEVEL	Outcome
	DENOMINATOR	Infant rapid HIV test around 18 months	USE AND CONTEXT	Monitors the HIV sero-conversion of HIV-exposed infants after 18 months.	KEY REPORTS	None
HIV TEST POSITIVE 12–59 MONTHS RATE	NUMERATOR	HIV test positive 12–59 months	DENOMINATOR FORMULA	SUM([Infant rapid HIV test around 18 months]) + SUM([HIV test child 19-59 months])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant rapid HIV test positive around 18 months]) + SUM([HIV test positive child 19–59 months])	DEFINITION	Children 12 to 59 months who tested HIV-positive as a proportion of children who were tested for HIV in this age group	LEVEL	Outcome
	DENOMINATOR	HIV test 12–59 months	USE AND CONTEXT	Monitors HIV counselling and testing services as stipulated in the Integrated Management of Childhood Illness (IMCI) guidelines. It also monitors the impact of the pandemic on children to assist in planning for effective strategies in combating HIV and AIDS.	KEY REPORTS	MDG; MTSF 2014–2019

Cataract surgery rate	NUMERATOR	Cataract surgery	DENOMINATOR FORMULA	SUM([Total population]) - SUM([Total population (MedicAid)])	TYPE	%
	NUMERATOR FORMULA	SUM([Cataract surgery total])	DEFINITION	Clients who had cataract surgery per 1 million uninsured population	LEVEL	Output
	DENOMINATOR	Population uninsured total	USE AND CONTEXT	Accessibility of theatres. Availability of human resources and consumables.	KEY REPORTS	APP; WHO Tracking UHC/ SDGs; Strategic Plan 2015/16-2019/20
Spectacles issued rate	NUMERATOR	Spectacles issued	DENOMINATOR FORMULA	SUM([Spectacles applications received- total])	TYPE	%
	NUMERATOR FORMULA	SUM([Spectacles issued- total])	DEFINITION	Spectacles issued as a % of the applications received	LEVEL	Process/Activity
	DENOMINATOR	Spectacle applications received - total	USE AND CONTEXT	Planning and budgeting of services. Advice on national contracts not currently available.	KEY REPORTS	None

DTaP-IPV-Hib-HBV 4th dose coverage	NUMERATOR	DTaP-IPV/Hib (Pen) 4th dose plus DTaP-IPV-Hib-HBV (Hex) 4th dose	DENOMINATOR FORMULA	SUM([Male 1 year]) + SUM([Female 1 year])	TYPE	%
	NUMERATOR FORMULA	SUM([DTaP-IPV/Hib (Pentavalent) 4th dose]) + SUM([DTaP-IPV-Hib-HBV (Hexavalent) 4th dose])	DEFINITION	Children under 1 year who received DTaP-IPV-Hib-HBV 4th dose, normally at 18 months as a proportion of the 1 year population. Both Pentaxim and hexavalent will form part of the numerator to ensure accurate coverage of historical data.	LEVEL	Output
	DENOMINATOR	Target population 1 year	USE AND CONTEXT	Monitors protection of children against diphtheria, tetanus, a-cellular pertussis, polio, haemophilus influenza and hepatitis B. DTaP-IPV-Hib-HBV (known as hexavalent) was implemented in 2015 to replace DTaP-IPV/Hib (Pentaxim) and HepB	KEY REPORTS	WHO
Td dose at 6 years coverage	NUMERATOR	Td dose at 6 years	DENOMINATOR FORMULA	(SUM([Female 05-09 years]) + SUM([Male 05-09 years])) / 5	TYPE	%
	NUMERATOR FORMULA	SUM([Td dose at 6 years])	DEFINITION	Td dose at 6 years given as a proportion of average population 5–9 years	LEVEL	Output
	DENOMINATOR	Population 5–9 years / 5	USE AND CONTEXT	Monitors protection of children against tetanus and diphtheria, including vaccinations done at schools. Vaccines given as part of mass vaccination campaigns should not be counted here.	KEY REPORTS	None
Td dose at 12 years coverage	NUMERATOR	Td dose at 12 years	DENOMINATOR FORMULA	SUM([Female 10-14 years]) + SUM([Male 10-14 years])) / 5	TYPE	%
	NUMERATOR FORMULA	SUM([Td dose at 12 years])	DEFINITION	Td dose at 12 years given as a proportion of average population	LEVEL	Output
	DENOMINATOR	Population 10–14 years / 5	USE AND CONTEXT	Monitors protection of children against tetanus and diphtheria, including vaccinations done at schools. Vaccines given as part of mass vaccination campaigns should not be counted here.	KEY REPORTS	

PCV 3rd dose coverage	NUMERATOR	PCV 3rd dose	DENOMINATOR FORMULA	SUM([Female under 1 year]) + SUM([Male under 1 year])	TYPE	%
	NUMERATOR FORMULA	SUM([PCV 3rd dose])	DEFINITION	Children under 1 year who received PCV 3rd dose, normally at 9 months as a proportion of population under 1 year	LEVEL	Output
	DENOMINATOR	Population under 1 year	USE AND CONTEXT	Monitors protection of children against pneumococcal disease. Vaccines given as part of mass vaccination campaigns should not be counted here.	KEY REPORTS	WHO
Immunisation under 1 year coverage	NUMERATOR	Immunised fully under 1 year new	DENOMINATOR FORMULA	SUM([Female under 1 year]) + SUM([Male under 1 year])	TYPE	%
	NUMERATOR FORMULA	SUM([Immunised fully under 1 year new])	DEFINITION	Children under 1 year who completed their primary course of immunisation as a proportion of population under 1 year	LEVEL	Output
	DENOMINATOR	Population under 1 year	USE AND CONTEXT	The child should only be counted ONCE as fully immunised when receiving the last vaccine in the course (usually PCV3 vaccine) AND if there is documented proof of all required vaccines (BCG, OPV1, DTaP-IPV-Hib-HBV 1, 2, 3, PCV 1,2,3, RV 1,2 and measles 1) on the Road-to-Health Card/Booklet AND the child is under 1 year old.	KEY REPORTS	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20, SDG3 Proxy
Measles 2nd dose coverage	NUMERATOR	Measles 2nd dose	DENOMINATOR FORMULA	SUM([Male 1 year]) + SUM([Female 1 year])	TYPE	%
	NUMERATOR FORMULA	SUM([Measles 2nd dose])	DEFINITION	Children 1 year (12 months) who received measles 2nd dose, as a proportion of the 1 year population	LEVEL	Output
	DENOMINATOR	Target population 1 year	USE AND CONTEXT	Monitors protection of children against measles. Because the 1st measles dose is only around 85% effective, the 2nd dose is important as a booster. Vaccines given as part of mass vaccination campaigns should not be counted here.	KEY REPORTS	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20

DTaP-IPV-Hib-HBV 3rd dose coverage	NUMERATOR	DTaP-IPV/Hib (Pen) 3rd dose plus DTaP-IPV-Hib-HBV (Hex) 3rd dose	DENOMINATOR FORMULA	SUM([Female under 1 year]) + SUM([Male under 1 year])	TYPE	%
	NUMERATOR FORMULA	SUM([DTaP-IPV/Hib (Pentavalent) 3rd dose]) + SUM([DTaP-IPV-Hib-HBV (Hexavalent) 3rd dose])	DEFINITION	Children under 1 year who received DTaP-IPV-Hib-HBV 3rd dose, normally at 14 weeks as a proportion of population under 1 year. Both Pentaxim and hexavalent will form part of the numerator to ensure accurate coverage of historical data.	LEVEL	Output
	DENOMINATOR	Population under 1 year	USE AND CONTEXT	Monitors protection of children against diphtheria, tetanus, a-cellular pertussis, polio, haemophilus influenza and hepatitis B. DTaP-IPV-Hib-HB (known as hexavalent) was implemented in 2015. DTaP-IPV/Hib to replace Pentaxim and HepB.	KEY REPORTS	WHO; HADACC
Measles 1st dose under 1 year coverage	NUMERATOR	Measles 1st dose under 1 year	DENOMINATOR FORMULA	SUM([Female under 1 year]) + SUM([Male under 1 year])	TYPE	%
	NUMERATOR FORMULA	SUM([Measles 1st dose under 1 year])	DEFINITION	Children under 1 year who received measles 1st dose, as a proportion of population under 1 year	LEVEL	Output
	DENOMINATOR	Population under 1 year	USE AND CONTEXT	Monitors protection of children under 1 year of age against measles. Vaccines given as part of mass vaccination campaigns should not be counted here.	KEY REPORTS	None
DTaP-IPV-Hib-HBV 3 - Measles 1st dose drop-out rate	NUMERATOR	DTaP-IPV-Hib-HBV 3 to Measles1st dose drop-out	DENOMINATOR FORMULA	SUM([DTaP-IPV-Hib-HBV (Hexavalent) 3rd dose]) + SUM([DTaP-IPV/Hib (Pentavalent) 3rd dose])	TYPE	%
	NUMERATOR FORMULA	SUM([DTaP-IPV/Hib (Pentavalent) 3rd dose]) + SUM([DTaP-IPV-Hib-HBV (Hexavalent) 3rd dose])) - SUM([Measles 1st dose under 1 year])	DEFINITION	Children who dropped out of the immunisation schedule between DTaP-IPV-Hib-HBV 3rd dose, normally at 14 weeks and measles 1st dose, normally at 6 months as a proportion of population under 1 year	LEVEL	Outcome
	DENOMINATOR	DTaP-IPV/Hib PLUS DTaP-IPV-Hib-HBV 3rd dose	USE AND CONTEXT	Monitors protection of children against diphtheria, tetanus, a-cellular pertussis, polio, haemophilus influenza and hepatitis B. DTaP-IPV-Hib-HBV (known as hexavalent) was implemented in 2015 to replace DTaP-IPV/Hib (Pentaxim) and HepB.	KEY REPORTS	MTSF 2014-2019; Strategic Plan 2015/16–2019/20

BCG DOSE COVERAGE	NUMERATOR	BCG dose	DENOMINATOR FORMULA	(SUM([Female under 1 year]) + SUM([Male under 1 year]))	TYPE	%
	NUMERATOR FORMULA	SUM([BCG dose])	DEFINITION	Children under 1 year who received BCG, normally given just after birth, as a proportion of population under 1 year	LEVEL	Output
	DENOMINATOR	Target population under 1 year	USE AND CONTEXT	Monitors BCG vaccination to protect children against miliary TB and TB meningitis. Vaccines given as part of mass vaccination campaigns should not be counted here.	KEY REPORTS	WHO
DTaP-IPV-Hib-HBV 1st DOSE COVERAGE	NUMERATOR	DTaP-IPV/Hib (Pen) 1st dose plus DTaP-IPV-Hib-HBV (Hex) 1st dose	DENOMINATOR FORMULA	(SUM([Female under 1 year]) + SUM([Male under 1 year]))	TYPE	%
	NUMERATOR FORMULA	SUM([DTaP-IPV/Hib (Pentavalent) 1st dose]) + SUM([DTaP-IPV-Hib-HBV (Hexavalent) 1st dose])	DEFINITION	Children under 1 year who received DTaP-IPV-Hib-HBV 1st dose, normally at 6 weeks as a proportion of population under 1 year. Both Pentaxim and hexavalent will form part of the numerator to ensure accurate coverage of historical data.	LEVEL	Output
	DENOMINATOR	Population under 1 year	USE AND CONTEXT	Monitors protection of children against diphtheria, tetanus, a-cellular pertussis, polio, haemophilus influenza and hepatitis B. DTaP-IPV-Hib-HB (known as hexavalent) was implemented in 2015. DTaP-IPV/Hib to replace Pentaxim and HepB.	KEY REPORTS	WHO
RV 2nd DOSE COVERAGE	NUMERATOR	RV 2nd dose	DENOMINATOR FORMULA	SUM([Female under 1 year]) + SUM([Male under 1 year])	TYPE	%
	NUMERATOR FORMULA	SUM([RV 2nd dose under 1 year])	DEFINITION	Children under 1 year who received RV 2nd dose as a proportion of children under 1 year	LEVEL	Output
	DENOMINATOR	Population under 1 year	USE AND CONTEXT	Monitors protection of children under 1 year against rotavirus.	KEY REPORTS	WHO; HADACC

EH International Imported Consignment Inspection Rate	NUMERATOR	EH International imported consignment inspected at first point of entry	DENOMINATOR FORMULA	SUM([EH International imported consignment arrival at first point of entry])	TYPE	%
	NUMERATOR FORMULA	SUM([EH International imported consignment inspected at first point of entry])	DEFINITION	International imported consignments inspected in terms of International Health Regulations and national legislation as a proportion of international consignment arrivals at first point of entry	LEVEL	Process/ Activity
	DENOMINATOR	EH International imported consignment arrivals at first point of entry	USE AND CONTEXT	Monitors international imported consignment inspection.	KEY REPORTS	None
EH International Imported Consignment Compliance Rate	NUMERATOR	EH International imported consignment compliant at first point of entry	DENOMINATOR FORMULA	SUM([EH International imported consignment arrival at point of entry])	TYPE	%
	NUMERATOR FORMULA	SUM([EH International imported consignment inspected])	DEFINITION	International imported consignments that complied to international health standards, compliant in terms of the National Port Health Standard Operating Procedures (revised SOPs) and International Health Regulations (IHR, 2005) as a proportion of inspection	LEVEL	Process/ Activity
	DENOMINATOR	EH International imported consignments inspected at first point of entry	USE AND CONTEXT	Monitors international imported consignment compliance.	KEY REPORTS	None

EH Health care risk waste generator compliance rate	NUMERATOR	EH Health care Risk waste generator compliant	DENOMINATOR FORMULA	SUM([EH Health care waste generator inspected])	TYPE	%
	NUMERATOR FORMULA	SUM([EH Health care waste generator compliant])	DEFINITION	Proportion inspected health care risk waste generators that complied with minimum standards according to SANS 10248. (Target 90%)	LEVEL	Process/ Activity
	DENOMINATOR	EH Health care waste generator inspected	USE AND CONTEXT	Monitors health care waste generator compliance.	KEY REPORTS	None
EH Hazardous substance dealers compliance rate	NUMERATOR	EH Hazardous substance dealer compliant	DENOMINATOR FORMULA	SUM([EH Hazardous substance dealer inspected])	TYPE	%
	NUMERATOR FORMULA	SUM([EH Hazardous substance dealer compliant])	DEFINITION	Hazardous substances dealers that were found complied with the regulations for hazardous substances as a proportion of EH hazardous substance dealer inspected (Target 90%)	LEVEL	Process/ Activity
	DENOMINATOR	EH Hazardous substance dealer inspected	USE AND CONTEXT	Monitors hazardous substance dealer compliance.	KEY REPORTS	None
EH International conveyance inspection rate	NUMERATOR	EH International conveyances (commercial) Inspected at first point of entry	DENOMINATOR FORMULA	SUM([EH International conveyance arrival at first point of entry])	TYPE	%
	NUMERATOR FORMULA	SUM([EH International conveyance inspected at first point of entry])	DEFINITION	International conveyance at arrival inspected for compliance to international health standards as a proportion of international conveyance arrivals at first point of entry (Target 85%)	LEVEL	Process/ Activity
	DENOMINATOR	EH International conveyance arrivals at first point of entry	USE AND CONTEXT	Monitors international conveyance inspection.	KEY REPORTS	None
EH International conveyance (commercial) compliance rate	NUMERATOR	EH International conveyances (commercial) compliant at first point of entry	DENOMINATOR FORMULA	SUM([EH International conveyance (commercial) inspected at first point of entry])	TYPE	%
	NUMERATOR FORMULA	SUM([EH International conveyance (commercial) compliant at first point of entry])	DEFINITION	International conveyances (commercial) arriving at points of entry inspected that comply with provisions of the International Health Regulations and national legislation as a proportion of arrivals at first point of entry (Target 95%)	LEVEL	Process/ Activity
	DENOMINATOR	EH International conveyances (commercial) inspected at a points of entry	USE AND CONTEXT	Monitors international conveyances (commercial) compliance.	KEY REPORTS	None

EH Notifiable Medical Conditions Investigation Rate	NUMERATOR	EH notifiable medical conditions investigated new	DENOMINATOR FORMULA	SUM([EH notifiable medical conditions reported to District Health Office])	TYPE	%
	NUMERATOR FORMULA	SUM([EH notifiable medical conditions investigated new])	DEFINITION	Notifiable medical conditions (food, mercury, lead and pesticide poisoning) investigated by EHP officers as a proportion of cases reported to the District Health Office (Target 85%)	LEVEL	Process/ Activity
	DENOMINATOR	EH notifiable medical conditions reported to District Health Office	USE AND CONTEXT	Monitors notifiable medical conditions investigated by EHP Officers.	KEY REPORTS	None

EH Notifiable Medical Conditions Investigation Rate	NUMERATOR	EH notifiable medical conditions investigated new	DENOMINATOR FORMULA	SUM([EH notifiable medical conditions reported to District Health Office])	TYPE	%
	NUMERATOR FORMULA	SUM([EH notifiable medical conditions investigated new])	DEFINITION	Notifiable medical conditions (food, mercury, lead and pesticide poisoning) investigated by EHP officers as a proportion of cases reported to the District Health Office (Target 85%)	LEVEL	Process/ Activity
	DENOMINATOR	EH notifiable medical conditions reported to District Health Office	USE AND CONTEXT	Monitors notifiable medical conditions investigated by EHP Officers.	KEY REPORTS	None

EH Chemical Poisoning Case Investigation Rate	NUMERATOR	EH chemical poisoning investigated new	DENOMINATOR FORMULA	SUM([EH chemical poisoning conditions reported to District Health Office])	TYPE	%
	NUMERATOR FORMULA	EH chemical poisoning investigated new	DEFINITION	Chemical poisoning cases (food, mercury, lead and pesticide poisoning) investigated by EHP officers as a proportion of the cases reported to the District Health Office. The target is 85%.	LEVEL	Process/ Activity
	DENOMINATOR	EH chemical poisoning conditions reported to District Health Office	USE AND CONTEXT	Monitors chemical poisoning inspections by EHP officers.	KEY REPORTS	None

EH Mills Fortification Compliance Rate	NUMERATOR	EH Milling establishment compliant	DENOMINATOR FORMULA	SUM([EH Milling establishment inspected])	TYPE	%
	NUMERATOR FORMULA	SUM([EH Milling establishment compliant])	DEFINITION	Operational flour and maize milling establishments that were compliant with fortification Regulations under the FCD Act as a proportion of milling establishments that were inspected (Target 98%)	LEVEL	Process/ Activity
	DENOMINATOR	EH Milling establishment that were Inspected	USE AND CONTEXT	Monitors flour and maize milling establishment compliance.	KEY REPORTS	None

EH Domestic water compliance rate	NUMERATOR	EH Domestic water sample compliant	DENOMINATOR FORMULA	SUM([EH Domestic water sample collected])	TYPE	%
	NUMERATOR FORMULA	SUM([EH Domestic water sample compliant])	DEFINITION	Domestic bacteriological and chemical water samples taken from Water Services Authorities and water service intermediaries at a point of use that conforms to the standards set out in SANS 241 for drinking water quality and safety as a proportion of water	LEVEL	Process/ Activity
	DENOMINATOR	EH Domestic water sample collected	USE AND CONTEXT	Monitors domestic water safety.	KEY REPORTS	None
EH Surface water compliance rate	NUMERATOR	EH Surface water sample compliant	DENOMINATOR FORMULA	SUM([EH Surface water sample collected])	TYPE	%
	NUMERATOR FORMULA	SUM([EH Surface water sample compliant])	DEFINITION	Surface water samples taken from rivers/streams/lakes/dams that is found safe. Routine (normal water sampling schedule and points) – excluding samples collected in outbreaks and/or for other specific purposes as a proportion of surface water samples collected	LEVEL	Process/ Activity
	DENOMINATOR	EH Surface water sample collected	USE AND CONTEXT	Monitors surface water safety.	KEY REPORTS	None
EH Food sample chemical compliance rate	NUMERATOR	EH Food sample chemically compliant	DENOMINATOR FORMULA	SUM([EH Food sample chemical analysis])	TYPE	%
	NUMERATOR FORMULA	SUM([EH Food sample chemically compliant])	DEFINITION	Food samples chemically tested that complied with the Foodstuffs, Cosmetics and Disinfectants Act (54 Of 1972) as a proportion of food samples chemically analysed (Target 90%)	LEVEL	Process/ Activity
	DENOMINATOR	EH Food samples chemical analysis	USE AND CONTEXT	Monitors food safety. Includes imported food stuffs.	KEY REPORTS	None
EH Food sample bacteriological compliance rate	NUMERATOR	EH Food sample bacteriologically compliant	DENOMINATOR FORMULA	SUM([EH Food sample bacteriological analysis])	TYPE	%
	NUMERATOR FORMULA	SUM([EH Food sample bacteriologically compliant])	DEFINITION	Food samples bacteriologically tested that complied with the Foodstuffs, Cosmetics and Disinfectants Act (54 Of 1972) as a proportion of food samples bacteriologically analysed. (Target 98%)	LEVEL	Process/ Activity
	DENOMINATOR	EH Food sample bacteriological analysis	USE AND CONTEXT	Monitors food safety. Includes imported food stuffs.	KEY REPORTS	None
EH food-handling compliance rate	NUMERATOR	EH food-handling premises compliant	DENOMINATOR FORMULA	SUM([EH food handling premises inspected])	TYPE	%
	NUMERATOR FORMULA	SUM([EH food-handling premises compliant])	DEFINITION	Food-handling premises inspected and found to be compliant with the requirements of the Regulation R962 of the FCD as a proportion of food-handling premises inspected (Act 54 of 1972). (Target 80%)	LEVEL	Process/ Activity
	DENOMINATOR	EH food-handling premises inspected	USE AND CONTEXT	Monitors food-handling premises compliance.	KEY REPORTS	None

EMS P1 Obstetric Clients Transport Rate	NUMERATOR	EMS P1 obstetric client transported	DENOMINATOR FORMULA	SUM([EMS clients total])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS P1 obstetric inter-facility transfer])	DEFINITION	Obstetric clients as a proportion of total EMS clients transported	LEVEL	Output
	DENOMINATOR	EMS clients - total	USE AND CONTEXT	Monitors need for and use of obstetric dedicated ambulances. Includes obstetric clients transported in obstetric and other operational ambulances.	KEY REPORTS	None
EMS Inter-facility Transfer Rate	NUMERATOR	EMS inter-facility transfer	DENOMINATOR FORMULA	SUM([EMS clients total])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS inter-facility transfer])	DEFINITION	Inter-facility (from one inpatient facility to another inpatient facility) transfers as proportion of total EMS patients transported	LEVEL	Output
	DENOMINATOR	EMS clients - total	USE AND CONTEXT	Monitors use of ambulances for inter-facility transfers as opposed to emergency responses.	KEY REPORTS	None
EMS Obstetric Inter-facility Transfer Rate	NUMERATOR	EMS Obstetric emergency inter-facility transfer	DENOMINATOR FORMULA	SUM([EMS clients total])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS obstetric inter-facility transfer])	DEFINITION	Obstetric inter-facility (from one inpatient facility to another inpatient facility) transfers as proportion of total EMS patients transported	LEVEL	Output
	DENOMINATOR	EMS clients - total	USE AND CONTEXT	Monitors use of ambulances for obstetric inter-facility transfers as opposed to emergency responses and all other inter-facility transfers.	KEY REPORTS	None

EMS EMERGENCY RURAL INTER-FACILITY TRANSFER (IFT) UNDER 60 MINUTES RATE	NUMERATOR	EMS emergency rural inter-facility transfer under 60 minutes	DENOMINATOR FORMULA	SUM([EMS emergency rural inter-facility transfer])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS emergency rural inter-facility transfer under 60 minutes])	DEFINITION	Emergency inter-facility transfers response times under 60 minutes as a proportion of EMS rural inter-facility transfers	LEVEL	Output
	DENOMINATOR	EMS emergency rural inter-facility transfer	USE AND CONTEXT	Monitors compliance with responses to emergency inter-facility transfers in rural areas.	KEY REPORTS	None
EMS OBSTETRIC EMERGENCY URBAN INTER-FACILITY TRANSFER UNDER 30 MINUTES RATE	NUMERATOR	EMS Obstetric emergency urban inter-facility transfer under 30 minutes	DENOMINATOR FORMULA	SUM([EMS obstetric emergency urban inter-facility transfer])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS obstetric emergency urban inter-facility transfer under 30 minutes])	DEFINITION	Emergency obstetric inter-facility transfers response times under 30 minutes as a proportion of EMS obstetric urban inter-facility transfers	LEVEL	Output
	DENOMINATOR	EMS obstetric emergency urban inter-facility transfer	USE AND CONTEXT	Monitors compliance with responses to emergency obstetric inter-facility transfers in an urban areas.	KEY REPORTS	None
EMS OBSTETRIC EMERGENCY RURAL INTER-FACILITY TRANSFER UNDER 60 MINUTES RATE	NUMERATOR	EMS Obstetric emergency rural inter-facility transfer under 60 minutes	DENOMINATOR FORMULA	SUM([EMS obstetric emergency rural inter-facility transfer])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS obstetric emergency rural inter-facility transfer under 60 minutes])	DEFINITION	Emergency obstetric inter-facility transfers response times under 60 minutes as a proportion of EMS obstetric rural inter-facility transfers	LEVEL	Output
	DENOMINATOR	EMS obstetric emergency rural inter-facility transfer	USE AND CONTEXT	Monitors compliance with responses to emergency obstetric inter-facility transfers in an rural areas.	KEY REPORTS	None
EMS P1 CLIENTS TRANSPORT RATE	NUMERATOR	EMS P1 client transported	DENOMINATOR FORMULA	SUM([EMS clients total])	TYPE	No
	NUMERATOR FORMULA	EMS P1 client transported	DEFINITION	EMS P1 clients as a proportion of total EMS clients transported	LEVEL	Output
	DENOMINATOR	EMS clients - total	USE AND CONTEXT	Monitors use of ambulances for primary emergencies.	KEY REPORTS	None

EMS P1 URBAN RESPONSE UNDER 15 MINUTES RATE	NUMERATOR	EMS P1 urban response under 15 minutes	DENOMINATOR FORMULA	SUM([EMS P1 urban calls])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS P1 urban response under 15 minutes])	DEFINITION	Emergency P1 calls in urban locations with response times under 15 minutes as a proportion of EMS P1 urban calls. Response time is calculated from the time the call is received to the time that the first dispatched medical resource arrives on scene.	LEVEL	Output
	DENOMINATOR	EMS P1 urban calls	USE AND CONTEXT	Monitors compliance with the norm for critically ill or injured patients to receive EMS within 15 minutes in urban areas.	KEY REPORTS	None
EMS P1 RURAL RESPONSE UNDER 40 MINUTES RATE	NUMERATOR	EMS P1 rural response under 40 minutes	DENOMINATOR FORMULA	SUM([EMS P1 rural calls])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS P1 rural response under 40 minutes])	DEFINITION	Emergency P1 calls in rural locations with response times under 40 minutes as a proportion of EMS P1 rural call	LEVEL	Output
	DENOMINATOR	EMS P1 rural calls	USE AND CONTEXT	Monitors compliance with the norm for critically ill or injured patients to receive EMS within 40 minutes in rural areas.	KEY REPORTS	None
EMS P1 URBAN OBSTETRIC RESPONSE UNDER 15 MINUTES RATE	NUMERATOR	EMS P1 urban obstetric response under 15 minutes	DENOMINATOR FORMULA	SUM([EMS P1 EMS P1 urban obstetric calls])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS P1 urban obstetric response under 15 minutes])	DEFINITION	Obstetric calls responded to under 15 minutes in an urban area as a proportion of EMS P1 urban obstetric calls total	LEVEL	Output
	DENOMINATOR	EMS P1 urban obstetric calls	USE AND CONTEXT	Monitors compliance with the norm for obstetric patients to receive EMS under 15 minutes in urban areas.	KEY REPORTS	None
EMS P1 RURAL OBSTETRIC RESPONSE UNDER 40 MINUTES RATE	NUMERATOR	EMS P1 rural obstetric response under 40 minutes	DENOMINATOR FORMULA	SUM([EMS P1 EMS P1 rural obstetric calls])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS P1 rural obstetric response under 40 minutes])	DEFINITION	Obstetric calls responded to under 40 minutes in a rural area as a proportion of EMS P1 rural obstetric calls total	LEVEL	Output
	DENOMINATOR	EMS P1 rural obstetric calls	USE AND CONTEXT	Monitors compliance with the norm for critically ill or injured patients to receive EMS under 40 minutes in rural areas.	KEY REPORTS	None
EMS EMERGENCY URBAN INTER-FACILITY TRANSFER (IFT) UNDER 30 MINUTES RATE	NUMERATOR	EMS emergency urban inter-facility transfer under 30 minutes	DENOMINATOR FORMULA	SUM([EMS emergency urban inter-facility transfer])	TYPE	No
	NUMERATOR FORMULA	SUM([EMS emergency urban inter-facility transfer under 30 minutes])	DEFINITION	Emergency inter-facility transfers response times under 30 minutes as a proportion of EMS urban inter-facility transfers	LEVEL	Output
	DENOMINATOR	EMS emergency urban inter-facility transfer	USE AND CONTEXT	Monitors compliance with responses to emergency inter-facility transfers in an urban areas.	KEY REPORTS	None

BILHARZIA INCIDENCE						
	NUMERATOR	Bilharzia case reported	DENOMINATOR FORMULA	SUM([Total population])	TYPE	%
	NUMERATOR FORMULA	SUM([Bilharzia case reported])	DEFINITION	New cases of bilharzia (schistosomiasis) reported irrespective of the causative organism	LEVEL	Outcome
	DENOMINATOR	Population - total	USE AND CONTEXT	To assess the burden of bilharzia	KEY REPORTS	WHO

DIABETES CLIENTS ON TREATMENT CASELOAD *						
	NUMERATOR	Diabetes clients on treatment	DENOMINATOR FORMULA	SUM([PHC headcount 5 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM ([Diabetes treatment visit])	DEFINITION	Total number of clients with diabetes diagnosed and on treatment	LEVEL	Output
	DENOMINATOR	PHC headcount 5 years and older	USE AND CONTEXT	Caseload to reflect the burden of care required and assist with planning at facility and district level	KEY REPORTS	None

Clients 40 years and older screened for hypertension *	NUMERATOR	Client 40 years and older screened for hypertension	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([Client 40 years and older screened for hypertension])	DEFINITION	Number of clients not on treatment for hypertension screened for hypertension in PHC clinics and OPD	LEVEL	Process/Activity
	DENOMINATOR	None	USE AND CONTEXT	This should assist with increasing the number of clients detected and referred for treatment	KEY REPORTS	APP; Strategic Plan 2015/16–2019/20
Hypertension treatment caseload *	NUMERATOR	Hypertension visit by clients on treatment	DENOMINATOR FORMULA	SUM([PHC headcount 20 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([Hypertension visit by clients on treatment])	DEFINITION	Total number of visits by clients with hypertension diagnosed and on treatment	LEVEL	Output
	DENOMINATOR	PHC headcount 20 years and above	USE AND CONTEXT	Case load to reflect the burden on care. This can assist with planning at all levels.	KEY REPORTS	None
Clients 40 years and older screened for diabetes *	NUMERATOR	Client 40 years and older screened for diabetes	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([Client 40 years and older screened for diabetes])	DEFINITION	Number of clients not diagnosed and not on treatment for diabetes screened for diabetes in PHC clinics and OPD	LEVEL	Process/Activity
	DENOMINATOR	None	USE AND CONTEXT	This should assist with increasing the number of clients with diabetes detected and referred for treatment.	KEY REPORTS	APP
Diabetes client treatment new *	NUMERATOR	Diabetes client treatment new	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([Diabetic client treatment new])	DEFINITION	Newly diagnosed clients with a fasting blood glucose of >7mmol/L or random blood glucose >11.1mmol/L	LEVEL	Process/Activity
	DENOMINATOR	None	USE AND CONTEXT	This should increase as detection of cases improves.	KEY REPORTS	None
Diabetes new client 40 years and older detection rate *	NUMERATOR	Diabetes client 40 years and older new	DENOMINATOR FORMULA	SUM(SUM([Female 40–44 years]) + SUM([Female 45 years and older]) + SUM([Male 40–44 years]) + SUM([Male 45 years and older])	TYPE	%
	NUMERATOR FORMULA	SUM([Diabetes client 40 years and older new])	DEFINITION	Newly diagnosed clients with a fasting blood glucose of >7mmol/L or random blood glucose >11.1mmol/L. initiated on treatment	LEVEL	Process/Activity
	DENOMINATOR	Population 40 years and older	USE AND CONTEXT	This should increase as detection of cases improves.	KEY REPORTS	None

DEATH IN FACILITY UNDER 5 YEARS RATE						
	NUMERATOR	Death in facility under 5 years total	DENOMINATOR FORMULA	SUM([Death in facility 0-7 days]) + SUM([Death in facility 8-28 days]) + SUM([Death in facility 29 days - 11 months]) + SUM([Death in facility 12-59 months]) + SUM([Inpatient discharge under 5 years]) + SUM([Inpatient transfer out under 5 years)	TYPE	%
	NUMERATOR FORMULA	SUM([Death in facility 0-7 days]) + SUM([Death in facility 8-28 days]) + SUM([Death in facility 29 days - 11 months]) + SUM([Death in facility 12-59 months])	DEFINITION	Children under 5 years who died during their stay in the facility as a proportion of inpatient separations under 5 years. Inpatient separations under 5 years is the total of inpatient discharges, inpatient deaths and inpatient transfers out.	LEVEL	Impact
	DENOMINATOR	Inpatient separations under 5 years	USE AND CONTEXT	Monitors treatment outcome for admitted children under 5 years. Includes under 1 year deaths.	KEY REPORTS	MSTF 2014–2019, SDG3 Proxy

Severe acute malnutrition death under 5 years rate	NUMERATOR	Severe acute malnutrition (SAM) death in facility under 5 years	DENOMINATOR FORMULA	SUM([Death in facility 0–7 days]) + SUM([Death in facility 8–28 days]) + SUM([Death in facility 29 days–11 months]) + SUM([Death in facility 12–59 months])	TYPE	%
	NUMERATOR FORMULA	SUM([Severe acute malnutrition (SAM) death in facility under 5 years])	DEFINITION	Severe acute malnutrition deaths in children under 5 years as a proportion of total deaths in facility under 5 years	LEVEL	Impact
	DENOMINATOR	Death in facility under 5 years	USE AND CONTEXT	Monitors treatment outcome for children under 5 years who were separated with severe acute malnutrition (SAM)	KEY REPORTS	MTSF 2014–2019, Strategic Plan 2015/16–2019/20, SDG2 Proxy
Moderate acute malnutrition case fatality under 5 years rate	NUMERATOR	Moderate acute malnutrition (MAM) death in facility under 5 years	DENOMINATOR FORMULA	SUM([Moderate acute malnutrition (MAM) under 5 years in facility])	TYPE	%
	NUMERATOR FORMULA	SUM([Moderate acute malnutrition (MAM) death in facility under 5 year])	DEFINITION	Moderate acute malnutrition deaths in children under 5 years as a proportion of Moderate acute malnutrition (MAM) under 5 years in health facilities	LEVEL	Impact
	DENOMINATOR	Moderate acute malnutrition (MAM) in facility under 5 years	USE AND CONTEXT	Monitors treatment outcome for children under 5 years who were separated with moderate acute malnutrition (NAM).	KEY REPORTS	MTSF 2014–2019, Strategic Plan 2015/16–2019/20, SDG2 Proxy
Death in facility under 1 year rate	NUMERATOR	Death in facility under 1 year total	DENOMINATOR FORMULA	SUM([Death in facility 0-7 days]) + SUM([Death in facility 8-28 days]) + SUM([Death in facility 29 days - 11 months]) + SUM([Inpatient discharge under 1 year]) + SUM([Inpatient transfer out under 1 year])	TYPE	%
	NUMERATOR FORMULA	SUM([Death in facility under 1 year total])	DEFINITION	Children under 1 year who died during their stay in the facility as a proportion of inpatient separations under 1 year. Inpatient separations under 1 year is the total of inpatient discharges, inpatient deaths and inpatient transfers out.	LEVEL	Impact
	DENOMINATOR	Inpatient separations under 1 year	USE AND CONTEXT	Monitors treatment outcome for admitted children under 1 year. Includes neonatal deaths.	KEY REPORTS	MSTF 2014–2019, SDG3 Proxy

PNEUMONIA CASE FATALITY UNDER 5 YEARS RATE	NUMERATOR	Pneumonia death under 5 years	DENOMINATOR FORMULA	SUM([Pneumonia separation under 5 years])	TYPE	%
	NUMERATOR FORMULA	SUM([Pneumonia death under 5 years])	DEFINITION	Pneumonia deaths in children under 5 years as a proportion of pneumonia separations under 5 years in health facilities	LEVEL	Impact
	DENOMINATOR	Pneumonia separation under 5 years	USE AND CONTEXT	Monitors treatment outcome for children under 5 years who were separated with pneumonia.	KEY REPORTS	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
SEVERE ACUTE MALNUTRITION (SAM) UNDER 5 YEARS IN FACILITY RATE	NUMERATOR	Severe acute malnutrition (SAM) under 5 years in facility	DENOMINATOR FORMULA	SUM([Severe acute malnutrition (SAM) under 5 years in facility]) + Sum([Moderate acute malnutrition (MAM) under 5 years in facility]) + SUM([No acute malnutrition (NAM) under 5 years in facility])	TYPE	%
	NUMERATOR FORMULA	SUM([Severe acute malnutrition (SAM) under 5 years in facility])	DEFINITION	Severe acute malnutrition (SAM) under 5 years as a proportion of total nutritional assessments under 5 years in facility	LEVEL	Impact
	DENOMINATOR	Total nutritional assessments under 5 years in facility: » No acute malnutrition (NAM) » Moderate acute malnutrition (MAM) » SAM	USE AND CONTEXT	This will allow for an assessment of whether the nutritional status of all admitted children is being assessed.	KEY REPORTS	SDG2 Proxy
SEVERE ACUTE MALNUTRITION CASE FATALITY UNDER 5 YEARS RATE	NUMERATOR	Severe acute malnutrition (SAM) death in facility under 5 years	DENOMINATOR FORMULA	SUM([Severe acute malnutrition (SAM) under 5 years in facility])	TYPE	%
	NUMERATOR FORMULA	SUM([Severe acute malnutrition (SAM) death in facility under 5 year])	DEFINITION	Severe acute malnutrition deaths in children under 5 years as a proportion of severe acute malnutrition (SAM) under 5 years in health facilities	LEVEL	Impact
	DENOMINATOR	Severe acute malnutrition (SAM) in facility under 5 years	USE AND CONTEXT	Monitors treatment outcome for children under 5 years who were separated with severe acute malnutrition (SAM)	KEY REPORTS	MTSF 2014 - 2019, Strategic Plan 2015/16-2019/20, SDG2 Proxy

CHILD UNDER 5 YEARS FOOD NUTRITIONAL SUPPLEMENTATION COVERAGE	NUMERATOR	Child under 5 years on food nutritional supplementation new	DENOMINATOR FORMULA	SUM([Female under 5 years]) + SUM([Male under 5 years])	TYPE	Per1K
	NUMERATOR FORMULA	SUM([Child under 5 years on food supplementation new])	DEFINITION	Children under 5 newly started on food nutritional supplementation as proportion of population under 5 years	LEVEL	Output
	DENOMINATOR	Population under 5 years	USE AND CONTEXT	Monitors access to and efficient use of food supplementation in children under 5 years of age. Around 2% of children under 5 are underweight and need food supplementation.	KEY REPORTS	None
DIARRHOEA WITH DEHYDRATION IN CHILD UNDER 5 YEARS INCIDENCE	NUMERATOR	Diarrhoea with dehydration new in child under 5 years	DENOMINATOR FORMULA	SUM([Female under 5 years]) + SUM([Male under 5 years])	TYPE	Per1K
	NUMERATOR FORMULA	SUM([Child under 5 years with diarrhoea with dehydration new])	DEFINITION	Children under 5 years newly diagnosed with diarrhoea with dehydration per 1 000 children under 5 years in the population	LEVEL	Output
	DENOMINATOR	Population under 5 years	USE AND CONTEXT	Monitors prevention of diarrhoea with dehydration (IMCI classification) in children under 5 years. Count only once when diagnosed. Follow-up visits for the same episode of diarrhoea should not be counted here.	KEY REPORTS	None
DIARRHOEA CASE FATALITY UNDER 5 YEARS RATE	NUMERATOR	Diarrhoea death under 5 years	DENOMINATOR FORMULA	SUM([Diarrhoea separation under 5 years])	TYPE	%
	NUMERATOR FORMULA	SUM([Diarrhoea death under 5 years])	DEFINITION	Diarrhoea deaths in children under 5 years as a proportion of diarrhoea separations under 5 years in health facilities	LEVEL	Impact
	DENOMINATOR	Diarrhoea separation under 5 years	USE AND CONTEXT	Monitors treatment outcome for children under 5 years who were separated with diarrhoea.	KEY REPORTS	2016/17–2018/19 APP, MTSF 2014–2019, Strategic Plan 2015/16–2019/20
PNEUMONIA IN CHILD UNDER 5 YEARS INCIDENCE	NUMERATOR	Pneumonia new in child under 5 years	DENOMINATOR FORMULA	SUM([Female under 5 years]) + SUM([Male under 5 years])	TYPE	Per1K
	NUMERATOR FORMULA	SUM([Child under 5 years with pneumonia new])	DEFINITION	Children under 5 years newly diagnosed with pneumonia per 1 000 children under 5 years in the population	LEVEL	Output
	DENOMINATOR	Population under 5 years	USE AND CONTEXT	Monitors prevention and diagnosis of pneumonia (IMCI definition) in children under 5 years. Count only once when diagnosed. Follow-up visits for the same episode of pneumonia should not be counted here.	KEY REPORTS	None

Vitamin A dose 12–59 months coverage	NUMERATOR	Vitamin A dose 12–59 months	DENOMINATOR FORMULA	(SUM([Female 1 year]) + SUM([Female 02-04 years]) + SUM([Male 1 year]) + SUM([Male 02-04 years])) * 2	TYPE	%
	NUMERATOR FORMULA	SUM([Vitamin A dose 12–59 months])	DEFINITION	Children 12–59 months who received vitamin A 200 000 units, every six months as a proportion of population 12–59 months	LEVEL	Output
	DENOMINATOR	Target population 12–59 months * 2	USE AND CONTEXT	Monitors Vitamin A supplementation to children aged 12–59 months. The denominator is multiplied by 2 because each child should receive supplementation twice a year.	KEY REPORTS	None
Deworming dose 12–59 months coverage	NUMERATOR	Deworming dose 12–59 months	DENOMINATOR FORMULA	(SUM([Female 1 year]) + SUM([Female 02-04 years]) + SUM([Male 1 year]) + SUM([Male 02-04 years])) * 2	TYPE	%
	NUMERATOR FORMULA	SUM([Deworming dose 12–59 months])	DEFINITION	Children 12 to 59 months who received deworming medication, every six months as a proportion of population 12 to 59 months	LEVEL	Output
	DENOMINATOR	Target population 12–59 months * 2	USE AND CONTEXT	Monitors deworming in children aged 12–59 months	KEY REPORTS	None
Infant exclusively breastfed at DTaP-IPV-Hib-HBV 3rd dose rate	NUMERATOR	Infant exclusively breastfed at DTaP-IPV-Hib-HBV 3rd dose	DENOMINATOR FORMULA	SUM([HepB 3rd dose under 1 year]) + SUM([DTaP-IPV-Hib-HBV (Hexavalent) 3rd dose])	TYPE	%
	NUMERATOR FORMULA	SUM([Infant exclusively breastfed at DTaP-IPV-Hib-HBV (Hexavalent) 3rd dose])	DEFINITION	Infants exclusively breastfed at 14 weeks as a proportion of the DTaP-IPV-Hib-HBV 3rd dose vaccination. Take note that DTaP-IPV-Hib-HBV 3rd dose (Hexavalent) was implemented in 2015 to include the HepB dose.	LEVEL	Output
	DENOMINATOR	DTaP-IPV-Hib-HBV 3rd dose	USE AND CONTEXT	Monitors infant feeding practices at 14 weeks to identify where community interventions need to be strengthened.	KEY REPORTS	None
Child under 2 years underweight for age incidence	NUMERATOR	Child under 2 years underweight - new (weight between -2 SD and -3 SD new)	DENOMINATOR FORMULA	SUM([Female under 1 year]) + SUM([Female 1 year]) + SUM([Male under 1 year]) + SUM([Male 1 year])	TYPE	Per1K
	NUMERATOR FORMULA	SUM([Child under 2 years underweight - new (weight between -2 SD and -3 SD new)])	DEFINITION	Children under 2 years newly diagnosed as underweight (weight between -2 and -3 Standard Deviations) per 1 000 children under 2 years in the population	LEVEL	Outcome
	DENOMINATOR	Population under 2 years	USE AND CONTEXT	Monitors prevention of malnutrition during 1 000 days between start of pregnancy and second birthday. Count only once when diagnosed. Follow-up visits for the same episode of underweight should not be counted.	KEY REPORTS	SDG2 Proxy

STABLE CLIENT CENTRAL CHRONIC MEDICINES DISPENSING AND DISTRIBUTION (CCMDD) PROGRAMME – TOTAL						
	NUMERATOR	Stable client Central Chronic Medicines Dispensing and Distribution (CCMDD) programme – new + Stable client Central Chronic Medicines Dispensing and Distribution (CCMDD) programme – repeat	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([Stable client Central Chronic Medicines Dispensing and Distribution (CCMDD) programme – new]) + SUM([Stable client Central Chronic Medicines Dispensing and Distribution (CCMDD) programme – repeat])	DEFINITION	All stable clients (new and repeats) who are enrolled on the CCMDD programme and who collected their medicines at the designated pick-up point during the period.	LEVEL	Process/ Activity
	DENOMINATOR	None	USE AND CONTEXT	The information is used to monitor coverage of the CCMDD programme. Appointed service providers deliver medicines at the designated service points for enrolled groups of stable patients (new and repeats) who are registered on the CCMDD project. All medicines delivered are bar-coded. Medicines which were not collected are returned and not counted.	KEY REPORTS	APP, CCMDD Programme Monitoring

ART CLIENT VIRAL LOAD SUPPRESSED RATE (VLS)	**NUMERATOR**	ART client viral load under 400 cps/mL (VLS)	**DENOMINATOR FORMULA**	SUM([Adult viral load done (VLD) at 6 months])	**TYPE**	%
	NUMERATOR FORMULA	SUM([Adult viral load under 400 cps/mL (VLS) at 6 months])	**DEFINITION**	Proportion of ART clients with viral load suppressed at different time intervals. This indicates the population-level immunological impact of clients on ART. Note: Cohorts will be followed for the following indicators at 6 and 12 months and thereafter annually and at 5-year intervals after 60 months.	**LEVEL**	Outcome
	DENOMINATOR	ART viral load done (VLD) at intervals	**USE AND CONTEXT**	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	**KEY REPORTS**	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
PREGNANT AT ART START REMAINING IN CARE AT 12 MONTHS RATE (RIC)	**NUMERATOR**	Pregnant at ART start remaining in care at 12 months	**DENOMINATOR FORMULA**	SUM([Antenatal client start on ART])	**TYPE**	%
	NUMERATOR FORMULA	SUM([Pregnant at ART start remaining in care at 12 months])	**DEFINITION**	ART pregnant at start remaining in care at 12 months as a proportion of antenatal clients who started ART 12 months prior	**LEVEL**	Outcome
	DENOMINATOR	Antenatal client start on ART	**USE AND CONTEXT**	This value will enable the programme to assess retention at 12 months. Note that the TFO, LTF and RIP used in the formula are the cumulative totals of all clients who left care to that point, as opposed to the total in the most recent interval.	**KEY REPORTS**	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

ART CLIENT DIED RATE (RIP)	NUMERATOR	ART client cumulative RIP	DENOMINATOR FORMULA	SUM([Adult naive male started ART]) + SUM([Adult naive female started ART])) - (SUM([Adult ART transferred out (TFO)]	TYPE	%
	NUMERATOR FORMULA	SUM([Adult ART died (RIP) at 3 months])	DEFINITION	Cumulative proportion of adults who died after interval. At different time intervals, the cumulative deaths can be analysed. *Note that the TFO, LTF and RIP used in the formula are the cumulative totals of all clients who left care to that point, as opposed to the total in the most recent interval. *Note: Cohorts will be followed for the following indicators at 6 and 12 months and thereafter annually and at 5-year intervals after 60 months.	LEVEL	Outcome
	DENOMINATOR	ART cumulative started ART minus cumulative TFO	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
ART CLIENT VIRAL LOAD DONE RATE (VLD)	NUMERATOR	ART client viral load done (VLD)	DENOMINATOR FORMULA	SUM([Adult ART on first-line regimen (FLR) at 6 months]) + SUM([Adult ART on second-line regimen (SLR) at 6 months])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult viral load done (VLD) at 6 months])	DEFINITION	Proportion of clients still on treatment who had viral load test done at specific time intervals. This is a programme quality indicator. *Note: Cohorts will be followed for the following indicators at six and 12 months and thereafter annually. Died, LTF and VL indicators need monitoring only at five-year intervals after 60 months.	LEVEL	Output
	DENOMINATOR	ART first line regimen + ART second line regimen at intervals	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring; National Strategic Plan: HIV and AIDS; Global AIDS Report

ART CLIENT REMAINING IN CARE RATE (RIC)						
	NUMERATOR	ART TOT minus cumulative LTF minus cumulative RIP minus cumulative TFO	DENOMINATOR FORMULA	(SUM([Adult naive male started ART]) + SUM([Adult naive female started ART])) - (SUM([Adult ART transferred out (TFO)])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult naive male started ART]) + SUM([Adult naive female started ART])) - (SUM([Adult ART lost to follow up (LTF)])) - (SUM([Adult ART died (RIP)])) - (SUM([Adult ART transferred out (TFO)])	DEFINITION	Cumulative proportion of clients on treatment after interval. This value will enable the programme to assess retention on treatment over time. Note that the TFO, LTF and RIP used in the formula are the cumulative totals of all clients who left care to that point, as opposed to the total in the most recent interval. Note: These data will be disaggregated by duration (months) and age (adult + child).	LEVEL	Outcome
	DENOMINATOR	ART cumulative started ART minus ART cumulative TFO	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

ART CLIENT LOST TO FOLLOW-UP RATE (LTF)						
	NUMERATOR	ART cumulative LTF	DENOMINATOR FORMULA	SUM([Adult naive male started ART]) + SUM([Adult naive female started ART])) - (SUM([Adult ART transferred out (TFO) at 3 months])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult ART lost to follow up (LTF)])	DEFINITION	Cumulative proportion of clients who were lost to follow-up after interval. At different time intervals, the cumulative deaths can be analysed. Note that the TFO, LTF and RIP used in the formula are the cumulative totals of all clients who left care to that point, as opposed to the total in the most recent interval. Note: These data will be disaggregated by duration (months) and age (adult + child).	LEVEL	Outcome
	DENOMINATOR	ART cumulative started ART minus cumulative TFO	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

ART CLIENT ON SECOND-LINE REGIMEN RATE (SLR)	NUMERATOR	ART on second-line regimen (SLR)	DENOMINATOR FORMULA	SUM([ART on first-line regimen (FLR)]) + SUM([ART on second-line regimen (SLR)]) + SUM([ART stopped (STO)])	TYPE	%
	NUMERATOR FORMULA	SUM([ART on second-line regimen (SLR)])	DEFINITION	ART client on second-line regimen after a period of time. This shows retention in care and if this proportion increases over time, it shows that clients may be building more resistance to first-line drug regimens. Note: This indicator will be disaggregated by duration (months) and age (adult + child).	LEVEL	Outcome
	DENOMINATOR	ART First line regimen + Second line regimen + Stopped at intervals	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
ART CLIENT ON THIRD-LINE REGIMEN RATE (TLR)	NUMERATOR	ART client on third-line regimen (TLR)	DENOMINATOR FORMULA	SUM([ART on first-line regimen (FLR)]) + SUM([ART on second-line regimen (SLR)]) + SUM([ART on third-line regimen (TLR)]) + SUM([ART stopped (STO)])	TYPE	%
	NUMERATOR FORMULA	SUM([ART on third-line regimen (TLR)])	DEFINITION	ART client on second-line regimen after a period of time. This shows retention in care and if this proportion increases over time, it shows that clients may be building more resistance to first-line drug regimens. Note: This indicator will be disaggregated by duration (months) and age (adult + child).	LEVEL	Outcome
	DENOMINATOR	ART First line regimen + Second line regimen + Third line regiment + Stopped at intervals	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

ART CLIENT NAIVE START ART DURING MONTH – TOTAL	NUMERATOR	ART adult and child under 15 years naive start ART during month	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([ART adult naïve start ART in month) + SUM([ART child under 15 years naïve start ART in month])	DEFINITION	Total naive clients who started ART are the sum of the following: » Clients who have never been exposed to ART for more than 30 days in total » Clients from the PEP programme » Clients from the dual PMTCT programme	LEVEL	Output
	DENOMINATOR	None	USE AND CONTEXT	Monitors total number of clients started on ART during month	KEY REPORTS	APP, ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
ART CLIENT REMAIN ON ART END OF MONTH – TOTAL	NUMERATOR	ART adult and child under 15 years remaining on ART end of month	DENOMINATOR FORMULA	None	TYPE	No
	NUMERATOR FORMULA	SUM([ART adult remain on ART end of period]) + SUM([ART child under 15 years remain on ART end of period])	DEFINITION	Total clients remaining on ART (TROA) are the sum of the following: » Any client who has a current regimen in the column designating the month you are reporting on » Any client who has a star without a circle (someone who is not yet considered lost to follow-up (LTF) in the column designating the month you are reporting on Clients remaining on ART equals [naive (including PEP and PMTCT) + Experienced (Exp) + Transfer in (TFI) + Restart] minus [Died (RIP) + loss to follow-up (LTF) + Transfer out (TFO)]	LEVEL	Output
	DENOMINATOR	None	USE AND CONTEXT	Monitors the total clients remaining on ART at the month	KEY REPORTS	APP, ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

ART CLIENT SCREENED FOR TB AT BASELINE RATE	NUMERATOR	ART client not on TB treatment screened for TB at baseline	DENOMINATOR FORMULA	SUM([Adult naïve male started ART]) + SUM([Adult naïve male started ART]) - ART on TB treatment at baseline (TBT)	TYPE	%
	NUMERATOR FORMULA	SUM([Adult screened for TB at baseline of ART]) + SUM([Child under 15 years screened for TB at baseline of ART])	DEFINITION	Proportion of clients not on tuberculosis (TB) treatment who were screened for tuberculosis at start of ART. The indicator is disaggregated by age (adult and child).	LEVEL	Output
	DENOMINATOR	ART not on TB treatment at baseline	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
ART ON TB TREATMENT AT BASELINE RATE (TBT)	NUMERATOR	ART client on TB treatment at baseline	DENOMINATOR FORMULA	SUM([Adult naïve male started ART]) + SUM([Adult naïve male started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult on TB treatment at baseline of ART]) + SUM([Child under 15 years on TB treatment at baseline of ART])	DEFINITION	Proportion of clients who were on tuberculosis (TB) treatment at start of ART. The indicator is disaggregated by age (adult and child).	LEVEL	Output
	DENOMINATOR	Naive started ART	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
ART CLIENT EXPERIENCED AT ART BASELINE RATE (EXP)	NUMERATOR	ART client experienced (adult + child)	DENOMINATOR FORMULA	SUM([Adult naïve male started ART]) + SUM([Adult naïve female started ART]) + SUM([Child under 1 year naïve started ART]) + SUM([Child 1 to under 5 years naïve started ART]) + SUM([Child 5 to under 15 years naïve started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult ART experienced started]) + SUM([Child under 15 years ART experienced started])	DEFINITION	Proportion of clients with treatment experience at ART start	LEVEL	Output
	DENOMINATOR	Adult and Child under 15 years started on ART	USE AND CONTEXT	None	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

CD4 DONE AT BASELINE RATE (CDD)	NUMERATOR	Adult CD4 done at start of ART	DENOMINATOR FORMULA	SUM([Adult naïve male started ART]) + SUM([Adult naïve female started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult CD4 below 100 cls/µl at start of ART]) + SUM([Adult CD4 100 to 199 cls/µl at start of ART]) + SUM([Adult CD4 200 to 349 cls/µl at start of ART + SUM([Adult CD4 350 to 499 cls/µl at start of ART + SUM([Adult CD4 > = 500 cls/µl at start of ART])	DEFINITION	Proportion of adults who had a baseline CD4 count (BCDD) done before they started on ART. This indicator is disaggregated by age (adult and child).	LEVEL	Output
	DENOMINATOR	ART started ART	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
ART CLIENT ON CPT AT BASELINE RATE (CPT)	NUMERATOR	Adult on Co-trimoxazole prevention therapy (CPT) at ART baseline	DENOMINATOR FORMULA	SUM([Adult naïve male started ART]) + SUM([Adult naïve female started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult on Co-trimoxazole prevention therapy (CPT) at baseline of ART]	DEFINITION	Proportion of adults who were on Co-trimoxazole prevention therapy at start of ART	LEVEL	Output
	DENOMINATOR	Adult started ART	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
ART CLIENT ON IPT AT BASELINE RATE (IPT)	NUMERATOR	ART client on INH prevention therapy (IPT) baseline	DENOMINATOR FORMULA	SUM([Adult naïve male started ART]) + SUM([Adult naïve male started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult on INH prevention therapy (IPT) at baseline of ART])	DEFINITION	Proportion of clients who were on INH prevention therapy at start of ART. The indicator is disaggregated by adult and child.	LEVEL	Output
	DENOMINATOR	Naive started ART	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

CD4 200 TO 349 CLS/µL AT BASELINE RATE (C200-349)	NUMERATOR	CD4 200 to 349 cls/µl at baseline	DENOMINATOR FORMULA	SUM([Adult CD4 below 100 cls/µl at baseline of ART]) + SUM([Adult CD4 100 to 199 cls/µl at baseline of ART]) + SUM([Adult CD4 200 to 349 cls/µl at baseline of ART + SUM([Adult CD4 350 to 499 cls/µl at baseline of ART + SUM([Adult CD4 > = 500 cls/µl at start of ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult CD4 200 to 349 cls/µl at baseline])	DEFINITION	Proportion of clients with CD4 between 200 and 349 cells/µl at baseline. For children, between 20 and 25 per cent is also counted.	LEVEL	Output
	DENOMINATOR	CD4 done at baseline	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
CD4 350 TO 499 CLS/µL AT BASELINE RATE (C350-499)	NUMERATOR	CD4 350 to 499 cls/µl at baseline	DENOMINATOR FORMULA	SUM([Adult CD4 below 100 cls/µl at baseline of ART]) + SUM([Adult CD4 100 to 199 cls/µl at baseline of ART]) + SUM([Adult CD4 200 to 349 cls/µl at baseline of ART + SUM([Adult CD4 350 to 499 cls/µl at baseline of ART + SUM([Adult CD4 > = 500 cls/µl at start of ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult CD4 350 to 499 cls/µl at baseline])	DEFINITION	Proportion of clients with CD4 between 350 and 499 cells/µl at baseline.	LEVEL	Output
	DENOMINATOR	CD4 done at baseline	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
CD4 500 AND ABOVE AT BASELINE RATE (C > = 500)	NUMERATOR	ART CD4 500 cls/µl and above at baseline (C > = 500)	DENOMINATOR FORMULA	SUM([Adult CD4 below 100 cls/µl at baseline of ART]) + SUM([Adult CD4 100 to 199 cls/µl at baseline of ART]) + SUM([Adult CD4 200 to 349 cls/µl at baseline of ART + SUM([Adult CD4 350 to 499 cls/µl at baseline of ART + SUM([Adult CD4 > = 500 cls/µl at start of ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult CD4 > = 500 cls/µl at baseline])	DEFINITION	Proportion of clients with CD4 count 500 cells/µl and above at baseline. This indicator reflects the implementation of ART policy guidelines.	LEVEL	Output
	DENOMINATOR	CD4 done at baseline	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

CD4 BELOW 100 CLS/µL AT BASELINE RATE (C<100)	NUMERATOR	CD4 below 100 cls/µl at baseline	DENOMINATOR FORMULA	SUM([Adult CD4 below 100 cls/µl at baseline of ART]) + SUM([Adult CD4 100 to 199 cls/µl at baseline of ART]) + SUM([Adult CD4 200 to 349 cls/µl at baseline of ART + SUM([Adult CD4 350 to 499 cls/µl at baseline of ART + SUM([Adult CD4 >=500 cls/µl at start of ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult CD4 below 100 cls/µl at baseline])	DEFINITION	Proportion of clients with CD4 count below 100 cells/µl at baseline. Shows the proportion of clients starting ART who are severely immunologically compromised. If the value changes over time, the programme will be able to assess the impact of the intervention. The indicator is broken down by adults and children. In children, below 15 per cent is also counted under this indicator.	LEVEL	Output
	DENOMINATOR	CD4 done at baseline	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
CD4 100 TO 199 CLS/µL AT BASELINE RATE (C100-199)	NUMERATOR	CD4 100 to 199 cls/µl at baseline	DENOMINATOR FORMULA	SUM([Adult CD4 below 100 cls/µl at start of ART]) + SUM([Adult CD4 100 to 199 cls/µl at start of ART]) + SUM([Adult CD4 200 to 349 cls/µl at start of ART + SUM([Adult CD4 350 to 499 cls/µl at start of ART + SUM([Adult CD4 >=500 cls/µl at start of ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult CD4 100 to 199 cls/µl at baseline])	DEFINITION	Proportion of clients with CD4 between 100–199 cells/µl at baseline. The indicator is disaggregated by adults and children. For children, between 15 and 19 per cent is also counted.	LEVEL	Output
	DENOMINATOR	CD4 done at baseline	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

ADULT MALE START ART RATE (MEN)	NUMERATOR	Adult male started ART	DENOMINATOR FORMULA	SUM([Adult naïve male started ART]) + SUM([Adult naïve female started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Adult naïve male started ART])	DEFINITION	Proportion of adult males who started ART. Treatment-experienced clients are excluded.	LEVEL	Output
	DENOMINATOR	Adult started on ART	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
CHILD UNDER 1 YEAR START ART RATE (K<1)	NUMERATOR	Child under 1 year started ART	DENOMINATOR FORMULA	SUM([Child under 1 year naïve started ART]) + SUM([Child 1 to under 5 years naïve started ART]) + SUM([Child 5 to under 15 years naïve started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Child under 1 year naïve started ART])	DEFINITION	Proportion of children under 1 year who started ART. Treatment-experienced clients are excluded.	LEVEL	Output
	DENOMINATOR	Child under 15 years started ART	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
CHILD 1 TO UNDER 5 YEARS START ART RATE (K1-<5)	NUMERATOR	Child 1-<5 years started ART	DENOMINATOR FORMULA	SUM([Child under 1 year naïve started ART]) + SUM([Child 1 to under 5 years naïve started ART]) + SUM([Child 5 to under 15 years naïve started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Child 1 to under 5 years naïve started ART])	DEFINITION	Proportion of children 1–5 years who started ART. Treatment-experienced clients are excluded.	LEVEL	Output
	DENOMINATOR	Child under 15 years started ART	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report
CHILD 5 TO UNDER 15 YEARS START ART RATE (K5-<15)	NUMERATOR	Child 5-<15 years started ART	DENOMINATOR FORMULA	SUM([Child under 1 year naïve started ART]) + SUM([Child 1 to under 5 years naïve started ART]) + SUM([Child 5 to under 15 years naïve started ART])	TYPE	%
	NUMERATOR FORMULA	SUM([Child 5 to under 15 years naïve started ART])	DEFINITION	Proportion of children 1–5 years who started ART. Treatment-experienced clients are excluded.	LEVEL	Output
	DENOMINATOR	Child under 15 years started ART	USE AND CONTEXT	This indicator only refers to ART-naive patients started in this facility and does not track Transfers in or Treatment-experienced patients.	KEY REPORTS	ART Programme Performance Monitoring, National Strategic Plan: HIV and AIDS, Global AIDS Report

PHC UTILISATION RATE 10–19 YEARS						
	NUMERATOR	PHC headcount 10–19 years	DENOMINATOR FORMULA	SUM([Female 10–19 years]) + SUM([Male 10–19 years])	TYPE	No
	NUMERATOR FORMULA	SUM([PHC headcount under 10– 19 years])	DEFINITION	Adolescents and youth 10–19 years accessing health services as a proportion of total population	LEVEL	Output
	DENOMINATOR	Population 10–19 years	USE AND CONTEXT	This indicator is meant to measure access of adolescent and youth health services by young people age 10–19 years in primary health care facilities.	KEY REPORTS	None

INDICATORS

RIC	Remaining in care
RIP	Died [rest in peace]
RV	Rotavirus
STH	Soil-transmitted helminths
STI	Sexually transmitted infections
TLR	Third-line regimen
TROA	Total remaining on ART
SAC	Severity assessment code
SAM	Severe acute malnutrition
SD	Standard deviation
SLR	Second-line regimen
StatsSA	Statistics South Africa
TB	Tuberculosis
Td	Tetanus and diphtheria
TFI	Transferred in
TFO	Transferred out
VDS	Vaginal discharge syndrome
Vit A	Vitamin A
VLD	Viral load
VLS	Suppressed viral load
XDR	Extensively drug-resistant

ACRONYMS

ANC	Antenatal care
ART	Antiretroviral therapy
BCG	Bacille Calmette-Guérin
CCMDD	Central Chronic Medicines Dispensing and Distribution
CDC	Community day centre
CD4	Cluster of differentiation 4
CHC	Community health centre
CPT	Co-trimoxazole prevention therapy
DTaP-IPV-Hib-HBV	Diphtheria, tetanus, inactivated polio vaccine, haemophilus influenza B, hepatitis vaccine
EH	Environmental health
EMS	Emergency medical services
EXP	Experienced
HIV	Human Immunodeficiency Virus
IC	Ideal Clinic
ICD	International Classification of Diseases
ICU	Intensive care unit
IPT	Isoniazid Preventive Therapy
ISHP	Integrated School Health Programme
IUCD	Intra-uterine contraceptive device
LTF	Loss/lost to follow-up
MAM	Moderate acute malnutrition
NAM	No acute malnutrition
MDR	Multidrug-resistant
MUS	Male urethritis syndrome
NCS	National Core Standards
OHH	Outreach households
OPD	Outpatient department
OPV	Oral polio vaccine
PCR	Polymerase chain reaction
PCV	Pneumococcal conjugated vaccine
PDE	Patient-day equivalent
PHC	Primary health care
PrEP	Pre-exposure prophylaxis
PSI	Patient safety incident
PPTICRM	Perfect Permanent Team for Ideal Clinic Realisation and Maintenance

COLOUR KEY

- Adolescent Health
- ART Baseline
- ART Monthly
- ART Outcome
- Central Chronic Medicines Dispensing and Distribution
- Child and Nutrition
- Chronic
- Communicable Diseases
- Emergency Medical Services
- Environmental Health
- Expanded Programme on Immunisation
- Eye Care
- HIV
- Malaria
- Management Inpatients
- Management PHC
- Maternal and Neonatal
- Mental Health
- Oral Health
- PHC Ward Based Outreach Teams
- Quality
- Rehabilitation
- School Health
- Sexually Transmitted Infections
- TB Monthly
- TB Quarterly
- Women's Health

ABOUT THIS GUIDE

The National Indicator Data Set (NIDS) is a minimum group of indicators introduced by the National Department of Health that every public health facility is expected to collect, use and report on. Facilities are required to report only on indicators in which they render the service according to their service package. The NIDS is use for monitoring and tracking progress with the implementation of national strategic goals, objectives and priority health programmes. The NIDS will help the Department of Health to identify the needs in the healthcare system and provide managers with information to guide programme development and budgeting, provide support to improve the healthcare system, and demonstrate national commitment to globally accepted measures of care.

The District Health Management Information System (DHMIS) Policy requires that the NIDS be revised every two years; the last review was done in 2013 and was implemented in April 2013. Due to the rationalisation of the data collection tools in 2015, the NIDS was not revised in 2015. The 2017 NIDS will be implemented in April 2017 and is effective until March 2019. There are special cases, for instance: policy pronouncements, which will require the Department to make additions or subtractions to the NIDS. In these cases, an official document signed by the Director-General of Health will be sent to all the provinces for implementation.

The quality of the information collected is vital to ensure that planning and the distribution of funds, equipment and personnel match the needs of the communities served by the health facility.

This resource is a comprehensive guide to the NIDS, and is designed to assist in maximising the accuracy and understanding of the data required to be collected.

Each section (or programme) in the guide is colour-coded to make its usage easier for health workers at facilities engaged in this work.

One half of the guide is dedicated to the data elements, their definitions, inclusion and exclusion criteria and collection points, and the reverse half is dedicated to the indicators calculated from these data elements.

Reproduction of this publication for commercial distribution is prohibited.
Requests to reprint this publication should be addressed to the Department of Health,
Director: Monitoring and Evaluation,
Nhlanhla Ntuli at NtuliNH@health.gov.za.

Note:

Implementation of Indicators marked with * in Chronic, Mental Health and Quality indicator groups will be confirmed after 30 October 2016.

The production of this document was made possible by the support of Health Systems Trust.

NIDS

INDICATORS

health
Department:
Health
REPUBLIC OF SOUTH AFRICA

National Indicator Data Set

INDICATOR DEFINITIONS APRIL 2017 TO MARCH 2019